

EN

Horizon Europe
Work Programme 2026-2027

4. Health

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Introduction

This Work Programme part is the final instalment for the Health Cluster under Horizon Europe (2021-2027), representing the last opportunity to deliver on the programme's objectives. It aims to address the remaining gaps, emerging research needs, and future challenges identified in the second Horizon Europe strategic plan¹, covering years 2025 to 2027. It also aligns with the Commission's Political Guidelines for 2024-2029², which focus on strengthening healthcare resilience, leveraging biotechnology and artificial intelligence, and addressing public health needs including supporting the development of critical medicines and strengthening societal preparedness and response. This will contribute to Europe's sustainable prosperity and competitiveness. Collaboration in research and innovation is key to achieving these goals.

In 2026-2027, the Health Cluster will pursue the following priorities:

- Contributing to the goal of making the EU the most attractive place of life sciences by 2030 and implementing the “Strategy for European Life Sciences”³.
- Addressing non-communicable diseases, including mental and cardiovascular health, through prevention, treatment, and management, supporting initiatives such as the “Healthier Together” EU Non-communicable Diseases Initiative and the EU Cardiovascular Health (CVH) plan which was presented by the Commission on 16th of December 2025, as part of an overall health package⁴.
- Understanding and mitigating the impacts of climate change, pollution, and biodiversity loss on human health and healthcare systems, supporting both the European Climate Adaptation Plan and the European Green Deal. This dual approach addresses the need to adapt to unavoidable climate impacts while contributing to broader mitigation efforts through transformative healthcare solutions.
- Building pandemic preparedness and response, including addressing antimicrobial resistance, in support of the European Health Union and the European Medical Countermeasures Strategy⁵ and coherent with the forthcoming Critical Medicines Act⁶.

¹ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/strategic-plan_en

² https://commission.europa.eu/about/commission-2024-2029_en

³ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

⁴ https://health.ec.europa.eu/document/download/dfb60cde-21a5-426d-8616-e394a326abc2_en?filename=ncd_com-2025-1024_act_en.pdf

⁵ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

⁶ https://health.ec.europa.eu/publications/proposal-critical-medicines-act_en

- Transforming Europe's healthcare systems to make them more effective, efficient, equitable, accessible, and sustainable, complementing the work of the co-funded European Partnership on Transforming Health and Care Systems⁷.
- Supporting digitalisation in healthcare, leveraging the innovation potential of health data and data-driven approaches, including AI, in the context of the European Health Data Space (EHDS)⁸ Regulation.
- Developing and using innovative tools and critical technologies, such as AI and biotechnology, to secure a competitive EU health industry and technological sovereignty in the healthcare sector, in line with the EU's Artificial Intelligence Strategy and the “Biotechnology and Biomanufacturing Communication”⁹.

While the entire Health Cluster Work Programme contributes to the “Strategy for European Life Sciences”, the following topics directly support key actions outlined in it: i) HORIZON-HLTH-2026-01-STAYHLTH-03: “Building public trust and outreach in the life sciences”, ii) HORIZON-HLTH-2026-01-ENVHLTH-01: “Towards a better understanding and anticipation of the impacts of climate change on health”, iii) HORIZON-HLTH-2027-01-ENVHLTH-02: “Integrating climate-related exposures into the human exposome and characterising its changes in response to climate change”, iv) HORIZON-HLTH-2027-01-ENVHLTH-MISSCLIMA-03: “Tools and technologies to support health adaptation to climate change”, v) HORIZON-HLTH-2026-01-ENVHLTH-04: “Towards climate resilient, prepared and carbon neutral populations and healthcare systems”, vi) HORIZON-HLTH-2026-01-ENVHLTH-05: “Support for a multilateral initiative on climate change and health research”, vii) HORIZON-HLTH-2027-02-DISEASE-01-two-stage: “Innovative healthcare interventions for non-communicable diseases”, viii) HORIZON-HLTH-2026-02-DISEASE-12: “European Partnership on Rare Diseases (ERDERA) (Phase 2)”, ix) HORIZON-HLTH-2026-03-DISEASE-13: “European Partnership for Pandemic Preparedness (Phase 2)”, x) HORIZON-HLTH-2027-02-DISEASE-14-two-stage: “Clinical trials for advancing innovative interventions for neurodegenerative diseases”, xi) HORIZON-HLTH-2026-01-CARE-01: “Public procurement of innovative solutions for improving citizens' access to healthcare through integrated or personalised approaches”, xii) HORIZON-HLTH-2026-01-TOOL-03: “Integrating New Approach Methodologies (NAMs) to advance biomedical research and regulatory testing”, xiii) HORIZON-HLTH-2026-01-TOOL-05: “Pilot actions for follow-on funding: Leveraging EU-funded collaborative research in regenerative medicine”, xiv) HORIZON-HLTH-2026-01-TOOL-06: “Support to European Research Area (ERA) action on accelerating New Approach Methodologies (NAMs) to advance biomedical research and testing of medicinal products and medical devices”, xv) HORIZON-HLTH-2026-01-TOOL-07: “Establishing a European network of Centres of Excellence (CoEs) for Advanced

⁷ <https://cordis.europa.eu/project/id/101095654>, <https://www.thcspartnership.eu>

⁸ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

⁹ https://research-and-innovation.ec.europa.eu/document/download/47554adc-dffc-411b-8cd6-b52417514cb3_en

Therapies Medicinal Products (ATMPs)” and xvi) HORIZON-HLTH-2025-02-DISEASE-01: “European Partnership for Brain Health”.

In addition to the priorities listed above, the Health Cluster will also continue to address the needs of specific populations, such as persons with disabilities and their families, with a focus on their empowerment. This is a crucial step towards ensuring that persons with disabilities can live independently and participate fully in society. Empowerment is also key for behavioural interventions, which this Work Programme part supports, by inviting proposals for the development of behavioural interventions as primary prevention for non-communicable diseases, to empower young people to adopt healthy lifestyles and reduce their risk of developing these diseases later in life.

Mental health remains a priority, with topics focusing on developing interventions to address the impact of climate change on mental health, as well as promoting healthy lifestyles and preventing mental health disorders. This includes a focus on the mental health of children and young adults, who are particularly vulnerable to the negative effects of digital technologies. The development of innovative interventions to prevent the harmful effects of using digital technologies on the mental health of children and young adults, in a gender sensitive way is a key objective, in line with the Commission's Political Guidelines for 2024-2029, which call for *"protecting the mental health of our children and young people"* in an increasingly digitalised world.

The Health Cluster will leverage public procurement to drive innovation, with two actions: a Pre-commercial Procurement (PCP) action on climate-resilient healthcare and a public procurement of innovative solutions action on integrated care, both aimed at improving healthcare outcomes.

This Work Programme part also supports the second phase of the co-funded European Partnership on Rare Diseases¹⁰ and the co-funded European Partnership for Pandemic Preparedness¹¹, providing continued funding to build on the progress achieved in the first phase and to further address the significant research, medical and societal challenges posed by rare diseases and pandemics.

It further identifies one action in support of STEP¹² objectives, for which proposals meeting the minimum requirements indicated in the specific call conditions will receive a STEP seal¹³ (See topic HORIZON-HLTH-2026-01-TOOL-07: “Establishing a European network of Centres of Excellence (CoEs) for Advanced Therapies Medicinal Products (ATMPs)”).

¹⁰ <https://cordis.europa.eu/project/id/101156595>, <https://erdera.org>

¹¹ <https://cordis.europa.eu/project/id/101226682>, <https://beready4pandemics.eu>

¹² In March 2024, the Commission adopted the Strategic Technologies for Europe Platform (STEP) to boost investments in critical technologies in Europe: clean and resource efficient technologies, digital and deep innovation technologies and biotechnologies. STEP will mobilise funding from existing EU programmes to support the development and manufacturing of these critical technologies, while safeguarding and strengthening the respective value chains, as well as associated services and skills critical for and specific to the development and manufacturing of the final products.

¹³ https://strategic-technologies.europa.eu/investors_en

Realising the potential of new Research and Innovation for society, requires close collaboration between research teams and prospective users of the knowledge and technology developed is paramount. It is therefore essential to involve these users -such as patients, healthy citizens, healthcare professionals, providers and payers, public health authorities, regulators, and innovators from academia and industry- early in the process of knowledge generation and technology development. This involvement can take the form of patient and citizen engagement, community involvement, and other social innovation approaches, ensuring that Research and Innovation activities align with the specific expectations, needs, constraints, and potential of users. Furthermore, effective intellectual property management strategies are crucial to maximise the benefits of such cooperation.

It is in the EU's strategic interest to cooperate with countries beyond the EU, particularly for multilateral cooperation on (global) health issues. This includes countries associated to Horizon Europe as well as other partner countries and regions worldwide. In line with the EU's Global Approach to Research and Innovation¹⁴, participation in the Health Cluster of Horizon Europe is open to third countries. Supporting the Global Gateway Strategy¹⁵, projects involving international partners should aim to increase scientific knowledge and facilitate technology transfer among partner countries, addressing global health challenges and fostering sustainable growth and job creation. Such cooperation should be value-based, creating linkages rather than dependencies. Please note that eligibility to participate is also subject to the 'Participation of Chinese universities linked to the Ministry of Industry and Information Technology (MIIT)' eligibility condition (see Annex B of the General Annexes of this Work Programme).

Applicants are encouraged to explore opportunities for synergies between the Health Cluster and other EU programmes¹⁶ to enhance the reach and impact of their projects, such as through broader stakeholder cooperation and follow-on activities. Synergies are in particular foreseen between the Health Cluster and the EU4Health Programme (2021-2027)¹⁷ to facilitate the uptake, further development and deployment of new knowledge and technologies in fields such as cancer, non-communicable diseases, mental health, pandemic preparedness and antimicrobial resistance, health systems and digital health. Synergies are also foreseen between the Health Cluster and the Digital Europe Programme¹⁸ to leverage Horizon Europe Research and Innovation results, such as deploying digital, privacy-preserving (distributed) data infrastructures, high-performance computing resources, and developing methods and tools for modelling complex phenomena related to human health.

¹⁴ COM(2021) 252 final

¹⁵ JOIN(2021) 30 final

¹⁶ E.g. the EU4Health Programme, the Digital Europe Programme, the European Regional Development Fund (ERDF), including Interreg, the European Social Fund (ESF+), the Structural Reform Support Programme (SRSP), the Just Transition Fund (JTF), the European Maritime, Fisheries and Aquaculture Fund (EMFAF), the European Agricultural Fund for Rural Development (EAFRD), the European Defence Fund (EDF) or InvestEU.

¹⁷ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

¹⁸ <https://digital-strategy.ec.europa.eu/en/activities/digital-programme>

The European Regional Development Fund (ERDF) -including Interreg- focuses, amongst others, on the development and strengthening of regional and local Research and Innovation ecosystems and smart economic transformation, in line with regional/national smart specialisation strategies. The programme can for example support investment in research infrastructure, activities for applied Research and Innovation, including industrial research, experimental development and feasibility studies, building on Research and Innovation stemming from Horizon Europe¹⁹.

To further strengthen the impact of Research and Innovation efforts, Horizon Europe applicants could consider tapping into complementary activities offered by other relevant initiatives funded under the Horizon Europe programme. These include the innovation ecosystems and service provisions of the Knowledge and Innovation Communities (KICs) of the European Institute of Innovation and Technology (EIT), particularly EIT-KIC Health and EIT-KIC Digital, or the interregional networks funded under the European Innovation Ecosystems (EIE) component of Pillar III.

In addition, applicants to the Health Cluster are encouraged to explore opportunities for complementary topics and activities in other Clusters or parts of the Horizon Europe programme that address thematically similar challenges and areas of intervention. This can be in the Clusters of Pillar II, in the European Research Infrastructures Work Programme part (Pillar I), or in the European Innovation Council Work Programme (Pillar III). More specifically, beneficiaries of Horizon Europe grants are invited to consider possible collaborations and cross-fertilisation between their project and other projects selected under the same or other relevant calls.

For topics in this Cluster, consortia could consider voluntarily contributing data, indicators, and knowledge to relevant platforms of the European Commission's Joint Research Centre (JRC) and, where relevant, exchanging data and access with the decentralised EU agencies. This would help capitalise on the knowledge developed in their projects and enhance their relevance to policymaking^{20, 21, 22, 23, 24, 25}.

In the context of the Health Cluster Work Programme part for 2026-2027, FAIR data are data which meet the principles of findability, accessibility, interoperability, and reusability. Data may include, amongst others, exploitation of information, digital research data generated in the action, data from European research infrastructures and programmes such as Copernicus, European Space Agency and the GEO initiative. For further details, see the FAIR principles

¹⁹ Synergies between Horizon Europe and ERDF programmes (including Interreg): https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=oj:JOC_2022_421_R_0003

²⁰ https://health.ec.europa.eu/system/files/2022-02/eu_cancer-plan_en_0.pdf

²¹ The European Cancer Information System (ECIS - <https://ecis.jrc.ec.europa.eu>) and the European Network of Cancer Registries (ENCR - <https://www.encreu.eu>)

²² European Commission Initiatives on Breast and Colorectal Cancer: <https://healthcare-quality.jrc.ec.europa.eu>

²³ European Cancer Inequalities Registry: <https://cancer-inequalities.jrc.ec.europa.eu>

²⁴ European Platform on Rare Disease Registration (EU RD Platform - <https://eu-rd-platform.jrc.ec.europa.eu/en>) - for rare cancers

²⁵ Health Promotion and Disease Prevention Knowledge Gateway Horizon Europe: https://knowledge4policy.ec.europa.eu/health-promotion-knowledge-gateway_en

website²⁶, the FAIR cookbook²⁷ and the guides for researchers on how to make your data FAIR²⁸.

Applicants to calls of the Health Cluster are encouraged to consider, where relevant, the services offered by current and future EU-funded European Research Infrastructures, including those prioritised by the European Strategy Forum on Research Infrastructures (ESFRI)²⁹, European Research Infrastructure Consortia (ERICs)³⁰ and the European Open Science Cloud³¹. Moreover, if projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, they must make use of European space technologies and services provided by Copernicus and/or Galileo/EGNOS (other data and services may additionally be used)³².

In the context of the Health Cluster Work Programme part for 2026-2027, a clinical study covers clinical studies/trials/investigations/cohorts and is defined as any systematic prospective or retrospective collection and analysis of health data obtained from individual patients or healthy persons in order to address scientific questions related to the understanding, prevention, diagnosis, monitoring or treatment of a disease, mental illness, or physical condition. It includes but it is not limited to clinical studies as defined by Regulation 536/2014 (on medicinal products), clinical investigation and clinical evaluation as defined by Regulation 2017/745 (on medical devices), performance study and performance evaluation as defined by Regulation 2017/746 (on in-vitro diagnostic medical devices).

Please note that the European Union (EU) pharmaceutical legislation known as the Clinical Trials Regulation No 536/2014³³ entered into application on 31 January 2022, repealing the Clinical Trials Directive (EC) No. 2001/20/EC and national implementing legislation in the EU Member States, which regulated clinical trials in the EU until the Regulation's entry into application. As a result, from 31 January 2023, all initial clinical trial applications in the European Union (EU) must be submitted via the Clinical Trials Information System (CTIS)³⁴. CTIS is now the single-entry point for sponsors and regulators of clinical trials for the submission and assessment of clinical trial data.

Applicants are encouraged to consider the latest innovations and advances in the clinical trial design and research methods in order to evaluate promising interventions allowing shorter

²⁶ <https://www.go-fair.org/fair-principles>

²⁷ <https://faircookbook.elixir-europe.org/content/home.html>

²⁸ <https://www.openaire.eu/how-to-make-your-data-fair>

²⁹ <https://ri-portfolio.esfri.eu>

³⁰ <https://www.eric-forum.eu/the-eric-landscape>

³¹ https://research-and-innovation.ec.europa.eu/strategy/strategy-2020-2024/our-digital-future/open-science/european-open-science-cloud-eosc_en

³² European space technology based earth observation, positioning, navigation and timing services provided by: Copernicus, the European Union's Earth observation programme <https://www.copernicus.eu/en/copernicus-services>; Galileo, the European Global Satellite Navigation System (GNSS) <https://www.gsc-europa.eu/galileo/services/galileo-initial-services>; and the European Geostationary Navigation Overlay Service (EGNOS) <https://www.euspa.europa.eu/eu-space-programme/egnos>

³³ <https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=celex%3A32014R0536>

³⁴ <https://euclinicaltrials.eu>

development timings. Applicants are also encouraged to follow the World Health Organization (WHO) guidance for best practices for clinical trials³⁵.

The Horizon Europe strategic plan (2025-2027) sets out three Key Strategic Orientations (KSOs) for the last three years of the EU's Framework Programme for Research and Innovation, namely: KSO 1: "The Green Transition," aiming to support Europe in becoming the world's first climate-neutral continent by 2050, tackling biodiversity loss and pollution; KSO 2: "The Digital Transition," focusing on reinforcing Europe's competitiveness and strategic autonomy through research in core digital technologies; and KSO 3: "A More Resilient, Competitive, Inclusive, and Democratic Europe," aiming to bolster Europe's social rights and democratic values, ensuring they are globally promoted. This includes research in civil security, health and wellbeing, a fair economic model, and democratic participation.

The Health Cluster will support these KSOs by enhancing the understanding of climate change impacts on health, developing tools to protect against global health challenges, and reducing the sector's carbon footprint. It will promote technological and digital advancements to improve healthcare systems, focusing on disease prevention, personalised treatment, and equitable access to health services. Additionally, it will foster inclusive and resilient healthcare systems capable of responding to cross-border health threats and demographic changes, leveraging digital technologies such as AI to accelerate health research and improve health outcomes.

More specifically, the Health Cluster will support the KSOs by contributing to the six expected impacts set out for the Health Cluster in the strategic plan 2025-2027, which translate into the following six destinations of the Health Cluster Work Programme part for 2026-2027:

Destination "Staying healthy in a rapidly changing society": The expected impact is that people of all ages in the EU stay healthy, resilient, and independent even as society changes fast. This will arise from healthier lifestyles and behaviour, healthier diets, healthier environments, improved evidence-informed health policies, and more effective solutions for health and wellbeing promotion, disease prevention and monitoring, and rehabilitation.

Destination "Living and working in a health-promoting environment": The expected impact is that people's living and working environments are health-promoting and sustainable thanks to a better understanding of the environmental, occupational, social, sex and gender-related, and economic determinants of health.

Destination "Tackling diseases and reducing disease burden": The expected impact is that healthcare providers improve their ability to tackle and manage diseases (infectious diseases, including poverty-related and neglected diseases, non-communicable and rare diseases) thereby reducing the disease burden on patients and enabling healthcare systems to perform more effectively. It can be achieved through better understanding, prevention, diagnostics, treatment, management, and cure of diseases and their co- and multi-morbidities, more

³⁵ <https://www.who.int/publications/i/item/9789240097711>

effective and innovative health technologies and medical countermeasures, better ability and preparedness to manage pandemic and/or epidemic outbreaks, and improved patient safety.

Destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”: The expected impact is that healthcare systems provide equal access to innovative, sustainable and high-quality healthcare thanks to the development and uptake of safe, cost-effective and people-centred solutions. This is to be accompanied by management models focusing on population health, health systems resilience, and health equity and patient safety, and also improved evidence-informed health policies.

Destination “Developing and using new tools, technologies and digital solutions for a healthy society”: The expected impact is that health technologies, data, new tools, and digital solutions are applied effectively thanks to their inclusive, ethically sound, secure and sustainable delivery, integration and deployment in health policies and in health and care systems.

Destination “Maintaining an innovative, sustainable, and competitive EU health industry”: The expected impact is that the EU health industry is innovative, sustainable, and globally competitive thanks to improved uptake of breakthrough technologies and innovations (including social innovations) that make the EU with its Member States and Associated Countries more resilient and less reliant on imports of critical health technologies.

Please note that the 'Horizontal Activities' Work Programme part also includes topics relevant to the areas covered in this Work Programme part.

Calls

Call - Cluster 1 - Health (Single stage - 2026)

HORIZON-HLTH-2026-01

Overview of this call³⁶

Proposals are invited against the following Destinations and topic(s):

Topics	Type of Action	Budgets (EUR million)	Expected EU contribution per project (EUR million) ³⁷	Indicative number of projects expected to be funded
		2026		
Opening: 10 Feb 2026 Deadline(s): 16 Apr 2026				
Destination - Staying healthy in a rapidly changing society				
HORIZON-HLTH-2026-01-STAYHLTH-02: Behavioural interventions as primary prevention for Non-Communicable Diseases (NCDs) among young people	RIA	20.60	9.00 to 10.00	2
HORIZON-HLTH-2026-01-STAYHLTH-03: Building public trust and outreach in the life sciences	CSA	1.90	1.50 to 1.90	1
Destination - Living and working in a health-promoting environment				
HORIZON-HLTH-2026-01-ENVHLTH-01: Towards a better understanding and anticipation of the impacts of climate change on health	RIA	55.00	7.00 to 8.00	7

³⁶ The Director-General responsible for the call may decide to open the call up to one month prior to or after the envisaged date(s) of opening.

The Director-General responsible may delay the deadline(s) by up to two months.

All deadlines are at 17.00.00 Brussels local time.

The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

³⁷ Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

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HORIZON-HLTH-2026-01-ENVHLTH-04: Towards climate resilient, prepared and carbon neutral populations and healthcare systems	RIA	45.00	7.00 to 8.00	6
HORIZON-HLTH-2026-01-ENVHLTH-05: Support for a multilateral initiative on climate change and health research	CSA	3.00	Around 3.00	1
Destination - Tackling diseases and reducing disease burden				
HORIZON-HLTH-2026-01-DISEASE-02: Innovative interventions to prevent the harmful effects of using digital technologies on the mental health of children and young adults	RIA	44.20	Around 8.00	6
HORIZON-HLTH-2026-01-DISEASE-03: Advancing research on the prevention, diagnosis, and management of post-infection long-term conditions	RIA	39.30	6.00 to 8.00	5
HORIZON-HLTH-2026-01-DISEASE-04: Development of novel vaccines for viral pathogens with epidemic potential	RIA	44.20	9.00 to 11.00	5
HORIZON-HLTH-2026-01-DISEASE-09: Multisectoral approach to tackle chronic non-communicable diseases: implementation research maximising collaboration and coordination with sectors and in settings beyond the healthcare system (GACD)	RIA	9.80	3.00 to 4.00	3
HORIZON-HLTH-2026-01-DISEASE-11: Understanding of sex and/or gender-specific mechanisms of cardiovascular diseases: determinants, risk factors and pathways	RIA	39.30	6.00 to 7.00	6
HORIZON-HLTH-2026-01-DISEASE-15: Scaling up innovation in cardiovascular health	CSA	1.90	Around 1.90	1
Destination - Ensuring equal access to innovative, sustainable, and high-quality healthcare				
HORIZON-HLTH-2026-01-CARE-01: Public procurement of innovative solutions for improving citizens' access to healthcare through integrated or personalised approaches	PPI	24.50	3.00 to 8.00	4
HORIZON-HLTH-2026-01-CARE-03: Identifying and addressing low-value care in	RIA	38.00	Around	4

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health and care systems			10.00	
Destination - Developing and using new tools, technologies and digital solutions for a healthy society				
HORIZON-HLTH-2026-01-TOOL-03: Integrating New Approach Methodologies (NAMs) to advance biomedical research and regulatory testing	RIA	49.00	5.00 to 8.00	7
HORIZON-HLTH-2026-01-TOOL-05: Pilot actions for follow-on funding: Leveraging EU-funded collaborative research in regenerative medicine	IA	29.50	6.00 to 8.00	4
HORIZON-HLTH-2026-01-TOOL-06: Support to European Research Area (ERA) action on accelerating New Approach Methodologies (NAMs) to advance biomedical research and testing of medicinal products and medical devices	CSA	2.90	Around 2.90	1
HORIZON-HLTH-2026-01-TOOL-07: Establishing a European network of Centres of Excellence (CoEs) for Advanced Therapies Medicinal Products (ATMPs)	CSA	3.90	Around 3.90	1
Destination - Maintaining an innovative, sustainable, and competitive EU health industry				
HORIZON-HLTH-2026-01-IND-03: Regulatory science to support translational development of patient-centred health technologies	RIA	19.60	4.00 to 6.00	4
Overall indicative budget		471.60		

General conditions relating to this call

<i>Admissibility conditions</i>	The conditions are described in General Annex A.
<i>Eligibility conditions</i>	The conditions are described in General Annex B.
<i>Financial and operational capacity and exclusion</i>	The criteria are described in General Annex C.

<i>Award criteria</i>	The criteria are described in General Annex D.
<i>Documents</i>	The documents are described in General Annex E.
<i>Procedure</i>	The procedure is described in General Annex F.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G.

Call - Partnerships in Health (2026/1)

HORIZON-HLTH-2026-02

Overview of this call³⁸

Proposals are invited against the following Destinations and topic(s):

Topics	Type of Action	Budgets (EUR million)		Expected EU contribution per project (EUR million) ³⁹	Indicative number of projects expected to be funded
		2026	2027		
Opening: 10 Feb 2026 Deadline(s): 15 Sep 2026					
Destination - Tackling diseases and reducing disease burden					
HORIZON-HLTH-2026-02-DISEASE-12: European Partnership on Rare Diseases (ERDERA) (Phase 2)	COFUND	48.70	42.60	Around 91.30	1
Overall indicative budget		48.70	42.60		

³⁸ The Director-General responsible for the call may decide to open the call up to one month prior to or after the envisaged date(s) of opening.
The Director-General responsible may delay the deadline(s) by up to two months.

All deadlines are at 17.00.00 Brussels local time.
The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

³⁹ Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

General conditions relating to this call	
<i>Admissibility conditions</i>	The conditions are described in General Annex A.
<i>Eligibility conditions</i>	The conditions are described in General Annex B.
<i>Financial and operational capacity and exclusion</i>	The criteria are described in General Annex C.
<i>Award criteria</i>	The criteria are described in General Annex D.
<i>Documents</i>	The documents are described in General Annex E.
<i>Procedure</i>	The procedure is described in General Annex F.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G.

Call - Partnerships in Health (2026/2)

HORIZON-HLTH-2026-03

Overview of this call⁴⁰

Proposals are invited against the following Destinations and topic(s):

Topics	Type of Action	Budgets (EUR million)		Expected EU contribution per project (EUR million) ⁴¹	Indicative number of projects expected to be funded
		2026	2027		

⁴⁰ The Director-General responsible for the call may decide to open the call up to one month prior to or after the envisaged date(s) of opening.
The Director-General responsible may delay the deadline(s) by up to two months.
All deadlines are at 17.00.00 Brussels local time.

The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

⁴¹ Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

Opening: 10 Feb 2027 Deadline(s): 13 Apr 2027					
Destination - Tackling diseases and reducing disease burden					
HORIZON-HLTH-2026-03-DISEASE-13: European Partnership for Pandemic Preparedness (Phase 2)	COFUND	30.00	33.00	Around 63.00	1
Overall indicative budget		30.00	33.00		

General conditions relating to this call	
<i>Admissibility conditions</i>	The conditions are described in General Annex A.
<i>Eligibility conditions</i>	The conditions are described in General Annex B.
<i>Financial and operational capacity and exclusion</i>	The criteria are described in General Annex C.
<i>Award criteria</i>	The criteria are described in General Annex D.
<i>Documents</i>	The documents are described in General Annex E.
<i>Procedure</i>	The procedure is described in General Annex F.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G.

Call - Partnerships in Health (2026/3)

HORIZON-HLTH-2026-04

Overview of this call⁴²

Proposals are invited against the following Destinations and topic(s):

⁴² The Director-General responsible for the call may decide to open the call up to one month prior to or after the envisaged date(s) of opening.
The Director-General responsible may delay the deadline(s) by up to two months.
All deadlines are at 17.00.00 Brussels local time.
The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

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Topics	Type of Action	Budgets (EUR million)	Expected EU contribution per project (EUR million) ⁴³	Indicative number of projects expected to be funded
		2026		
Opening: 10 Feb 2026 Deadline(s): 16 Apr 2026				
Destination - Ensuring equal access to innovative, sustainable, and high-quality healthcare				
HORIZON-HLTH-2026-04-CARE-04: Enhancing and enlarging the European Partnership on Personalised Medicine (EP PerMed) (Top-up)	COFUND	9.80	Around 9.80	1
Overall indicative budget		9.80		

General conditions relating to this call	
<i>Admissibility conditions</i>	The conditions are described in General Annex A.
<i>Eligibility conditions</i>	The conditions are described in General Annex B.
<i>Financial and operational capacity and exclusion</i>	The criteria are described in General Annex C.
<i>Award criteria</i>	The criteria are described in General Annex D.
<i>Documents</i>	The documents are described in General Annex E.
<i>Procedure</i>	The procedure is described in General Annex F.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G.

⁴³ Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

Call - Cluster 1 - Health (Single stage - 2027/1)

HORIZON-HLTH-2027-01

Overview of this call⁴⁴

Proposals are invited against the following Destinations and topic(s):

Topics	Type of Action	Budgets (EUR million)	Expected EU contribution per project (EUR million) ⁴⁵	Indicative number of projects expected to be funded
		2027		
Opening: 29 Oct 2026 Deadline(s): 17 Feb 2027				
Destination - Staying healthy in a rapidly changing society				
HORIZON-HLTH-2027-01-STAYHLTH-01: Addressing disabilities through the life course to support independent living and inclusion	RIA	39.30	6.00 to 8.00	5
Destination - Living and working in a health-promoting environment				
HORIZON-HLTH-2027-01-ENVHLTH-02: Integrating climate-related exposures into the human exposome and characterising its changes in response to climate change	RIA	45.00	10.00 to 11.00	4
HORIZON-HLTH-2027-01-ENVHLTH-MISSCLIMA-03: Tools and technologies to support health adaptation to climate change	PCP	20.00 ⁴⁶	4.00 to 5.00	4
Destination - Tackling diseases and reducing disease burden				
HORIZON-HLTH-2027-01-DISEASE-05: Development of novel small molecule antiviral	RIA	44.20	9.00 to 11.00	5

⁴⁴ The Director-General responsible for the call may decide to open the call up to one month prior to or after the envisaged date(s) of opening.
The Director-General responsible may delay the deadline(s) by up to two months.
All deadlines are at 17.00.00 Brussels local time.
The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

⁴⁵ Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

⁴⁶ Of which EUR 10.00 million from the 'Climate, Energy and Mobility' budget.

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therapeutics for pathogens with epidemic potential				
HORIZON-HLTH-2027-01-DISEASE-06: Development of monoclonal antibodies to prevent and treat infections from Flaviviruses	RIA	37.30	9.00 to 10.00	4
HORIZON-HLTH-2027-01-DISEASE-07: Development of monoclonal antibodies to prevent and treat infections from Filo-, Nairo-, Phenui-, Picorna- and Toga viruses	RIA	37.30	9.00 to 10.00	4
HORIZON-HLTH-2027-01-DISEASE-08: Development of innovative antimicrobials against pathogens resistant to antimicrobials	RIA	44.20	8.00 to 10.00	5
HORIZON-HLTH-2027-01-DISEASE-10: Prevention and management of chronic non-communicable diseases in children and young people (GACD)	RIA	11.80	3.00 to 4.00	3
Destination - Ensuring equal access to innovative, sustainable, and high-quality healthcare				
HORIZON-HLTH-2027-01-CARE-02: Personalised approaches to reduce risks from Adverse Drug Reactions due to administration of multiple medications	RIA	38.00	8.00 to 10.00	4
Destination - Maintaining an innovative, sustainable, and competitive EU health industry				
HORIZON-HLTH-2027-01-IND-01: Development of cell-free protein synthesis platforms for discovery and/or production of biologicals	RIA	24.50	6.00 to 8.00	4
Overall indicative budget		341.60		

General conditions relating to this call

<i>Admissibility conditions</i>	The conditions are described in General Annex A.
<i>Eligibility conditions</i>	The conditions are described in General Annex B.
<i>Financial and operational capacity and exclusion</i>	The criteria are described in General Annex C.

<i>Award criteria</i>	The criteria are described in General Annex D.
<i>Documents</i>	The documents are described in General Annex E.
<i>Procedure</i>	The procedure is described in General Annex F.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G.

Call - Cluster 1 - Health (Two stage - 2027)

HORIZON-HLTH-2027-02-two-stage

Overview of this call⁴⁷

Proposals are invited against the following Destinations and topic(s):

Topics	Type of Action	Budgets (EUR million) 2027	Expected EU contribution per project (EUR million) ⁴⁸	Indicative number of projects expected to be funded
Opening: 29 Oct 2026 Deadline(s): 17 Feb 2027 (First Stage), 16 Sep 2027 (Second Stage)				
Destination - Tackling diseases and reducing disease burden				
HORIZON-HLTH-2027-02-DISEASE-01-two-stage: Innovative healthcare interventions for non-communicable diseases	RIA	63.80	7.00 to 8.00	8
HORIZON-HLTH-2027-02-DISEASE-14-two-stage: Clinical trials for advancing	RIA	39.30	Around 10.00	4

⁴⁷ The Director-General responsible for the call may decide to open the call up to one month prior to or after the envisaged date(s) of opening.
The Director-General responsible may delay the deadline(s) by up to two months.

All deadlines are at 17.00.00 Brussels local time.
The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

⁴⁸ Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

Horizon Europe - Work Programme 2026-2027
Health

innovative interventions for neurodegenerative diseases				
Destination - Developing and using new tools, technologies and digital solutions for a healthy society				
HORIZON-HLTH-2027-02-TOOL-01-two-stage: Development of predictive biomarkers of disease progression and treatment response by using AI methodologies for chronic non-communicable diseases	RIA	44.20	6.00 to 8.00	6
Destination - Maintaining an innovative, sustainable, and competitive EU health industry				
HORIZON-HLTH-2027-02-IND-02-two-stage: Portable and versatile Point-of-care diagnostics	IA	39.30	5.00 to 7.00	6
Overall indicative budget		186.60		

General conditions relating to this call	
<i>Admissibility conditions</i>	The conditions are described in General Annex A.
<i>Eligibility conditions</i>	The conditions are described in General Annex B.
<i>Financial and operational capacity and exclusion</i>	The criteria are described in General Annex C.
<i>Award criteria</i>	The criteria are described in General Annex D.
<i>Documents</i>	The documents are described in General Annex E.
<i>Procedure</i>	The procedure is described in General Annex F.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G.

Call - Cluster 1 - Health (Single stage - 2027/2)

HORIZON-HLTH-2027-03

Overview of this call⁴⁹

Proposals are invited against the following Destinations and topic(s):

Topics	Type of Action	Budgets (EUR million)	Expected EU contribution per project (EUR million) ⁵⁰	Indicative number of projects expected to be funded
		2027		
Opening: 03 Jun 2027 Deadline(s): 16 Sep 2027				
Destination - Developing and using new tools, technologies and digital solutions for a healthy society				
HORIZON-HLTH-2027-03-TOOL-02: Advancing bio-printing of living cells for regenerative medicine	RIA	39.30	7.00 to 10.00	4
HORIZON-HLTH-2027-03-TOOL-04: Virtual Human Twins (VHTs) for integrated clinical decision support in prevention and diagnosis	RIA	39.30	10.00 to 12.00	4
HORIZON-HLTH-2027-03-TOOL-08: Towards the next generation of frontier Artificial Intelligence models for healthcare	CSA	2.90	Around 2.90	1
Overall indicative budget		81.50		

General conditions relating to this call

<i>Admissibility conditions</i>	The conditions are described in General Annex A.
<i>Eligibility conditions</i>	The conditions are described in General Annex B.

⁴⁹ The Director-General responsible for the call may decide to open the call up to one month prior to or after the envisaged date(s) of opening.
The Director-General responsible may delay the deadline(s) by up to two months.
All deadlines are at 17.00.00 Brussels local time.
The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

⁵⁰ Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

<i>Financial and operational capacity and exclusion</i>	The criteria are described in General Annex C.
<i>Award criteria</i>	The criteria are described in General Annex D.
<i>Documents</i>	The documents are described in General Annex E.
<i>Procedure</i>	The procedure is described in General Annex F.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G.

Destinations

Destination - Staying healthy in a rapidly changing society

Topics under this destination are directed towards the Key Strategic Orientations "A *more resilient, competitive, inclusive, and democratic Europe*" and "*The Digital transition*" of Horizon Europe's strategic plan 2025-2027⁵¹.

Research and Innovation supported under this destination should contribute to the following expected impact, set out in the strategic plan impact summary for the Health Cluster: "*people of all ages in the EU stay healthy, resilient, and independent even as society changes fast. This will arise from healthier lifestyles and behaviour, healthier diets, healthier environments, improved evidence-informed health policies, and more effective solutions for health and well-being promotion, disease prevention and monitoring, and rehabilitation*".

People's healthcare needs are different depending on their age, gender, stage of life, health status and socioeconomic background. In 2021, nearly 860,000 premature deaths across the EU⁵² could have been prevented with effective primary prevention and other public health measures. In addition, an estimated 135 million people in Europe live with a disability⁵³, highlighting the critical need for healthcare systems that are both accessible and adaptable. This number is expected to rise due to population ageing and the increasing prevalence of chronic conditions resulting from noncommunicable diseases and injuries. It is also important to consider disabilities arising from other causes, such as war-related injuries and Post-Traumatic Stress Disorder (PTSD), which add to the complexity and diversity of healthcare needs.

Aligning with the Commission's Political Guidelines for 2024-2029⁵⁴, which call for stepping up work on preventive health, this destination aims to strengthen disease prevention and early detection, placing support and empowerment of individuals regarding their own health, well-being and living and working conditions at the core of future public health programmes.

Research and Innovation under this destination should help enhance the dialogue and coordination among stakeholders and policymakers, ensuring integration across different care settings for holistic health promotion and disease prevention. Funded activities should seek to leverage the wealth of data sources, including real-world health data and establish a European interconnected health data ecosystem to develop integrated and personalised health promotion and disease prevention strategies. These activities will benefit from and actively support and

⁵¹ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/strategic-plan_en

⁵² "Health at a Glance: Europe 2024", available from https://health.ec.europa.eu/state-health-eu/health-glance-europe_en

⁵³ <https://www.who.int/europe/news-room/fact-sheets/item/disability> The WHO European Region comprises 53 countries, covering a vast geographical region from the Atlantic to the Pacific oceans.

⁵⁴ https://commission.europa.eu/about/commission-2024-2029_en

enrich emerging data resources such as the European Health Data Space (EHDS)⁵⁵ and European Open Science Cloud (EOSC)⁵⁶, and contribute to the European care strategy⁵⁷ and the digital transformation of health and care in the EU⁵⁸. Since Horizon Europe's launch in 2021, this destination has addressed important issues such as obesity prevention, understanding health-to-disease transitions, life course approaches to physical and mental health, healthy ageing, digital health literacy, and Artificial Intelligence (AI) for chronic disease risk prediction.

In this Work Programme part, destination “*Staying healthy in a rapidly changing society*” will focus on: i) addressing disabilities through the life course to support independent living and inclusion, with an emphasis on empowering persons with disabilities and their families. This priority aligns with the EU Strategy for the Rights of Persons with Disabilities 2021-2030; and ii) developing behavioural interventions as primary prevention for Non-Communicable Diseases (NCDs), with an emphasis on promoting healthy habits and sustained behavioural change among youth. This priority aligns with the ‘Healthier together’ EU non-communicable diseases initiative.

To increase the impact of EU investments under Horizon Europe, the Commission encourages collaboration between EU-funded projects to foster synergies through networking, joint workshops, knowledge exchange, best practices, and joint communication activities. Synergies can be explored between projects funded under the same or different topics, Clusters or Pillars of Horizon Europe. This includes collaborations between projects funded under the Health Cluster and the 'Culture, Creativity and Inclusive Society' Cluster for complementary actions, such as promoting social inclusion, health equity (including gender equality and support for groups at risk of discrimination), and mental health initiatives in education, work, and daily life (including through culture, the arts and sports).

Expected impacts:

Proposals for topics under this destination should set out a credible pathway to contributing to staying healthy in a rapidly changing society, and more specifically to one or several of the following impacts:

- Citizens, including persons with disabilities and other groups in a vulnerable situation, adopt and maintain healthier lifestyles and behaviours, make healthier choices, and achieve, where applicable, longer healthy, independent, and active lives with a reduced burden of preventable disease throughout the life course.

⁵⁵ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

⁵⁶ https://research-and-innovation.ec.europa.eu/strategy/strategy-2020-2024/our-digital-future/open-science/european-open-science-cloud-eosc_en

⁵⁷ Communication from the European Commission on the European care strategy, COM(2022) 440, 7.9.2022

⁵⁸ Communication from the European Commission on enabling the digital transformation of health and care in the Digital Single Market; empowering citizens and building a healthier society, COM(2018) 233, 25.4.2018

- Citizens are empowered to effectively manage their physical and mental health and well-being, monitor their health status, and interact with healthcare providers to optimise their well-being throughout life through improved health literacy, increased engagement in and adherence to health promotion strategies.
- Children and young people are aware and empowered to better monitor and manage their physical, social and mental health with a view to lifelong healthy lifestyles.
- Society benefits from reduced economic and health burdens due to preventable illness and premature mortality, with efficiency increased by targeting scarce resources in appropriate, cost-effective ways to areas of high social return, thereby driving improvements in health and well-being for all citizens, and specifically reducing health inequalities.

Health policies and actions for health promotion and disease prevention are knowledge-based, people-centred, personalised and thus targeted and tailored to citizens' needs, and designed to reduce health inequalities.

Legal entities established in China are not eligible to participate in both Research and Innovation Actions (RIAs) and Innovation Actions (IAs) falling under this destination. For additional information please see “Restrictions on the participation of legal entities established in China” found in the Annex B of the General Annexes of this Work Programme.

The protection of European communication networks has been identified as an important security interest of the Union and its Member States. Entities that are assessed as high-risk suppliers⁵⁹ of mobile network communication equipment (and any entities they own or control) are not eligible to participate as beneficiaries, affiliated entities and associated partners to topics identified as “subject to restrictions for the protection of European communication networks”. Please refer to the Annex B of the General Annexes of this Work Programme for further details.

Proposals are invited against the following topic(s):

HORIZON-HLTH-2026-01-STAYHLTH-02: Behavioural interventions as primary prevention for Non-Communicable Diseases (NCDs) among young people

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU</i>	The Commission estimates that an EU contribution of between EUR

⁵⁹ Entities assessed as “high-risk suppliers”, are currently set out in the second report on Member States’ progress in implementing the EU toolbox on 5G cybersecurity of 2023 (NIS Cooperation Group, Second report on Member States’ progress in implementing the EU Toolbox on 5G Cybersecurity, June 2023) and the related Communication on the implementation of the 5G cybersecurity toolbox of 2023 (Communication from the Commission: Implementation of the 5G cybersecurity Toolbox, Brussels, 15.6.2023 C(2023) 4049 final).

<i>contribution per project</i>	9.00 and 10.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 20.60 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Staying healthy in a rapidly changing society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Healthcare professionals have access to behavioural interventions that can be used to establish and reinforce healthy habits and sustain behavioural changes.
- Health professionals and educators have access to evidence-based strategies to mitigate risks of Non-Communicable Diseases (NCDs) for youth, with clear metrics that can be used to assess health outcomes.
- Youth have increased individual responsibility through targeted education, digital services, including easily accessible tools for self-monitoring, and community-based

support, stemming from increased collaboration between healthcare professionals, educators and families.

- Researchers have access to Real-World Data (RWD)⁶⁰, existing health data infrastructure and digital tools, including Artificial Intelligence (AI), which can contribute to the sustained success of behavioural health interventions.
- Policymakers at local, regional, national and EU levels have new knowledge on behavioural interventions on NCDs among youth, which they can use to improve interventions in diverse European contexts.

Scope: The topic is focused on behavioural interventions for youth, defined as 12 to 25 years old, for the primary prevention of the top NCDs later in life, where “top NCDs” refers to the most prevalent NCDs⁶¹. For the purpose of this call, NCDs explicitly exclude cancer.

Implementation research should be conducted to implement existing behavioural interventions. These interventions should be evidence-based and have an emphasis on empowerment and self-management (e.g. health literacy, health education, health promotion). As self-monitoring is an essential element of self-management, proposals should include user-friendly hardware and software for efficient self-monitoring (i.e. wearables and point-of-care devices for measuring various physiological parameters and other predictors and other biomarkers and the corresponding apps for easy readout and tracking, possibly also including gamification elements). Hardware and software should be interoperable in line with internationally accepted standards in order to avoid lock-in effects and assure scalability.

Proposals should also include most of the following aspects:

- Ensure that gender-sensitive and intersectional approaches are integrated, addressing potential gender-specific barriers for groups at risk of discrimination, as well as cultural and socioeconomic backgrounds, and should also outline how digital tools, including AI and RWD and biomarkers (e.g. genomic data, wearables, etc.) or existing relevant administrative dataset, will be integrated to enhance the scalability, personalisation, and effectiveness of interventions in the long-term.
- Present a clear, evidence-based strategy showing how the interventions will be tailored, deployed, and assessed at individual, family, community, and societal levels, while considering social inequalities and lifestyle factors (i.e. nutrition, sleep rhythm) and ensuring a robust methodological framework for evaluating the effectiveness of interventions (e.g. randomised controlled trials, quasi-experimental designs, etc.), with clearly defined indicators of success of the intervention (e.g. biometric markers, psychosocial wellbeing metrics, physical activity change, etc.). Applicants should evaluate unintended consequences for all interventions.

⁶⁰ EMA definition: “Real-World Data are routinely collected data relating to patient health status or the delivery of healthcare from a variety of sources other than traditional clinical trials (e.g. claims databases, hospital data, electronic health records, registries, mhealth data, etc.)”.

⁶¹ The prevalence of a disease is the number of cases in a defined population at a specified point in time. See <https://iris.who.int/bitstream/handle/10665/36838/9241544465.pdf>

- Include formats that will increase collaboration between healthcare professionals, educators, families, and policymakers in promoting preventive health and should include plans for longer-term follow-up to estimate health impact and cost savings over time. Related to this, applicants should outline how policy changes related to the intervention (e.g. school meal programmes, safe urban infrastructure for exercise, digital literacy campaigns, circadian alignment, stress reduction strategies) can reinforce and scale up successful behavioural interventions, whilst taking into account how they can be replicated or adapted to different cultural, geographic and socio-economic contexts. As such, active involvement of key stakeholders throughout the study is strongly encouraged.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, organisations as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

The proposals should adhere to the FAIR⁶² data principles, adopt data quality standards, data integration operating procedures and GDPR⁶³ compliant data sharing/access good practices developed by the European research infrastructures, where relevant.

Applicants should provide details of their clinical studies⁶⁴ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2026-01-STAYHLTH-03: Building public trust and outreach in the life sciences

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 1.50 and 1.90 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 1.90 million.
<i>Type of Action</i>	Coordination and Support Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply:

⁶² See definition of FAIR data in the introduction to this Work Programme part.

⁶³ General Data Protection Regulation: https://commission.europa.eu/law/law-topic/data-protection_en, <https://gdpr-info.eu>

⁶⁴ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	In recognition of the opening of the US National Institutes of Health's programmes to European researchers, legal entities established in the United States of America may exceptionally participate as a beneficiary or affiliated entity, and are eligible to receive Union funding. Coordinators of projects must be legal entities established in an EU Member State or Associated Country. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ⁶⁵.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Staying healthy in a rapidly changing society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Strengthened capacity of life sciences actors in science communication, risk communication, public outreach, and citizen engagement.
- Strengthened awareness of risks and benefits of life sciences by the public, by showcasing the latest Research and Innovation (R&I) developments in the life sciences, and their societal impact.

⁶⁵ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

- New and innovative approaches developed to engage the public in R&I activities in the life sciences, with an emphasis on inclusive and participatory approaches, involving relevant stakeholders (e.g. researchers, research funders, policymakers, publishers, civil society organisations, business, and citizens).

Scope: Life science innovations significantly contribute to peoples' daily life and to individual and social well-being. To foster public trust, people must understand how life sciences work and how these technologies may impact people's lives.

This trust is not guaranteed. It is increasingly threatened by the rapid spread of mis- and disinformation and by insufficient outreach to and involvement of people to address their concerns and expectations. To maintain and deepen trust, especially among young people, R&I policymakers, researchers and industry players must be better equipped to engage with the public and to pursue responsible research and innovation.

Citizen engagement is particularly critical in areas like agriculture and food technology, where innovation intersects with health and sustainability considerations and values. Furthermore, citizen participation is key to build trust in the life sciences and ensure that they meet societal needs. By engaging citizens early on, we can ensure that research and innovation align with their values, concerns and expectations. This involves designing research and innovation processes that incorporate citizen input, such as setting research priorities, and create outcomes that are responsive to their needs.

To this end, proposals should address all the following activities:

- Provide advisory support and training to life science stakeholders, in order to upskill them in science communication and risk communication.
- Produce, publish and advertise to the relevant actors, guidance to engage citizens upstream in the development, co-production, and co-design of life sciences innovation.
- Produce, publish, and advertise to the relevant public an accessible repository of tools for life science stakeholders on risk communication.
- Design and run community engagement activities in the life sciences, in partnership with relevant local actors, such as science museums, R&I organisations, and/or community organisations.

Regarding advisory support and training, proposals should present how they will engage bilaterally with life science stakeholders to advise them and train them on science communication and risk communication in the life sciences. The proposal selected for funding should also establish links with the European Competence Centre for Science

Communication⁶⁶ currently being created by the COALESCE⁶⁷ project and expected to be launched in 2027⁶⁸.

Regarding engagement of citizens in the development, co-production and co-design of life sciences innovations, proposals should focus on advising and training life science actors in deliberative citizen participation and co-design with citizens, including tools that allow discussions about values and ethical considerations of innovations in this sector. Proposals should set out the ways in which they will support life science actors to involve citizens in co-design, and also set out how they will evaluate the impact of the citizen engagement activities that they have supported. The consortium selected for funding is encouraged to use the tools and methods developed under previous research and tailor them to the life sciences.

Regarding the repository of tools and support for risk communication, such tools already exist and have been developed, notably the EU funded projects listed in the CORDIS Results Pack “Science communication: Empowering citizens in the public discussion of science”⁶⁹, the CORDIS Results Pack “Ethics and integrity: Building bridges for trust and excellence in research and innovation”⁷⁰ and the World Health Organization (WHO) guidelines for emergency risk communication⁷¹. Proposals should present what tools they will gather, how they will publish them, and what publicity and outreach they will conduct to raise awareness of this repository among the relevant life science actors (policymakers, researchers, industry, civil society organisations), tailoring tools and trainings to the life sciences. Proposals should present a long-term strategy for how the repository of tools will continue to be accessible beyond the lifecycle of this Coordination and Support Action (CSA).

Regarding community engagement activities, proposals should experiment with new and engaging formats across the programmed activities. Proposals should promote both science education, and multiple forms of public engagement with science, focusing on the life sciences. Proposals should focus on any areas within the life sciences, but at least one community engagement activity should focus on agriculture and food technology. The consortium selected for funding is encouraged to make use of findings and tools for stakeholder engagement developed by other Horizon Europe projects, including projects funded under topic HORIZON-CL6-2023-GOVERNANCE-01-6: “Co-creation and trust-building measures for biotechnology and bio-based innovation systems”. The consortium selected for funding is also encouraged to establish links with the projects funded under topic HORIZON-WIDERA-2026-07-ERA-05: “Pillar III: Fostering citizen engagement for more responsible and democratic R&I”, that will develop tools and guidelines on public engagement in R&I, and to tailor these tools and guidelines to applications in the life sciences.

⁶⁶ <https://scicommcentre.eu>

⁶⁷ <https://cordis.europa.eu/project/id/101095230>

⁶⁸ The European Competence Centre for Science Communication will be fully established by March 2027. Its development is being undertaken with a strong basis of co-creation amongst multiple stakeholders by the COALESCE project

⁶⁹ <https://cordis.europa.eu/article/id/442429-science-communication-empowering-citizens-in-the-public-discussion-of-science>

⁷⁰ <https://cordis.europa.eu/article/id/450170-ethics-and-integrity-building-bridges-for-trust-and-excellence-in-research-and-innovation>

⁷¹ <https://www.who.int/publications/i/item/9789241550208>

Proposals should explain how they will partner with relevant local actors, such as science museums, R&I organisations, and/or community organisations, to run innovative community engagement activities on the life sciences.

Proposals may consider involving the European Commission's Joint Research Centre (JRC) to participate in the advisory board of the consortium selected for funding, notably to benefit from the expertise of the JRC's Competence Centre on Participatory and Deliberative Democracy⁷², regarding the engagement of citizens in the design of life sciences innovations and community engagement activities. Any such collaboration should be established after the proposal's approval.

HORIZON-HLTH-2027-01-STAYHLTH-01: Addressing disabilities through the life course to support independent living and inclusion

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 6.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 39.30 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

⁷² https://knowledge4policy.ec.europa.eu/participatory-democracy_en

<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ⁷³.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Staying healthy in a rapidly changing society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Persons with disabilities are empowered and can enjoy their rights to live independently, participate in society and be included in the community on an equal basis with others.
- The scientific community develops innovative solutions with a focus on removing barriers faced by persons with disabilities to live independently and they are provided with community support services where they live in the community.
- Policymakers, health and care services, social and service providers, disability organisations, funders, the scientific community, and other relevant bodies are informed of the research advances and best practices addressing the health and needs of persons with disabilities to support them living independently and being included in society.

Scope: The focus of this topic is human-centred on persons with long-term disabilities⁷⁴ - physical, mental, intellectual or sensory- aiming at supporting independent living across the life-course from a health perspective. Persons with disabilities have an equal right to live independently and be included in the community. Independent living requires a differentiated landscape of quality, accessible, person-centred and affordable, community- and family-based services comprising personal assistance, medical and social care and interventions by social

⁷³ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

⁷⁴ Persons with disabilities include those who have long-term physical, mental, intellectual or sensory impairments which in interaction with various barriers may hinder their full and effective participation in society on an equal basis with others (Art. 1 of the Convention on the Rights of Persons with Disabilities - <https://www.ohchr.org/en/instruments-mechanisms/instruments/convention-rights-persons-disabilities>).

workers, thereby facilitating everyday activities and providing choice to persons with disabilities and their families⁷⁵.

The objective of this topic is to explore new ways to promote independent living and inclusion in society of persons with disabilities, reducing to the maximum possible the impact of barriers faced in their daily lives, and supporting the transition from institutions to living in the community while addressing all-encompassing aspects of personal support, such as community transformation, service provision, assistive and accessible technologies and environments.

Research actions under this topic should address at least three of the following areas:

- Health related research addressing disabilities that stem from health conditions and health conditions associated to disabilities. Thus, research may look into finding the causes of the disease(s) leading to the disability and/or disease treatment with the purpose of supporting independent living. Innovative solutions could also include among others diagnoses, medicines, treatments, protocols, technologies, digital tools, low-tech solutions, etc. helping to improve the autonomy of persons with disabilities.
- Children with disabilities from the perinatal period, and/or young people with disabilities transitioning to adulthood, and/or older persons⁷⁶. Proposals should foster ways to improve autonomy and quality of life.
- Access to habilitation and rehabilitation services, including psychological rehabilitation and innovative rehabilitation with assistive technologies when appropriate, to increase, maintain, substitute or improve functional capabilities of persons with disabilities or for, alleviation and compensation of impairments, activity limitations or participation restrictions contributing to increasing independence.
- Prevention of disabilities through the life-course. Different aspects that could have an impact on persons with disabilities may be addressed, such as gender, age, socio-economic background, ethnicity, detection of risks factors leading to a loss of autonomy, the risk of overweight/obesity and related co-morbidities (e.g. diabetes, cardiovascular diseases), hospitalisation, nutrition (e.g. mother and child nutrition from pregnancy), high level of inactivity/sedentary lifestyle and related co-morbidities (e.g. frailty), physical activity/sports, screen-time dependency, smoking, drug use⁷⁷, alcohol use, stress, psychiatric and somatic diseases, loneliness and/or isolation, etc.
- Conditions for a successful transition from institutions to living in the community, including different tools to achieve it, such as needs assessments, service provision, budget and resources, management plans, monitoring, quality control, etc. Community support services to live independently may include personal assistance or support for

⁷⁵ <https://op.europa.eu/en/publication-detail/-/publication/3e1e2228-7c97-11eb-9ac9-01aa75ed71a1/language-en>

⁷⁶ An older person is defined by the United Nations as a person who is over 60 years of age.

⁷⁷ If proposals concern drug addiction, they are encouraged to liaise with the EU Drugs Agency.

decision-making, and/or disability inclusive and accessible community-based services - medical, technological, digital or other supportive initiatives- ensuring prevention of isolation or segregation and supporting deinstitutionalisation. Special attention is to be paid to children and young people transitioning to adulthood and older persons to facilitate they remain living at their homes⁷⁸.

- Innovative solutions, care models and strategies for high quality person-centred, accessible and targeted social and healthcare services to prevent barriers and to support independent living, including if possible, self-care to empower persons with disabilities, as well as different choices of care across the life-course. For many persons with disabilities, the lack of support and care services and insufficient support for families and unavailability of personal assistance undermines their independence and inclusion in the community.

Data collection is essential to understand the living situation of persons with disabilities and remains a challenge to collect data disaggregated per type of disability, sex, and age. In addition, data collected often lacks comparability as it follows different definitions in each Member State and Associated Country. Thus, applicants are encouraged to ensure harmonised data collection by using Eurostat standards and existing international sets of questions in their areas of research.

Persons with disabilities should be involved in the research through their representative organisations as actors in the research process. Research can also involve their families, friends, colleagues, supporters and carers and other service providers. Policymakers and public authorities, social services, and civil society organisations, could also be considered.

The relevant European research infrastructures⁷⁹ in the area of health may be exploited for available digital tools and services for dataset creation, standardisation, data discovery, secure access, management, visualization, harmonization, analysis and other functions as appropriate.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, organisations as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

Projects are also encouraged to explore potential complementarities with projects funded under topic HORIZON-CL2-2025-01-TRANSFO-09: “Good practices for increased autonomy of persons with disabilities, including physical, mental, intellectual and sensory disabilities” and topic HORIZON-HLTH-2025-03-STAYHLTH-01-two-stage: “Improving the quality of life of persons with intellectual disabilities and their families”.

⁷⁸ https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=OJ:C_202407188

⁷⁹ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

Applicants envisaging to include clinical studies⁸⁰ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

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⁸⁰ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

Destination - Living and working in a health-promoting environment

Topics under this destination are directed towards the Key Strategic Orientation 1 “*The Green transition*” and Key Strategic Orientation 3 “*A more resilient, competitive, inclusive, and democratic Europe*” of Horizon Europe’s strategic plan 2025-2027⁸¹.

Research and innovation supported under this destination should contribute to the following expected impact, set out in the strategic plan impact summary for the Health Cluster: “*people's living and working environments are health-promoting and sustainable thanks to a better understanding of the environmental, occupational, social, sex and gender-related, and economic determinants of health*”.

The environment we live and work in is a major determinant of our health and wellbeing and climate change acts as a risk multiplier, exacerbating the health effects of environmental stressors, increasing the incidence of non-communicable diseases, mental health conditions, and infectious diseases, particularly for populations in a vulnerable situation. The climatic crisis is a health crisis with impacts at the global level. Across Europe, the fastest-warming continent, heat and floods have caused devastating human and economic impact in recent years. In 2025, the Commission published a Strategic Research and Innovation Agenda on Health and Climate Change⁸², providing a forward-looking overview of the current and emerging research needs and gaps in the field. This agenda informs the focus and objectives of this destination, aligning with the Commission's Political Guidelines for 2024-2029⁸³, which emphasise the need to step up work on preventive health, climate resilience, adaptation, preparedness, and the green transition, while promoting circularity.

In this Work Programme part, Destination “*Living and working in a health-promoting environment*” focuses on understanding and addressing the impacts of climate change on human health, increasing climate adaptation and resilience and reducing the health sector's contribution to climate change. The results will support the EU Strategy on Adaptation to Climate change, the European Climate Adaptation Plan (thematic window on health) and the European Climate Risk Assessment by enhancing understanding of health risks and informing prevention, adaptation, and mitigation actions for populations and healthcare systems. Moreover, this destination aims to identify and amplify the co-benefits of climate action for health outcomes. This integrated approach recognises that climate mitigation measures can simultaneously deliver significant health benefits, creating positive feedback loops between climate protection and public health. Strong collaborations across sectors and with other Horizon Europe Clusters dealing with issues such as agriculture, fisheries and aquaculture, food, environment, climate, biodiversity, mobility, security, urban planning, social inclusion and gender will be needed to ensure that maximal societal benefits are reached. In view of increasing the impact of EU investments under Horizon Europe, the Commission welcomes and supports cooperation between EU-funded projects to enable cross-fertilisation and create

⁸¹ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/strategic-plan_en

⁸² <https://op.europa.eu/en/publication-detail/-/publication/616cce9c-39e5-11f0-8a44-01aa75ed71a1>

⁸³ https://commission.europa.eu/about/commission-2024-2029_en

synergies. This could range from networking to joint activities such as the participation in joint workshops, the exchange of knowledge, development and adoption of best practices, or joint communication activities. Unless specified otherwise, all topics are open to international collaboration to address global climate and health challenges.

Expected impacts:

Proposals for topics under this destination should set out a credible pathway to contributing to living and working in a health-promoting environment, and more specifically to one or several of the following impacts:

- Policymakers and regulators are aware and well informed about climatic, environmental, socio-economic and occupational risk factors as well as health-promoting factors across society.
- Climatic, environmental, occupational, social, economic, and health policies and practices at the EU, national and regional level are sustainable and based on solid scientific evidence.
- The upstream determinants of health are known, understood and reduced.
- The health threats and burden and patient safety burdens resulting from exposure to climate drivers are lessened, so that the related number of deaths and illnesses is substantially reduced.
- Living and working environments in European cities and regions are healthier, more inclusive, safer, resilient and sustainable.
- The healthcare sector reduces its environmental footprint and transitions towards carbon neutrality.
- The adaptive capacity and resilience of populations and health systems in the EU to climate and environmental change-related to mental and physical health risks are strengthened.
- Citizens' health and wellbeing are protected and promoted, and premature deaths, diseases and inequalities related to climate related risks are prevented.
- Citizens understand better complex climate, environment and health issues, and effective measures to address them and support related policies and regulations.

Legal entities established in China are not eligible to participate in both Research and Innovation Actions (RIAs) and Innovation Actions (IAs) falling under this destination. For additional information please see “Restrictions on the participation of legal entities established in China” found in the Annex B of the General Annexes of this Work Programme.

Proposals are invited against the following topic(s):

HORIZON-HLTH-2026-01-ENVHLTH-01: Towards a better understanding and anticipation of the impacts of climate change on health

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 7.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 55.00 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: To ensure a balanced project portfolio covering the broad focus areas targeted in this topic⁸⁴, grants will be awarded (within available budget) to proposals not only in order of ranking but at least also to those proposals that are the highest ranked within different broad focus areas targeted, provided that the proposals attain all thresholds.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: In order to maximise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities (and

⁸⁴ Broad focus area i to iii, as given in the scope of this topic.

	in determining modalities for their implementation and the specific responsibilities of projects). These activities will be included in a dedicated work package, having sufficient budget allocated to it (around 2% of the total requested budget). Depending on the scope of proposals selected for funding, these activities may include: • Attendance of regular joint meetings (e.g. common kick-off meeting and annual meetings). • Periodic report of joint activities (delivered at each reporting period). • Common dissemination and communication activities (which may include, for example: a common dissemination and communication strategy, web portal and visual identity, brochure, newsletters). • Common Data Management Strategy and Common Policy Strategy (including joint policy briefs). • Thematic workshops/trainings on issues of common interest. • Working groups on topics of common interest (e.g. data management and exchange, communication and dissemination, science-policy link, scientific synergies). The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025)⁸⁵.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Citizens, patients, public authorities, social care services, healthcare practitioners and policymakers have a better understanding of the climatic health risks and determinants of

⁸⁵ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

disease and are better equipped to address health outcomes through enhanced and inclusive prevention, resilience, adaptation, preparedness and response, including better diagnosis and treatment.

- Governments, public health authorities, researchers and civil society organisations are supported to tackle societal challenges linked to the health impacts of climatic factors.
- Public authorities, organisations and the research community can rely on data collection and sharing according to FAIR⁸⁶ principles and leveraging of data availability and quality.
- Policymakers and public authorities develop evidence-based climate change and health policies and interventions that are nature positive, inclusive and responsive to diverse population needs.

Scope: The climate crisis poses an existential challenge to planetary and human health with larger effects on populations, groups and regions in a vulnerable situation. Climate change increases the incidence of non-communicable diseases and the prevalence of mental health conditions and facilitates the emergence and spread of infectious diseases. Climate change can act as a risk multiplier and exacerbates existing health conditions and vulnerabilities.

Applicants should explicitly state in their proposal which of the following broad focus areas is targeted and the proposed work should address only this specific broad focus area:

- i. Non-Communicable Diseases (NCDs) and/or individual safety (e.g. injuries or fatalities), excluding mental health aspects: proposals should explore evidence on the complex interactions between climate change (e.g. changes in the frequency and intensity of extreme weather events) and NCDs and individual safety, which often involve multiple climate exposure pathways and compound and cascading climatic events.
- ii. Mental health, considering interactions with brain health if relevant: in the broad focus area of mental health and psychosocial well-being, proposals should increase the evidence on the acute and long-term impacts of climate change and the understanding of new syndromes related to climate stress.
- iii. Infectious diseases, including vector-borne and non-vector-borne: proposals should increase the understanding of the factors driving climate-related burden from infectious diseases.

In general, proposals should develop approaches to prevent and reduce the impacts of climate factors in the studied health outcomes and increase population and workforce resilience. A One Health approach should also be applied where relevant.

More specifically, research actions under this topic should include several of the following activities, depending on the relevance of each group of activities to the broad focus area targeted in the proposal:

⁸⁶ See definition of FAIR data in the introduction to this Work Programme part.

- Increase the understanding of correlations, causal pathways and mechanistic effects between climate change and disease/health outcomes, developing unified and standard methodologies and metrics to assess short- and long-term positive and negative impacts of climate change with an adequate level of granularity. Consider individual and/or cascading climatic events and exposure patterns, and risks and drivers of vulnerability and inequality.
- Develop longitudinal studies to better ascertain differential effects of climatic stressors on health including multiple scales of impacts, ranging from the molecular level to population health outcomes. Consider variability across populations, generations and life phases, regions and occupations, and collect real-world exposure and health data in living and occupational settings, considering the use of emerging ecosystems such as the European Health Data Space (EHDS)⁸⁷ and the European Open Science Cloud (EOSC)⁸⁸.
- Study differential acute and long-term health impacts of climate (including a wide range of factors and cumulative effects) on vulnerable, sensitive or exposed population groups. Consider also differences in geographical vulnerabilities including, when relevant, geographical settings outside of urban areas, in overseas regions and in low- and middle-income countries (LMICs)⁸⁹. Understand the role of inequalities and societal vulnerability in determining climate-related health impacts and adaptive capacity.
- Advance the knowledge on the climate, ecological and environmental drivers of pathogen abundance, including mechanisms and determinants of distribution, life-cycle patterns, transmission, virulence and survival. Consider climate change drivers of disease severity. Study host/pathogen and vector/host interactions clarifying the role of secondary reservoir hosts such as sylvatic, wildlife and livestock in the maintenance of pathogen life cycle. Assess the efficacy, cost-effectiveness and impact of control measures.
- Explore the role of climate-driven human and wildlife mobility (e.g. bird migration patterns, human migration) in enhancing the global spread of pathogens and creating opportunities for their local establishment. Collect better field data and develop tools for disease modelling, risk and scenario projections that encourage interoperable data systems and cross border collaboration.
- Increase the availability, accessibility, quality and standardisation of diagnostic testing for early diagnosis of infections and determining immune responses and vaccine efficacy. Increase the capacity for pathogen subtyping, and genomic surveillance for early warning and investigations of climate-related outbreaks. Develop rapid, portable, and affordable standardised diagnostic tools that can withstand climate extremes.

⁸⁷ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

⁸⁸ https://research-and-innovation.ec.europa.eu/strategy/strategy-2020-2024/our-digital-future/open-science/european-open-science-cloud-eosc_en

⁸⁹ As defined by the World Bank, <https://www.worldbank.org>

- Increase the understanding of the factors that strengthen health resilience to climate change at the individual, local and societal levels. Investigate the role of individual mechanisms, community resilience and local solutions in mitigating the health impacts of climate change and related environmental degradation.

International cooperation, in particular with LMICs, is strongly encouraged.

In order to maximise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities. Guidance on the potential activities to be developed can be obtained by consulting the clusters of projects ongoing under the Environment, Climate and Health research portfolio⁹⁰.

Proposals should make sure that relevant activities, outcomes and outputs are shared with the European Climate and Health Observatory⁹¹ through the cluster that will be formed after the approval of the proposals. Actions' results should also contribute to future European Climate Risk Assessments. When relevant proposals should build on the outcomes of the projects that are part of the European Climate-Health Cluster⁹².

Proposals are encouraged to consider, where relevant, the data, expertise and services offered by European research infrastructures⁹³ in the environment, climate and health domain.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

Applicants envisaging to include clinical studies⁹⁴ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

HORIZON-HLTH-2026-01-ENVHLTH-04: Towards climate resilient, prepared and carbon neutral populations and healthcare systems

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 7.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and

⁹⁰ https://research-and-innovation.ec.europa.eu/research-area/health/environment-climate-and-health_en

⁹¹ <https://climate-adapt.eea.europa.eu/en/observatory>

⁹² <https://climate-health.eu>

⁹³ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

⁹⁴ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

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	selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 45.00 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: In order to maximise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities (and in determining modalities for their implementation and the specific responsibilities of projects). These activities will be included in a dedicated work package, having sufficient budget allocated to it (around 2% of the total requested budget). Depending on the scope of proposals selected for funding, these activities may include: • Attendance of regular joint meetings (e.g. common kick-off meeting and annual meetings). • Periodic report of joint activities (delivered at each reporting period). • Common dissemination and communication activities (which may include, for example: a common dissemination and communication strategy, web portal and visual identity, brochure, newsletters). • Common Data Management Strategy and Common Policy Strategy (including joint policy briefs).

	• Thematic workshops/trainings on issues of common interest. • Working groups on topics of common interest (e.g. data management and exchange, communication and dissemination, science-policy link, scientific synergies). The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ⁹⁵.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- The healthcare sector is supported with new technological developments and frameworks for greening, decarbonising and adapting to climate change, thus also contributing to protect biodiversity and ecosystem services.
- Governments, public health authorities, healthcare providers and practitioners, social care services and civil society have access to the best available evidence on the health costs and benefits (including co-benefits) of climate adaptation and mitigation actions and interventions.
- Policymakers and public authorities develop environment, climate change and health policies and interventions based on robust frameworks and incorporating innovative, inclusive and accessible solutions and technologies.
- Governments and public health authorities are supported in their adoption of robust frameworks and interventions to tackle societal challenges linked to the health impacts of climatic and environmental factors.
- Populations are empowered and equipped with knowledge, tools and resources to adopt health-protective behaviours and adapt to health-related climate risks.

⁹⁵ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

Scope: The health sector accounts for nearly 5% of global greenhouse gas (GHG) emissions and generates significant demands for energy and materials, as well as dangerous polluting streams. Proactive mitigation efforts in the health sector can significantly reduce GHG emissions and pollution, saving many lives and contributing to relieve pressures on biodiversity and ecosystem services. However, specific mechanisms for emission reductions in the health sector remain less defined compared to those in other sectors.

At the same time, the climate crisis subjects healthcare systems to unprecedented pressures (e.g. on infrastructure, workforce, overall systems) while simultaneously having to respond to increasing healthcare needs. To reduce pressure in healthcare systems and generally improve public health, it is crucial to design inclusive and accessible interventions that prevent the health impacts of climate change and related environmental degradation, increase resilience and preparedness of individuals and communities and foster the adoption of health-protective behaviours.

Research activities under this topic should generate evidence on the opportunities and health co-benefits of mitigation in the health sector as well as foster the development of low-carbon medical technologies and digital solutions for the sector. Proposals should also support the design of effective, scalable, cost-effective and transferable interventions and frameworks that can be applied across a wide range of healthcare settings and/or in population, community and societal contexts and involving, when relevant, public and patient engagement. Proposals can consider both living and working environments.

More specifically research actions under this topic should include some of the following activities:

- Develop and/or pilot effective, inclusive, accessible and impactful interventions to address the impact of climate change in healthcare systems and/or in health outcomes across populations, sectors and regions. These interventions should aim at reducing health vulnerability and building health resilience. Consider where relevant the involvement of local communities and/or end users in the development of these interventions.
- Develop methodologies and analytical tools to assess the effectiveness and cost-benefit of health-related climate change adaptation interventions.
- Generate evidence on the health co-benefits of climate change mitigation and propose frameworks to quantify the magnitude of their impacts.
- Develop harmonised frameworks, assessment metrics and reporting methods to evaluate alternative mitigation strategies and interventions, as well as harmonised methodologies to assess the cost-benefit of different mitigation measures.
- Explore and estimate the impact of preventive healthcare and lifestyle practices for mitigating the impacts of climate change in the health sector and increasing the resilience and preparedness of communities.

- Propose best practices to enhance the climate resilience of healthcare infrastructures, healthcare professionals and relevant supply chains and logistics.
- Explore and assess the role of primary care in increasing the preparedness of communities and reduce the health impacts of climate change.
- Develop low-carbon medical technologies (including medical devices) and digital solutions to reduce the emissions of GHG and pollutants (to air, water and soil) of healthcare practices and their supply chains. Health technology assessment activities to evaluate new or alternative low carbon medical solutions may be included where appropriate.

Funded projects under this topic should consider the scalability and transferability of the developed solutions to ensure that any knowledge, frameworks, methodologies, pilots, etc., developed are actionable and applicable across different healthcare settings and community contexts. Proposals should also consider the use of implementation science approaches to support the relevance and broad applicability of the research outcomes. Proposals should take into consideration the broader socio-economic challenges faced by healthcare systems (e.g. funding challenges, workforce shortages, population ageing and increase of chronic diseases). Additionally, solutions and interventions proposed under this topic should consider the Do No Significant Harm principle.

In order to maximise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities. Guidance on the potential activities to be developed can be obtained by consulting the clusters of projects ongoing under the Environment, Climate and Health research portfolio⁹⁶.

International cooperation is encouraged.

Proposals should make sure that relevant activities, outcomes and outputs are shared with the European Climate and Health Observatory⁹⁷ through the cluster that will be formed after the approval of the proposals. When relevant proposals should build on the outcomes of the projects that are part of the European Climate-Health Cluster⁹⁸.

Proposals are encouraged to consider, where relevant, the data, expertise and services offered by European research infrastructures⁹⁹ in the environment and health domain.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

⁹⁶ https://research-and-innovation.ec.europa.eu/research-area/health/environment-climate-and-health_en

⁹⁷ <https://climate-adapt.eea.europa.eu/en/observatory>

⁹⁸ <https://climate-health.eu>

⁹⁹ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

Applicants envisaging to include clinical studies¹⁰⁰ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

HORIZON-HLTH-2026-01-ENVHLTH-05: Support for a multilateral initiative on climate change and health research

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 3.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 3.00 million.
<i>Type of Action</i>	Coordination and Support Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, legal entities established in the United States of America may exceptionally participate as a beneficiary or affiliated entity, and are eligible to receive Union funding. Coordinators of projects must be legal entities established in an EU Member State or Associated Country. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for

¹⁰⁰ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ¹⁰¹ .
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- An international multilateral initiative is established to facilitate coordination and synergy between different research and innovation funding organisations tackling climate change and health issues with adequate support of a secretariat that works towards the definition of a governance model and forward-looking plan for the implementation of this initiative.
- The international community of research funders working in the climate-health nexus is well connected and supported to improve coordination of activities and alignment of priorities and increase the collective impact of funding streams.
- The research community, civil society groups and policymakers working in the climate-health nexus are well informed about the initiative’s activities and benefit from the knowledge, tools and opportunities created during its implementation.

Scope: Climate-related health challenges are global and complex in nature, which calls for coordinated action bringing together different research disciplines, policy sectors, perspectives and approaches. This requires seamless communication and synergies between different Research and Innovation (R&I) funding instruments.

R&I is key to increasing our understanding of existing and emerging climate-related vulnerabilities experienced by populations and health systems alike, as well as to supporting the development and implementation of effective and sustainable interventions for prevention, adaptation and preparedness against climate-related health threats. They also play a crucial role in supporting the health sector’s transition towards decarbonisation and long-term sustainability while ensuring that quality of care is maintained or improved.

R&I funding schemes and programmes will structure and accelerate the health response needed to meet the severity of the climate crisis in the decades to come. Achieving this requires building on a truly collaborative and impactful network of key players that optimally and efficiently structure and align their research programmes and funding streams.

The Commission, in collaboration with other funders and international organisations, will work on the development of a collaborative interface that brings together research funders

¹⁰¹ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

from around the world working at the intersection of health and climate. This platform will establish opportunities for participating organisations to discuss challenges and priorities and share best practices, research strategies and implementation plans. The platform will also serve to identify areas of common interest for collaboration and to build a shared vision to support health and climate research globally.

Proposals under this topic should focus on the provision of administrative and technical support to the successful establishment and implementation of a multilateral initiative of research and innovation funders working in climate change and health.

More specifically, proposals are expected to focus on all the following activities:

- Build on the preparatory work done and further support exchanges between relevant parties to delineate the operational structure and governance model of the multilateral initiative, in close collaboration with the European Commission.
- Facilitate and support the drafting of the workplan for the first period of activities of the multilateral initiative.
- Provide administrative and organisational support to the board of the multilateral initiative, in close collaboration with the chairs of the initiative.
- Provide scientific and technical support in aspects relevant to the development and implementation of the multilateral initiative.
- Facilitate communication between members of the initiative as well as with external stakeholders and organisations, and support the organisation of necessary meetings and the preparation of supporting material.
- Support external communication activities such as the creation of a website, newsletters, social media outreach and any other relevant communication and dissemination materials to promote the multilateral initiative.

HORIZON-HLTH-2027-01-ENVHLTH-02: Integrating climate-related exposures into the human exposome and characterising its changes in response to climate change

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 10.00 and 11.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 45.00 million.
<i>Type of Action</i>	Research and Innovation Actions

<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Beneficiaries will be subject to the following additional dissemination obligations: Proposals should ensure that chemical monitoring and human biomonitoring data are shared in the Common Data Platform for Chemicals and its services such as the Information Platform for Chemical Monitoring - IPCHEM¹⁰² (through involvement with the European Commission's Joint Research Centre - JRC) and the environmental sustainability database. In order to maximise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities (and in determining modalities for their implementation and the specific responsibilities of projects). These activities will be included in a dedicated work package, having sufficient budget allocated to it (around 2% of the total requested budget). Depending on the scope of proposals selected for funding, these activities may include: • Attendance of regular joint meetings (e.g. common kick-off meeting and annual meetings). • Periodic report of joint activities (delivered at each reporting period).

¹⁰²

<https://ipchem.jrc.ec.europa.eu>

	• Common dissemination and communication activities (which may include, for example: a common dissemination and communication strategy, web portal and visual identity, brochure, newsletters). • Common Data Management Strategy and Common Policy Strategy (including joint policy briefs). • Thematic workshops/trainings on issues of common interest. • Working groups on topics of common interest (e.g. data management and exchange, communication and dissemination, science-policy link, scientific synergies). The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Researchers, policymakers, healthcare practitioners and the public have a more comprehensive understanding of the human exposome and the interactions between climatic, environmental and socio-behavioural factors, supported by FAIR¹⁰³ data linking these exposures to disease and health outcomes.
- Researchers, governments, policymakers, social care services and healthcare practitioners have improved knowledge on the links between the climatic, social, lifestyle and environmental factors of the exposome and global health burden, supporting their efforts to adopt the exposome approach to identify and address relevant health impacts.
- The public has access to the latest information on the influence of global environmental exposures on health, enabling the adoption of health-promoting, climate-resilient and nature-positive behaviours.

Scope: The exposome is the totality of exposures (and their interactions) experienced by an individual throughout their lifetime, including chemical, physical, biological, nutritional and psychosocial factors, from conception onwards. Many of these factors originate in the environment, including climate-related exposures such as extreme heat, heightened air pollution or drought. Climate change may amplify or interact synergistically with other better-established exposures, dynamically altering the human exposome and its health implications. Despite this, climate factors remain underrepresented in large-scale human exposome studies.

¹⁰³ See definition of FAIR data in the introduction to this Work Programme part.

Research activities under this topic should strengthen the use of the exposome approach to study global exposures and generate evidence on their health implications. Proposals should focus on integrating climate-related factors into exposome research and understanding how the exposome changes in response to direct and indirect climate exposures. Moreover, research activities should be multiscale and multidisciplinary and account for the complexity and multifactorial nature of health determinants and the most pressing unmet medical needs in relation to environmental degradation and disrupted ecosystems. Proposals should include climate-relevant social determinants of health as part of their proposed activities.

More specifically, research actions under this topic should include all the following activities:

- Incorporate multiple climate exposures into exposomics studies and provide insights on their influence on disease burden, through interactions with other exposome factors.
- Predict, identify and monitor changes in the exposome (including environmental, social and occupational exposures) resulting from climate-related pressures and study their health implications to identify emerging health risks and potential benefits of climate change.
- Advance data generation, analysis, integration and interpretation in human exposomics, developing methodologies and integrating novel approaches (e.g. AI technologies and machine learning) for advanced data analytics, including for Real-World Data (RWD)¹⁰⁴.

In addition, research actions should include several of the following targeted activities:

- Establish and investigate the biological pathways and mechanisms by which the exposome drives health impacts, jointly considering climate-related and other exposures. Build upon (when relevant) and study existing and/or newly generated longitudinal cohorts that combine individual exposome data with the corresponding medical, omics and biological data.
- Identify exposome-relevant indicators and biomarkers for exposome-related health risks and potential benefits using comprehensive exposome studies that combine climate, environmental, behavioural and social exposures. Account for disparities in individual trajectories and exposure patterns where relevant.
- Report on health-relevant exposome findings using, where possible, standardised metrics to ensure harmonised reporting of exposome-driven disease burden across regions and sectors. Build on existing exposome toolboxes and increase their robustness and coverage by integrating climate related exposures.
- Study the role of socioeconomic (e.g. income, energy poverty, occupation), demographic (e.g. gender, racial or ethnic origin¹⁰⁵, age) and behavioural (e.g. public trust, risk

¹⁰⁴ EMA definition: “Real-World Data are routinely collected data relating to patient health status or the delivery of healthcare from a variety of sources other than traditional clinical trials (e.g. claims databases, hospital data, electronic health records, registries, mhealth data, etc.)”.

perception) factors in determining patterns of exposure, using the exposome approach to generate knowledge on intersectional vulnerability and resilience to exposome-driven (including climate-driven) health impacts. Identify disproportionately affected populations and develop interventions to reduce disparities.

When handling vulnerability data and indicators, sex-, gender-, racial or ethnic origin¹⁰⁶-disaggregated data should be collected and analysed, incorporating intersectional factors where feasible and relevant.

International cooperation is encouraged, in particular with regions that are under-represented in human exposome research.

Projects should leverage the knowledge, data and tools already generated under past initiatives such as EHEN¹⁰⁷ and ongoing initiatives such as IHEN¹⁰⁸, ICOS ERIC¹⁰⁹ and EIRENE RI¹¹⁰.

Proposals are encouraged to consider, where relevant, the data, expertise and services offered by European research infrastructures¹¹¹ in the environment, climate and health domain. Projects should make the tools developed as part of their research available on the IHEN Exposome Toolbox¹¹² and upload their data sets in the IHEN Data Catalogue¹¹³.

In order to maximise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities. Guidance on the potential activities to be developed can be obtained by consulting the ongoing clusters of projects under the Environment, Climate and Health research portfolio¹¹⁴.

Proposals should make sure that relevant activities, outcomes and outputs are shared with the European Climate and Health Observatory¹¹⁵ through the cluster that will be formed after the approval of the proposals.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

¹⁰⁵ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

¹⁰⁶ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

¹⁰⁷ <https://www.humanexposome.eu>

¹⁰⁸ <https://humanexposome.net>

¹⁰⁹ <https://www.icos-cp.eu/about/organisation-governance/icos-eric>

¹¹⁰ <https://eirene.eu>

¹¹¹ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

¹¹² <https://bio.tools/t?domain=exposome>

¹¹³ <https://data-catalogue.molgeniscloud.org/catalogue/catalogue>

¹¹⁴ https://research-and-innovation.ec.europa.eu/research-area/health/environment-climate-and-health_en

¹¹⁵ <https://climate-adapt.eea.europa.eu/en/observatory>

Applicants envisaging to include clinical studies¹¹⁶ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

HORIZON-HLTH-2027-01-ENVHLTH-MISSCLIMA-03: Tools and technologies to support health adaptation to climate change

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 4.00 and 5.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 20.00 million.
<i>Type of Action</i>	Pre-commercial Procurement
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The specific conditions for actions with PCP/PPI procurements in section H of the General Annexes apply to grants funded under this topic.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Beneficiaries must ensure that the subcontracted work is performed in the EU and Associated Countries - unless otherwise approved by the granting authority. Grants award under this topic will have to submit the following deliverable: To stimulate dialogue with the supply side, procurers are required to organise an open market consultation before launching the procurement and deliver a report on the outcomes of this consultation. Beneficiaries will be subject to the following additional dissemination

¹¹⁶ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	obligations: Procurers are required to promote the call for tenders widely across the EU and Associated Countries to potentially interested suppliers. Beneficiaries may provide financial support to third parties to ensure the deployment and impact of the project outcomes. The support to third parties can only be provided in the form of grants. The maximum amount to be granted to each third party is EUR 60 000. The specific conditions are described in General Annex H. PCP procurement costs are eligible. The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment” and the EU Mission on Adaptation to Climate Change. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Populations, public authorities and healthcare systems benefit from innovative solutions and technologies to increase surveillance and prevention and reduce climatic and environmental health risks.
- Policymakers and public authorities develop and implement environment, climate change and health policies and interventions supported by nearly fit-for-use solutions that can be further upscaled and deployed.

Scope: Enhancing the adaptive capacity and resilience of healthcare systems and communities is crucial to prevent and reduce the health impacts of climate change. However, many of the urgently needed technologies, tools, systems and solutions are still at an early developmental stage, relying on further support for development and testing. Proposals under this topic are expected to close this gap and build on innovations being developed in the field, supported through, among others, EU Research and Innovation (R&I) funding. In this context, Pre-commercial Procurement (PCP) projects can drive innovation and speed up the development of technologies for health adaptation to climate change by supporting the research and development of solutions to increase the resilience and preparedness of healthcare systems, communities and individuals against climate change. By focusing on early-stage solutions, PCP fosters collaboration between public sector buyers (e.g. public authorities, local authorities, health organisations) and private developers to create climate adaptation technologies, systems and solutions in the context of human health. These solutions will accelerate the transition to more climate-resilient healthcare systems and societies.

PCP actions target consortia of procurers with similar needs that want to jointly procure the development of innovative solutions for supporting adaptation efforts. This topic does not

provide direct funding to developers, industry or research organisations to perform Research and Development (R&D). They will be able to respond to the call for tenders launched by consortia of procurers funded under this topic.

Proposals under this topic should support the development of innovative solutions, tools and models to enhance surveillance, prediction, prevention, risk management and diagnosis (e.g. testing), supporting the adaptation, resilience, and preparedness of healthcare systems and populations to climatic and climate-related environmental health risks.

More specifically proposals can support any of the areas listed below:

- Geospatial technologies and decision-support frameworks that help local authorities and healthcare providers track at “high resolution” and better manage direct and indirect health risks related to climate change.
- Real-time risk surveillance and early-warning technologies and monitoring tools that provide critical information for timely decision-making and responses related to the health risks of climate change.
- Technologies and solutions that facilitate the transition to climate-resilient healthcare facilities and services. Activities targeting the general infrastructure (e.g. ventilation, construction or refurbishment) are out of scope.
- Technologies, tools, procedures and solutions for health risk management, prevention and resilience, enhancing strategies and interventions for health adaptation to climate change in communities and occupational settings.
- Innovative tools reducing risk and exposure to climate related environmental factors that exacerbate health risks.

This topic considers tools and technologies that could be developed and tested to support adaptation at both the community and healthcare system levels¹¹⁷. This approach would comprehensively address the needs of health authorities and those of local authorities and public organisations involved in risk management. Consortium composition could include diverse stakeholders such as hospitals, primary healthcare providers, domestic care services, municipalities, civil protection entities and government agencies. The focus can extend beyond climate variables to include other related environmental and ecological factors (for example air pollution) that interact with climate change and impact public health.

Proposals are encouraged to consider, where relevant, the data, expertise and services offered by European research infrastructures¹¹⁸ in the environment, climate and health domain.

¹¹⁷ For digital technologies concerned, appropriate measures for the security of the communications between the intended parties should be considered, in particular based on the use of post-quantum cryptography.

¹¹⁸ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

Continuous dialogue between demand and supply side is required for the success of PCPs, therefore the effective involvement of end users should be considered in the proposal.

Involvement of procurement decision makers is needed to ensure that end solution(s) are adopted by healthcare systems and/or local authorities and public organisations increasing the societal impact of the related research activities. Therefore, procurers should declare in the proposal their interest to purchase at least one solution resulting from the PCP in case the PCP delivers successful solutions and indicate whether they will i) procure the solution(s) as part of the PCP or ii) in a separate follow-up procurement after the PCP. In the first case, procurers can implement the project as a fast-track PCP (see section H of the General Annexes of this Work Programme for further details) and foresee the budget to purchase at least one solution during the PCP. In the second case, the procurers should include in the proposal a deliverable that prepares the follow-up procurement to purchase successful solution(s) after the PCP.

This topic is co-financed by the EU Mission on Adaptation to climate change¹¹⁹ and supports the follow up to the 2023 Communication on the Missions¹²⁰. Projects are encouraged to channel their activities through the Mission Implementation Platform¹²¹ and the Mission's Community of Practice¹²².

¹¹⁹ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/eu-missions-horizon-europe/adaptation-climate-change_en

¹²⁰ <https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX%3A52023DC0457&qid=1693304388860>

¹²¹ Initially established by MIP4Adapt (<https://climate-adapt.eea.europa.eu/en/mission/the-mission/about-mip4adapt>, <https://fedarene.org/project/mip4adapt>) and extended under the contract CINEA/2025/OP/0014.

¹²² <https://climate-adapt.eea.europa.eu/en/mission/community-of-practice>

Destination - Tackling diseases and reducing disease burden

Topics under this destination are directed towards the Key Strategic Orientation 3 “*A more resilient, competitive, inclusive, and democratic Europe*” of Horizon Europe’s strategic plan 2025-2027¹²³.

Research and Innovation supported under this destination should contribute to the following expected impact, set out in the strategic plan impact summary for the Health Cluster: “*healthcare providers improve their ability to tackle and manage diseases (infectious diseases, including poverty-related and neglected diseases, non-communicable and rare diseases) thereby reducing the disease burden on patients and enabling healthcare systems to perform more effectively. It can be achieved through better understanding, prevention, diagnostics, treatment, management, and cure of diseases and their co- and multi-morbidities, more effective and innovative health technologies and medical countermeasures, better ability and preparedness to manage pandemic and/or epidemic outbreaks, and improved patient safety*”.

Communicable and non-communicable diseases pose a significant health, societal, and economic threat worldwide, causing premature deaths and disabilities. Despite being largely preventable, only 6% of healthcare budgets are spent on prevention¹²⁴. To address this, there is an urgent need to develop new public health interventions, preventive, diagnostic, and therapeutic approaches, alternatives to antimicrobials, as well as to improve existing preparedness and response strategies to create tangible impacts, considering sex/gender-related issues. To address these challenges, Research and Innovation will require international cooperation to leverage global expertise, access world-class research infrastructures and invest in priority needs, aligning with other funders of international cooperation in health Research and Innovation. The continuation of international partnerships and cooperation with international organisations is particularly needed to combat infectious diseases and respond to public health needs, including rare diseases and the global burden of non-communicable diseases.

In this Work Programme part, Destination “*Tackling diseases and reducing disease burden*” will focus on major societal challenges linked to the Commission's Political Guidelines for 2024-2029¹²⁵, such as the fight against non-communicable and communicable diseases, mental health, preparedness and response to and surveillance of health threats and epidemics, reduction and treatment, of Antimicrobial-Resistant (AMR) infections, coherent also with wider EU initiatives such as the European Medical Countermeasures Strategy¹²⁶ and the

¹²³ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/strategic-plan_en

¹²⁴ Preventive healthcare expenditure as a share of the current expenditure on healthcare: [https://ec.europa.eu/eurostat/statistics-explained/index.php?title=File:Preventive healthcare expenditure as a share of current expenditure on healthcare, 2021 \(%25\) HCE2024.png](https://ec.europa.eu/eurostat/statistics-explained/index.php?title=File:Preventive_healthcare_expenditure_as_a_share_of_current_expenditure_on_healthcare,_2021_(%25)_HCE2024.png)

¹²⁵ https://commission.europa.eu/about/commission-2024-2029_en

¹²⁶ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

forthcoming Critical Medicines Act¹²⁷. In particular, the topics under this destination will support activities aiming at: i) new treatment and disease management options to reduce burden on non-communicable diseases and long-term conditions after post-bacterial and post-viral infections; ii) improve and protect mental health of children and young adults; iii) new prevention and treatment options for infectious diseases with epidemic potential; iv) innovative therapies for AMR critical pathogens; and v) support to second phases of the co-funded European Partnership on Rare Diseases¹²⁸ and the co-funded European Partnership for Pandemic Preparedness¹²⁹.

To increase the impact of EU investments under Horizon Europe, the Commission encourages cooperation between EU-funded projects to enable cross-fertilisation and other synergies. This could range from networking to joint activities, such as participating in joint workshops, exchanging knowledge, developing and adopting best practices, or undertaking joint communication activities. Opportunities for potential synergies exist between projects funded under the same topic, as well as between projects funded under different topics, Clusters, or Pillars of Horizon Europe. For example, synergies could be sought with projects funded under the European health research infrastructures (Pillar I of Horizon Europe), the EIC¹³⁰ strategic challenges on health (Pillar III of Horizon Europe), or with projects on themes that cut across the Clusters under Pillar II of Horizon Europe, such as health security/emergencies under Cluster “Civil Security for Society”, Artificial Intelligence (AI)-based tools and technologies under Cluster “Digital, Industry and Space”, or antimicrobial resistance under Cluster “Food, Bioeconomy, Natural Resources, Agriculture and Environment”.

The Commission aims to foster synergies between Horizon Europe and other EU programmes. To this end, applicants are encouraged to explore the funding opportunities available through the EU4Health Programme (2021-2027)¹³¹, the EU's public health programme, as a means of capitalising on potential collaborations and maximising impact.

Expected impacts:

Proposals for topics under this destination should set out a credible pathway to contributing to tackling diseases and reducing disease burden, and more specifically to several of the following impacts:

- Disease burden in the EU and worldwide is reduced through effective disease management, including through the development and integration of innovative preventive, diagnostic and therapeutic approaches, digital and other people-centred solutions for healthcare.
- Premature mortality from non-communicable diseases is reduced by one third (by 2030), mental health and wellbeing are promoted, and the targets of the World Health

¹²⁷ https://health.ec.europa.eu/publications/proposal-critical-medicines-act_en

¹²⁸ <https://cordis.europa.eu/project/id/101156595>, <https://erdera.org>

¹²⁹ <https://cordis.europa.eu/project/id/101226682>, <https://beready4pandemics.eu>

¹³⁰ <https://eic.ec.europa.eu>

¹³¹ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

Organization (WHO) Global Action Plan for the Prevention and Control of non-communicable diseases^{132, 133} are attained, with an immediate impact on the related disease burden (Disability-Adjusted Life Years - DALYs)¹³⁴.

- Healthcare systems benefit from strengthened Research and Innovation expertise, human capacities and know-how for combatting communicable and non-communicable diseases, including through international cooperation.
- Citizens benefit from reduced (cross-border) health threat of epidemics and AMR pathogens, in the EU and worldwide^{135, 136, 137}.
- Patients and citizens are knowledgeable of disease threats, involved and empowered to make and shape decisions for their health, and better adhere to knowledge-based disease management strategies and policies (especially for controlling outbreaks and emergencies).

Legal entities established in China are not eligible to participate in both Research and Innovation Actions (RIAs) and Innovation Actions (IAs) falling under this destination. For additional information please see “Restrictions on the participation of legal entities established in China” found in the Annex B of the General Annexes of this Work Programme.

The protection of European communication networks has been identified as an important security interest of the Union and its Member States. Entities that are assessed as high-risk suppliers¹³⁸ of mobile network communication equipment (and any entities they own or control) are not eligible to participate as beneficiaries, affiliated entities and associated partners to topics identified as “subject to restrictions for the protection of European communication networks”. Please refer to the Annex B of the General Annexes of this Work Programme for further details.

Proposals are invited against the following topic(s):

¹³² <https://www.who.int/publications/i/item/9789241506236>

¹³³ <https://www.who.int/publications/m/item/implementation-roadmap-2023-2030-for-the-who-global-action-plan-for-the-prevention-and-control-of-ncds-2023-2030>

¹³⁴ Disability-adjusted life year (DALY) is a quantitative indicator of overall disease burden, expressed as the number of years lost due to ill-health, disability or early death.

¹³⁵ WHO global action plan on antimicrobial resistance, 2015

¹³⁶ EU One Health Action Plan against AMR, 2017

¹³⁷ <https://www.ema.europa.eu/en/news/one-health-joint-framework-action-published-five-eu-agencies>

¹³⁸ Entities assessed as “high-risk suppliers”, are currently set out in the second report on Member States’ progress in implementing the EU toolbox on 5G cybersecurity of 2023 (NIS Cooperation Group, Second report on Member States’ progress in implementing the EU Toolbox on 5G Cybersecurity, June 2023) and the related Communication on the implementation of the 5G cybersecurity toolbox of 2023 (Communication from the Commission: Implementation of the 5G cybersecurity Toolbox, Brussels, 15.6.2023 C(2023) 4049 final).

HORIZON-HLTH-2026-01-DISEASE-02: Innovative interventions to prevent the harmful effects of using digital technologies on the mental health of children and young adults

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 44.20 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination "Tackling diseases and reducing disease burden". To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Researchers and healthcare professionals have an improved understanding of the neuro-biological and cognitive/behavioural evidence base on the correlation and impact of digital technologies on mental health, including brain development.
- Policymakers and digital technology and content developers are provided with a robust evidence base on the impact (positive or negative) of digital technologies on mental health in children and young adults¹³⁹.
- Policymakers, digital technology developers, and educational institutions amongst others make use (e.g. developing guidelines) of the evidence base and widely implement the newly developed interventions aimed at promoting children and young adults' mental health while mitigating any negative impacts of digital technology use.
- Children, young adults, families, guardians, educators, and carers have access to the newly developed interventions designed to prevent harm and promote the positive use of digital technologies.
- Children and young adults are empowered and develop resilience, including digital literacy, enabling them to engage in a healthy and positive way with digital technologies.

Scope: Already before the COVID-19 pandemic, 1 in 6 people in the EU suffered from mental health issues. The economic costs of it are estimated at 4% of the Gross Domestic Product (GDP)¹⁴⁰ and since then these figures worsened¹⁴¹ in particular among vulnerable groups such as children and adolescents or those at risk of discrimination. Digital technologies have the potential to enhance mental health for instance by providing access to information, support networks and therapy services¹⁴². However, there are indications that the excessive or misguided use of digital technologies, particularly among children and young adults, can negatively affect mental health and exacerbate mental disorders. There is an urgent need for more robust data to foster a safer, responsible and healthier use of digital technologies among children and young adults, prioritising the protection of their mental health.

Therefore, proposals should aim at generating robust scientific evidence on the impact of digital technologies, as well as developing and testing context-specific digital interventions that promote the positive and responsible use of them to improve mental health, avoiding the development or exacerbation of mental disorders. These innovative digital interventions should leverage multi-source data (e.g. sleep patterns, heart rate, stress levels, screen-time analytics, social media use, biological data, clinical data), and could include the use of Artificial Intelligence (AI). When handling data and indicators, sex and gender identity-

¹³⁹ There is no universal definition of youth and young adults. For the purpose of this topic, we follow the WHO definition of young adult a person aged 15-24: <https://www.who.int/southeastasia/health-topics/adolescent-health>

¹⁴⁰ https://health.ec.europa.eu/system/files/2020-02/2018_healthatglance_rep_en_0.pdf

¹⁴¹ [https://www.europarl.europa.eu/RegData/etudes/BRIE/2023/751416/EPRS_BRI\(2023\)751416_EN.pdf](https://www.europarl.europa.eu/RegData/etudes/BRIE/2023/751416/EPRS_BRI(2023)751416_EN.pdf)

¹⁴² <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:52023DC0298>

disaggregated data should be collected and analysed, incorporating intersectional factors where feasible.

The applicants should address all the following aspects:

- Generate the neuro-biological and cognitive/behavioural evidence base on the correlation and impact of digital technologies on mental health, including brain development (both positive and negative).
- Develop and test innovative digital interventions aiming for example at: counteracting addictive design patterns (e.g. on social media and gaming platforms), gaining insights into risk patterns and enabling early risk detection (e.g. detecting early warning signs of mental disorders or digital addiction), redirecting users towards healthy use and positive engagement with digital technologies, and/or reducing exposure to harmful content.
- Assess the changes in behaviour in children and young adults of the newly developed interventions, aiming at fostering their resilience and promoting responsible use and healthy digital habits.

The topic is open to address any mental disorder¹⁴³ caused or aggravated by the use of digital technologies such as addiction, self-harm behaviour, increased anxiety or decreased self-esteem, sleeping-disorders, post-traumatic stress disorders.

Cohort studies and clinical studies are in the scope for this topic. Applicants envisaging to include longitudinal cohort studies are invited to indicate a sustainability plan on how those cohorts are maintained over an extended period beyond the end period of the project for a long-term follow-up. They should make use of existing cohorts data when available. Applicants are welcome to consider recruiting participants transnationally and from diverse settings in the clinical study design to ensure generalizability of findings. In addition, it should be detailed in the proposal how the proposed intervention(s) could be scaled-up and transferred to other settings. Applicants should also consider the inclusion of end-users in the codesign of the interventions, for example for the young age groups, this includes the involvement of families, carers, educators. Applicants should access and make best-use of already existing European Research Infrastructures relevant for brain-research (e.g. EBRAINS¹⁴⁴, Euro-BioImaging¹⁴⁵).

All projects funded under this topic should liaise with relevant European projects on mental health¹⁴⁶ and the future co-funded European Partnership for Brain Health¹⁴⁷. They are also encouraged to explore potential synergies with projects to be funded under the EU4Health

¹⁴³ ICD11, Chapter 6: <https://icd.who.int/browse/2025-01/mms/en#334423054>

¹⁴⁴ <https://www.ebrains.eu>

¹⁴⁵ <https://www.eurobioimaging.eu>

¹⁴⁶ Projects funded under topics HORIZON-HLTH-2024-STAYHLTH-01-02-two-stage: "Towards a holistic support to children and adolescents' health and care provisions in an increasingly digital society" and HORIZON-HLTH-2022-STAYHLTH-01-01-two-stage: "Boosting mental health in Europe in times of change".

¹⁴⁷ <https://www.brainhealth-partnership.eu>

Work Programme 2026 related to the harmful effects of using digital technologies on the mental health of children and young adults.

The participation of start-ups and/or micro, small and medium-sized enterprises (SMEs)¹⁴⁸ is encouraged with the aim to strengthen their scientific and technological basis and valorise their innovations and to advance commercial exploitation.

Proposals should adhere to the FAIR¹⁴⁹ data principles, adopt wherever relevant, data standards and data sharing/access good practices, and apply good practices for GDPR¹⁵⁰ compliant personal data protection.

The topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities. The support and involvement of citizens and civil society should be considered.

Applicants should provide details of their clinical studies¹⁵¹ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2026-01-DISEASE-03: Advancing research on the prevention, diagnosis, and management of post-infection long-term conditions

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 6.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 39.30 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the

¹⁴⁸ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

¹⁴⁹ See definition of FAIR data in the introduction to this Work Programme part.

¹⁵⁰ General Data Protection Regulation: https://commission.europa.eu/law/law-topic/data-protection_en, <https://gdpr-info.eu>

¹⁵¹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ¹⁵².

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- All players along the healthcare value chain have access to evidence-based treatment and management strategies for post-infection conditions and improve patient recovery and quality of life across diverse populations.
- Public health authorities and healthcare practitioners have access to effective prevention, diagnostic and treatment tools, ensuring better allocation of healthcare resources.
- Healthcare systems improve their efficiency and reduce long-term economic burdens by streamlining post-infectious disease care and addressing disparities in healthcare access.

¹⁵² This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

- Public health authorities have access to evidence-based information to integrate research findings into policy for improved public health preparedness and resilience, including training of healthcare staff and enhanced long-term disease management guidelines.

Scope: Microbial infections can lead to long-lasting consequences on patients' quality of life, leading to long-term conditions characterised by persistent inflammation, organ damage, and impaired functional capacity, which pose a growing public health and economic challenge. These conditions are insufficiently understood, underdiagnosed, and lack effective treatments. Advancing research into their prevention, treatment and management is essential to improving patient outcomes, reducing healthcare burdens, and strengthening workforce productivity.

The topic is open to long-term conditions resulting from infections by any type of microorganism (including viruses, bacteria, parasites, and fungi), which persist after the initial infection has been resolved. Research linked to cancer is excluded as it will be covered by the Cancer Mission.

Proposals should aim to develop innovative approaches for the prevention, diagnosis, and management of post-infection conditions. Proposals should address most of the following research areas:

- Identify protective and risk factors associated with the development of post-infection conditions to inform targeted prevention strategies, by integrating relevant information such as genetics, epigenetics, immune or inflammatory responses, and/or other relevant factors.
- Increase understanding of the pathophysiology of post-infection conditions (including inflammatory aspects) to identify biomarkers and develop clinically validated diagnostic approaches for early detection, disease progression and/or treatment optimisation.
- Develop and validate preventive and/or therapeutic interventions, including targeted pharmacological treatments, repurposing of existing drugs or precision medicine approaches, through early-stage clinical trials¹⁵³ that demonstrate clinical safety and efficacy.
- Identify effective supportive rehabilitation approaches, including physical therapy, cognitive interventions, and psychological support, to enhance patient recovery, mental health and quality of life and evaluate their effectiveness.
- Examine best practices for integrating post-infectious disease management into primary and specialised healthcare settings, improving coordination among healthcare professionals.

Specific attention should be given to sex and gender, as women often experience post-infectious diseases differently due to hormonal and other biological, as well as social factors, which can affect their diagnosis, treatment, and recovery. Moreover, age, disability, racial or

¹⁵³ For pharmacological interventions: phase 1 and phase 2 clinical trials.

ethnic origin¹⁵⁴, socio-economic, lifestyle and behavioural factors should also be considered. Particular emphasis should be placed on populations in a vulnerable situation and groups with pre-existing conditions to ensure equitable and inclusive healthcare solutions.

A multidisciplinary, cross-sectoral approach is encouraged, involving all relevant stakeholders (medical and non-medical), including patients, researchers, healthcare professionals, and policymakers.

Proposals should develop a harmonised approach to collection, storage, sharing and analysis of FAIR¹⁵⁵ data, leveraging existing European (research) infrastructures, including biobanks or cohorts' data¹⁵⁶ where relevant and contribute to emerging research infrastructures, established in the framework of the European Health Data Space (EHDS)¹⁵⁷ and the European Open Science Cloud (EOSC)¹⁵⁸.

Proposals should demonstrate complementarity with ongoing EU initiatives, including projects funded under relevant topics¹⁵⁹, and outline plans for collaboration where applicable, to maximise synergies and avoid duplication of research efforts.

All projects funded under this topic are expected to participate in networking and joint activities¹⁶⁰. They are also expected to engage early on with the European Medicines Agency (EMA) to ensure adequacy of the actions from a regulatory point of view. Where relevant, a Health Technology Assessment (HTA) should be conducted to evaluate the clinical, economic, and social implications of interventions.

If applicable, applicants are encouraged to incorporate artificial intelligence (AI) tools and advanced computational modelling/Virtual Human Twin (VHT)-powered tools to predict disease risk and progression, ensuring these tools are developed and tested for diverse populations to minimise bias. Hardware and software should be interoperable in line with internationally accepted standard.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of

¹⁵⁴ The use of the term 'racial or ethnic origin' does not imply an acceptance of theories that attempt to determine the existence of separate human races.

¹⁵⁵ See definition of FAIR data in the introduction to this Work Programme part.

¹⁵⁶ ORCHESTRA data portal: <https://orchestra-cohort.eu/data-portal>, Pathogens portal cohorts browser: <https://www.pathogensportal.org/cohorts>

¹⁵⁷ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

¹⁵⁸ https://research-and-innovation.ec.europa.eu/strategy/strategy-2020-2024/our-digital-future/open-science/european-open-science-cloud-eosc_en

¹⁵⁹ <https://ec.europa.eu/info/funding-tenders/opportunities/portal/screen/opportunities/topic-details/horizon-hlth-2021-corona-01-02>, <https://ec.europa.eu/info/funding-tenders/opportunities/portal/screen/opportunities/topic-details/horizon-hlth-2023-disease-03-07> and <https://ec.europa.eu/info/funding-tenders/opportunities/portal/screen/opportunities/topic-details/horizon-hlth-2025-01-disease-07>

¹⁶⁰ The details of these joint activities will be defined during the grant agreement preparation phase. Applicants should plan the necessary budget to cover those activities without the prerequisite to define concrete common actions at this stage.

relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

Participation of start-ups, micro, small and medium-sized enterprises (SMEs)¹⁶¹ is also encouraged to strengthen their scientific and technological foundations and enhance their innovation potential.

Applicants should provide details of their clinical studies¹⁶² in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2026-01-DISEASE-04: Development of novel vaccines for viral pathogens with epidemic potential

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 9.00 and 11.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 44.20 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply:

¹⁶¹ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

¹⁶² Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: To ensure a balanced project portfolio covering the viruses targeted in this topic¹⁶³, grants will be awarded (within available budget) to proposals not only in order of ranking but at least also to those proposals that are the highest ranked within different viruses targeted, provided that the proposals attain all thresholds.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental vaccines for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation, including better understanding of biological sex and social determinants that influence immune response, vaccine efficacy, safety, and uptake.
- Candidate vaccines are available for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope: Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of vaccine-based antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The availability of vaccines that can be adjusted to variants would provide a critical preparedness measure against future health threats, due to infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy¹⁶⁴.

¹⁶³ Virus i to viii, as given in the scope of this topic.

¹⁶⁴ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

Applicants should explicitly state in their proposal which of the following viruses is targeted and the proposed work should address only this specific virus. The proposed work should aim to advance the development of existing prophylactic and therapeutic vaccine candidates targeting exclusively one of the following viruses:

- i. Junin mammarenavirus
- ii. Lassa mammarenavirus
- iii. Andes virus,
- iv. Hantaan virus
- v. Sin Nombre virus
- vi. Hendra virus
- vii. Enterovirus D68
- viii. Venezuelan equine encephalitis virus

Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address all the following research areas:

- If necessary, finalisation of the in-vitro characterisation of existing vaccine candidates with regard to target specificity, epitope recognised, and their ability to impair or inactivate viral functions.
- In-vivo tests in at least one animal model or, if available in humanised immune system animal models, to demonstrate the protective function of the vaccine candidates deemed sufficient for moving to first clinical trials.
- If requested by regulators as pre-requisite for clinical studies, in-vivo tests in a non-human primate model.
- Production of batches of the most promising vaccine candidates according to the Good Manufacturing Practices (GMP)¹⁶⁵.
- First in human clinical safety studies demonstrating a clear regulatory pathway for market authorisation. Attention should be paid to critical biological and social factors such as sex, age, ethnicity and disability.

Participation of third countries where viruses addressed in the proposal are endemic or where outbreaks have occurred or are ongoing is encouraged.

¹⁶⁵ https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

The European Commission's Joint Research Centre (JRC) may contribute to the proposals selected for funding with work on strategic technologies for economic security and innovative industrial ecosystems, particularly activities on innovation in vitro biotechnologies.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)¹⁶⁶ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

All projects funded under this topic are expected to engage with regulatory bodies in a timely manner to ensure adequacy of the actions from a regulatory point of view.

Proposals should advance research by leveraging already existing and emerging state-of-the-art research infrastructures¹⁶⁷ such as those having contributed to the services developed under the ISIDORE project¹⁶⁸.

Applicants should provide details of their clinical studies¹⁶⁹ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2026-01-DISEASE-09: Multisectoral approach to tackle chronic non-communicable diseases: implementation research maximising collaboration and coordination with sectors and in settings beyond the healthcare system (GACD)

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 3.00 and 4.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 9.80 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding.

¹⁶⁶ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

¹⁶⁷ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

¹⁶⁸ <https://isidore-project.eu>

¹⁶⁹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ¹⁷⁰.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to some of the following expected outcomes:

- Researchers, policymakers, healthcare- and non-healthcare-related stakeholders and authorities in low- and middle-income countries (LMICs)¹⁷¹ and/or those in high-income countries (HICs) serving disadvantaged populations have access to improved insights and evidence on how to maximise collaboration and coordination with sectors and in settings beyond the healthcare system in the context of Non-Communicable Diseases (NCDs).
- Researchers, policymakers, healthcare- and non-healthcare-related stakeholders and authorities have an improved understanding how the proposed interventions draw on collaborative multisectoral engagement and utilise the different settings in which people are educated, work and live, to expand efforts to reduce risks, prevent, manage and control NCDs.

¹⁷⁰ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

¹⁷¹ As defined by the World Bank, <https://www.worldbank.org>

- Communities, relevant stakeholders from different sectors and authorities are fully engaged in implementing and taking up interventions that tackle the growing burden of NCDs through actions in sectors and settings outside the traditional health system and its facilities health-related outcomes, improve quality of life across the life course and extend healthy life expectancy.

Scope: The Commission is a member of the Global Alliance for Chronic Diseases (GACD)¹⁷². The GACD specifically addresses NCDs and supports implementation research¹⁷³ to improve health outcomes. This topic is launched in concertation with the other GACD members (international funding agencies) and aligned with the 11th GACD call.

Besides health-related determinants, the burden of NCDs is also driven by structural and social inequities, population ageing, the effects of globalisation on marketing and trade, diet and activity, commercial and economic determinants of health, rapid urbanisation and climate change, factors over which a conventional healthcare-oriented system has little sway. There is a need for a comprehensive approach, involving sectors outside of health, to meet the global targets that governments have agreed upon to protect people from chronic NCDs. Tackling chronic NCDs most effectively therefore requires engagement and coordinated policy development within and across many government departments, including education, workplace, environment, social systems, housing, transportation, agriculture, food industry and nutrition, leisure and culture.

The aim of this topic is to fund implementation research focused on strategies to tackle the growing burden of NCDs through actions in sectors and settings outside the traditional health system¹⁷⁴ and its facilities (with or without the involvement of the healthcare system) to attain equitable health-related outcomes or influence health determinants for people living in LMICs, and/or underserved populations in HICs.

Proposals can focus on more than one setting and/or include cross-sectoral approaches, involving both health and non-health settings to expand efforts to reduce risks, prevent, manage and control NCDs. Safety is a major concern in non-health settings, and proposals should ensure any risks and safety considerations are addressed.

The choice of intervention(s)¹⁷⁵ and provision of existing evidence of the intervention's effectiveness, cost-effectiveness, sustainability, scalability and potential for long-term health and other impacts should be justified (and in what context this evidence has been generated).

¹⁷² <https://www.gacd.org>

¹⁷³ https://iris.who.int/bitstream/handle/10665/91758/9789241506212_eng.pdf

¹⁷⁴ In this context, non-healthcare settings can include for instance: workplaces; schools, universities and other education venues (including pre-schools, nursery, etc.); faith-based communities, places of worship and traditional healers; recreation and sports clubs, fitness centres, swimming pools; prisons; communities (geographic and/or of identity) and families; community pharmacies; theatres, community spaces; retirement homes and care homes; homeless shelters; markets, malls, commercial settings; barbers, hairdressers and beauty salons; urban environments, parks, transportation (the list is not exhaustive).

¹⁷⁵ Research proposals might explore implementation, outcomes and impact of context relevant strategies to implement evidence-based interventions or initiatives including (though not limited to): i) Non-health

The majority of evidence-based interventions implemented outside of the health sector focus on prevention of NCDs: relatively few focus on strategies for management of these chronic conditions, and a limited number are implemented in LMIC contexts or underserved communities. Therefore, it may be important to undertake formative research as a part of the proposal to support readiness for implementation.

Applicants should explore the implementation of proposed intervention(s) for a selected study population(s) based in one or more LMICs, and/or underserved populations experiencing health disparities, including Indigenous populations, in HICs, considering the unique social, political, economic, and cultural context(s) in which the study will take place¹⁷⁶. Applicants should justify why any adaptation will not compromise the known effectiveness of the selected intervention(s).

Proposals should address all the following implementation research activities¹⁷⁷:

- Clearly describe the implementation research methodology, including the statistical design, and provide a rationale for the implementation strategy/ies to be explored (in light of the context), the community/population group(s) to benefit, the settings and sectors involved (and how these should be engaged), the current state of the art and how the proposal improves on this, and, if used, the theories, models and/or frameworks underpinning the research.
- Have an appropriate strategy for measuring implementation research outcomes and real-world effectiveness outcomes and indicators.
- Specifically address issues of equitable implementation to ensure interventions reach the populations that need them the most.

sector policy introduction, to tackle relevant social and/or structural determinants of NCDs; ii) Strategies to expand screening for NCDs and their risk factors in community, school, workplace, faith-based settings (e.g. Human Papillomavirus - HPV screening, blood pressure monitoring, blood sugar testing); ii) Partnered strategies to prevent NCDs in the community (e.g. educational campaigns, changes to school or work environments, promotion/delivery of healthy food choice and diet, opportunities for increased physical activity, strategies to support tobacco cessation and alcohol cessation); iv) Cost effective, patient centred treatment and management of NCDs in the community (e.g. mental health support, community medicine purchasing clubs, self-management groups); v) Non-health sector policy introduction e.g. environmental policy or practices (e.g. improvements to transport systems, public infrastructure) and the potential co-benefits on health; vi) Digital interventions e.g. for patient or care giver support, such as use of Artificial Intelligence for Patient support or to promote prevention messages on Chronic Disease Risk Factors).

¹⁷⁶ Focus on populations facing extreme vulnerabilities, such as individuals or communities living in informal settlements, post-disaster settings, or in situations of homelessness is encouraged (though not required).

¹⁷⁷ The following types of proposals are not in the scope of this topic: i) proposals with the primary aim of informing the development and/or selection of an intervention for a given context, where the implementation component will be explored in a future project (i.e. standalone feasibility projects); ii) epidemiological cohorts; iii) etiological work, mechanistic, or epidemiological research, unless an essential component of a focused study to develop implementation research approaches; iv) clinical trials, validation studies, or intervention efficacy studies for a new or established pharmacological agent or behavioural intervention.

- Engage an appropriately expert and skilled research team which can ensure a suitable multidisciplinary approach and that demonstrates equitable partnership and shared leadership between HIC-LMIC, and/or non-Indigenous-Indigenous members of the project team and external stakeholders through a clear governance strategy.
- Provide a stakeholder engagement strategy with evidence of support/engagement from key stakeholders for delivering the intervention and a pathway to sustain the proposed intervention (if proven effective) after the funding from the GACD grant ends.
- Provide opportunities for implementation research capacity building for early career researchers and team members from lower resourced environments, such as LMICs or disadvantaged communities.
- Ensure meaningful involvement of early career team members, including at least one early career member as a co-investigator.

The study population may include the general population, people with one or more existing NCDs, those currently without NCDs, or a combination of both. Applicants may propose implementation research focused on interventions that are implemented at the individual, family, community (e.g. work or school), population, and/or structural level. With regard to NCDs, applicants are encouraged to explore any chronic non-communicable condition (or combination of conditions), including mental health disorders, substance use disorders, autoimmune conditions, musculoskeletal conditions, neurological disorders and sleep disorders and/or any risk factor (or combination of risk factors). Additionally, whenever relevant, applicants are also encouraged to take a life course approach, adapting interventions for particular life stages with the goal of promoting life-long health.

Proposals should use an appropriate implementation research design and frameworks¹⁷⁸, cluster Randomised Control Trials (cRCTs), before and after studies, and additional implementation science classifications of study designs (e.g. hybrid designs¹⁷⁹), noting that applicants are not limited to any particular design.

Proposals are expected to generate evidence that is of direct relevance to policymakers, communities and practitioners. Proposed work should develop a strategy to include the relevant policymakers, local authorities, as well as other stakeholders such as community groups, or other individuals or organisations involved in the implementation of the intervention, with co-creation from the development of the proposal through to the knowledge translation phase. Project partners should be engaged from the beginning to contribute to the sustainability of the intervention after the end of project. Proposals should demonstrate sustainability of the strategy, beyond the lifespan of the project.

¹⁷⁸ Examples of frameworks include (this list is not exclusive): i) Consolidated Framework for Implementation Research (CFIR); ii) the context enhanced (RE-AIM) Reach, Effectiveness, Adoption, Implementation, Maintenance); iii) Practical Robust Implementation and Sustainability Model (PRISM) frameworks; iv) Framework for Developing and Evaluating Complex Interventions (MRC & NIHR).

¹⁷⁹ <https://pmc.ncbi.nlm.nih.gov/articles/PMC3731143> and <https://pmc.ncbi.nlm.nih.gov/articles/PMC6779135>

Poverty, discrimination based on sex, racial or ethnic origin¹⁸⁰, religion or belief, disability, age, and other inequities are directly associated with reduced potential for equitable access to quality care. Proposals should consider relevant determinants of health (e.g. social, structural, commercial, economic) and discuss their potential impact on the effective implementation of the intervention(s). If there is a focus on a particular population (e.g. gender, racial or ethnic origin¹⁸¹, etc.), then the reason for this should be justified.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

All projects funded under this topic are strongly encouraged to participate in networking and joint activities, including internationally, as appropriate. These activities could, for example, involve the participation in joint workshops, the Annual Scientific Meetings of the GACD, the exchange of knowledge, the development and adoption of best practices, or joint communication activities. Therefore, proposals are expected to include a budget for such activities and should consider covering the costs of any other potential joint activities without the prerequisite to detail concrete joint activities at this stage. The details of these joint activities will be defined during the grant agreement preparation phase.

Applicants should provide details of their clinical studies¹⁸² in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2026-01-DISEASE-11: Understanding of sex and/or gender-specific mechanisms of cardiovascular diseases: determinants, risk factors and pathways

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 6.00 and 7.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 39.30 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility</i>	The conditions are described in General Annex B. The following

¹⁸⁰ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

¹⁸¹ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

¹⁸² Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

<i>conditions</i>	exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ¹⁸³.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Researchers, developers of medical interventions, and healthcare professionals have a better understanding of biological sex and/or gender-specific health determinants, risk factors and pathways for cardiovascular diseases.
- Researchers, developers of medical interventions, and healthcare professionals have access and use sex and/or gender-specific or tailored risk models for better prevention, detection and diagnostic and treatment strategies.

¹⁸³ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

- Healthcare systems benefit from novel sex and/or gender-specific strategies for prevention, detection, diagnosis and treatment options, resulting in reduced burden of cardiovascular diseases.

Scope: Cardiovascular diseases (CVDs) are the leading cause of premature deaths in the EU and account for 32% of all deaths in 2021 (over 1.7 million deaths)¹⁸⁴.

Biological sex and gender play a specific role both in the incidence and the prevalence of certain diseases, including CVDs. Sex and gender disparities in CVDs are influenced by biological, behavioural, and sociocultural factors, affecting symptoms, prevalence, treatment, and outcomes. Hormonal influences, genetic predispositions, and/or physiological differences contribute to variations in how CVD presents and progresses in men and women. Risk factors such as diabetes, cholesterol, smoking, and age have different impacts across genders, highlighting the need for customised treatment strategies. Unique gender-specific conditions in women, such as menopause, pregnancy complications like preeclampsia and certain autoimmune diseases, also increase the risk for CVDs¹⁸⁵.

Mainstreaming a gender perspective into the research, prevention and control of CVDs is thus crucial to understanding and addressing the health risks and needs of women and men of all ages¹⁸⁶.

Although the significant progress has been done in investigating sex and/or gender-specific pathophysiological mechanisms of cardiovascular diseases, more research is needed to translate basic discoveries into the development of innovative prevention, detection, diagnosis, and treatment options.

Proposals should address most of the following aspects:

- Contribute to further the understanding on the structural, hormonal, and/or biological distinctions between sexes/genders to improve diagnostics and therapeutics for CVDs.
- Develop sex and/or gender-specific tailored risk models in a view of better prevention, detection and diagnostic, and treatment strategies.
- Identify and/or validate novel or existing sex and/or gender-specific health determinants, risk factors and pathways for cardiovascular disease(s) through the generation, integration and validation of data derived from relevant disciplines (e.g. molecular biology, behavioural science, nutrition, clinical, social and environmental epidemiology; exposure sciences; genetics and epigenetics, etc.).

¹⁸⁴ European Union takes action for the cardiovascular health of its 440 million people - EACH: <https://www.cardiovascular-alliance.eu/european-union-takes-action-for-the-cardiovascular-health-of-its-440-million-people>

¹⁸⁵ Gender Disparities in Cardiovascular Disease and Their Management: A Review - PMC: <https://pmc.ncbi.nlm.nih.gov/articles/PMC11148660>

¹⁸⁶ Political declaration of the 3rd High-Level Meeting of the General Assembly on the Prevention and Control of Non-Communicable Diseases : resolution adopted by the General Assembly: <https://digitallibrary.un.org/record/1648984?v=pdf>

- Make use of existing health data, including registries or cohorts, and/or assess the necessity to establish new ones, as well as, where relevant, exploit the knowledge gained from population-based biobanks. In case of the generation of new data, it should be managed in line with the FAIR¹⁸⁷ principles, when relevant.

Proposals are encouraged to consider, where relevant, the data, expertise and services offered by European research infrastructures¹⁸⁸ in the health domain.

The use and/or development of new technologies, including digital ones (e.g. (generative) Artificial Intelligence) that support research under this topic is encouraged.

Disease progression and overall health status at different life stages, as well as hormonal influences, genetic factors, etc. and psychosocial, socioeconomic, cultural and behavioural factors should be considered in the proposed research. Other intersecting factors such as racial or ethnic origin¹⁸⁹, often amplify existing inequalities in health access and outcomes. Proposals should, where relevant, consider these to design effective and inclusive interventions.

In the context of gender-specific research, this topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)¹⁹⁰ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

All projects funded under this topic are encouraged to participate in networking and joint activities, as appropriate. Proposals should include a budget for the attendance to regular joint meetings and may consider covering the costs of any other potential joint activities without the prerequisite to detail concrete joint activities at this stage. The details of these joint activities will be defined during the grant agreement preparation phase. All projects funded under this topic are also encouraged to explore complementarities and exploit potential synergies with the projects funded under topic HORIZON-CL6-2026-02-FARM2FORK-10: “Sustainable and healthy diets based on health status and socio-economic risk factors of ageing population”, once information on the funded projects is available.

All projects funded under this topic are encouraged to explore potential synergies with projects to be funded under the EU4Health Work Programme 2026 related to the gender and CVDs.

¹⁸⁷ See definition of FAIR data in the introduction to this Work Programme part.

¹⁸⁸ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

¹⁸⁹ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

¹⁹⁰ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

Applicants envisaging to include clinical studies¹⁹¹ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

HORIZON-HLTH-2026-02-DISEASE-12: European Partnership on Rare Diseases (ERDERA) (Phase 2)

Call: Partnerships in Health (2026/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 91.30 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 91.30 million.
<i>Type of Action</i>	Programme Co-fund Action
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: The proposal must be submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases”. This eligibility condition is without prejudice to the possibility to include additional partners. In recognition of the opening of the US National Institutes of Health’s programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. Because the US contribution will be considered for the calculation of the EU contribution to the partnership, the concerned consortium of research funders from eligible EU Members States and Associated Countries must expressly agree to this participation. The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following

¹⁹¹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	exceptions apply: The evaluation will take into account the existing context and the scope of the initial evaluation as relevant, and related obligations enshrined in the grant agreement. If the proposal is successful, the next stage of the procedure will be grant agreement amendment preparations. If the outcome of amendment preparations is an award decision, the coordinator of the consortium funded under topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases” will be invited to submit an amendment to the grant agreement, on behalf of the beneficiaries.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: This action is intended to be implemented in the form of an amendment of the grant agreement concluded pursuant to Article 24(2) of the Horizon Europe Regulation. For the additional activities covered by this action: • The funding rate is 50% of the eligible costs. This is justified by the pooling of proposers' in-kind contributions and in-house activities and by the nature of activities to be performed: in addition of joint calls, highly integrative activities (EU clinical trial preparedness, training, patients’ empowerment activities etc.) contributing to enhance the rare disease research and innovation ecosystem in the EU and Associated Countries, and beyond. • Beneficiaries may provide financial support to third parties (FSTP). The support to third parties can only be provided in the form of grants. As a co-funded European Partnership, providing financial support to third parties is a core activity of this action in order to achieve its objectives. Consequently, the EUR 60 000 threshold laid down in Article 207 of Financial Regulation (EU, Euratom) 2024/2509 does not apply. The maximum amount of FSTP that may be awarded to any single third party for the duration of the partnership is set at EUR 10.00 million. This ceiling is justified by the fact that FSTP is a primary activity of this action, by its expected duration of 7-10 years (exceeding a standard project lifespan), and by the extensive experience gained under predecessor partnerships. This ceiling is also justified by the fact that research on rare diseases, in particular clinical research, is complex and costly to put in place due to the scarcity, for each

	disease, of patients, of knowledge, of clinicians and of researchers, and by the request, if possible, to group diseases for research purposes, in order to tackle several diseases out of the estimated 6-8.000 rare diseases. However, if the objectives of the action would otherwise be impossible or overly difficult (and duly justified in the proposal) the maximum amount may be higher. • The starting date of the grant awarded under this topic may be as of the submission date of the application. Applicants must justify the need for a retroactive starting date in their application. Costs incurred from the starting date of the action may be considered eligible (and will be reflected in the entry into force date of the amendment to the grant agreement). The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.
<i>Total indicative budget</i>	The total indicative budget for the topic is EUR 91.3 million committed in annual instalments over the two years, 2026 and 2027 (EUR 48.7 million from the 2026 budget and EUR 42.6 million from the 2027 budget).

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The EU is reinforced as an internationally recognised driver of research and innovation in Rare Diseases (RD) and thereby substantially contributing to the achievement of the Sustainable Development Goals related to rare diseases.
- Research funders align, adopt and implement their RD research policies allowing for the optimal generation and translation of knowledge into meaningful health products and interventions responding to the needs of people living with a rare disease across Europe and globally.
- The RD research community at large benefit from and use an improved comprehensive knowledge framework and cross-border FAIR¹⁹² data access and analysis, including rare diseases registries, by integrating the EU, national/regional data and information infrastructures to improve translational research.
- People living with a rare disease, including those from underrepresented communities, benefit from a more timely, equitable access to innovative, sustainable and high-quality

¹⁹²

See definition of FAIR data in the introduction to this Work Programme part.

healthcare including novel diagnosis and treatments, taking stock of highly integrated research and healthcare systems.

- Researchers, innovators -as well as people living with a rare disease and their advocates (as co-creators)- effectively constitute and operate into an integrated research and innovation ecosystem to deliver cost-effective diagnosis and treatments.
- Public and private actors, including civil society (e.g. Non-Governmental Organisations, charities), establish coordinated and efficient multi-stakeholder collaborations at EU and national (including regional) levels, allowing for more effective clinical research, for example aiming at improved success rates of therapeutic development.

Scope: This topic targets an action under Article 24(2) HE Regulation aiming to add additional activities to existing grant agreements, together with additional partners (if relevant) that would deliver on those activities. The award of a grant to continue the partnership in accordance with this call should be based on a proposal submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases” and the additional activities (which may include additional partners) to be funded by the grant should be subject to an evaluation. Taking into account that the present action is a continuation of the topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases” and foresees an amendment to an existing grant agreement, the proposal should present the additional activities (including additional partners) to be covered by the award primarily in terms of grant agreement revisions. The existing action, the “European Rare Diseases Research Alliance” (ERDERA) can only reasonably be enhanced and enlarged on the basis of the existing consortium¹⁹³, as the co-funded framework established cannot simply be replaced without significant disruption, given the top-quality, long-term expertise and wide coverage of the beneficiaries comprising this consortium.

The proposal should thus present the specific additional activities (including, if relevant, additional partners) foreseen for the second instalment of the partnership. The partnership should continue to contribute to priorities of the communication “On effective, accessible and resilient health systems” (COM(2014) 215 final)¹⁹⁴, the “Communication from the Commission on enabling the digital transformation of health and care in the Digital Single Market; empowering citizens and building a healthier society” (COM(2018) 233 final)¹⁹⁵, the “Council conclusions on the Future of the European Health Union: A Europe that cares, prepares and protects” (9900/24)¹⁹⁶ and support the objectives of the EU4Health Programme (2021-2027)¹⁹⁷.

¹⁹³ Consortium which was awarded the grant under topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases”.

¹⁹⁴ https://health.ec.europa.eu/publications/communication-commission-com2014-215-final_en

¹⁹⁵ https://health.ec.europa.eu/publications/communication-commission-com-2018-233_en

¹⁹⁶ <https://www.consilium.europa.eu/en/press/press-releases/2024/06/21/european-health-union-council-calls-on-commission-to-keep-health-as-a-priority>

¹⁹⁷ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

This partnership should also contribute to achieving the objectives of the Pharmaceutical Strategy for Europe¹⁹⁸, in terms of fulfilling unmet medical needs and catalysing the clinical development of medicines for rare diseases (i.e. “orphan medicinal products”) and ensuring that the benefits of research and innovation reach patients in the EU and the Associated Countries. Moreover, the partnership is expected to contribute and align with the objectives of the Directive 2011/24/EU on the application of patients’ rights in cross-border healthcare¹⁹⁹ and of the European Health Data Space (EHDS)²⁰⁰.

Thanks to its capacity to bring together different stakeholders (e.g. research funders, health authorities, healthcare institutions, innovators, policymakers), the partnership will strengthen the European Research Area and consolidate the European research and innovation ecosystem with a critical mass of resources, and implement a long-term Strategic Research and Innovation Agenda (SRIA)²⁰¹.

The co-funded European Partnership on Rare Diseases should be implemented based on the priorities identified in the SRIA and through a joint programme of activities ranging from coordinating and funding transnational and clinical research to highly integrative and community-driven ‘in-house’ activities such as innovation strategies for the efficient exploitation of research results, EU clinical trial preparedness activities, optimisation of research infrastructures²⁰² and resources, including networking, training and dissemination activities. To this end, proposals are expected to build on the first phase of this partnership and should be structured along the following main objectives:

- Launch joint transnational calls for RD research and innovation actions, aligned with SRIA priorities, to fund patient-need-driven research across Europe, ensuring effective cross-border collaboration and scalability, while demonstrating short, medium and long-term impact and value creation through financial support to third parties and a rigorous monitoring strategy of research outputs.
- Further establish, strengthen and develop the different components of a European Clinical Research Network (CRN) to boost clinical trial readiness and capacity to readily implement well-coordinated multi-national clinical studies on rare diseases, building on the European Reference Networks (ERNs). The partnership is expected to showcase the CRN’s contribution to the cost-effective therapeutic development and decrease in diagnostic timelines linked with improvement in health outcomes ensuring durable collaboration among research, clinical, and regulatory actors.

¹⁹⁸ https://health.ec.europa.eu/medicinal-products/pharmaceutical-strategy-europe_en

¹⁹⁹ <https://eur-lex.europa.eu/eli/dir/2011/24/oj/eng> in particular articles 12 and 13 respectively on European Reference Networks (ERNs) and rare diseases

²⁰⁰ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

²⁰¹ <https://erdera.org/strategic-research-innovation-agenda-sria>

²⁰² The relevant European research infrastructures in the area of health should be exploited for available services, expertise and digital tools for dataset creation, standardisation, data discovery, secure access, management, visualisation, harmonisation, analysis and other functions as appropriate. The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

- Advance and consolidate the capacity building of the RD data ecosystem by supporting interoperable and/or federated cross-border access and analysis of FAIR research and healthcare data, including rare disease registries, ensuring ongoing their usability more efficient translational and clinical research, including regulatory science. The relevant European research infrastructures in the area of health should be exploited for available services, expertise and digital tools for the management and analyses of FAIR health data, as appropriate.
- Integrate basic, pre-clinical, clinical and implementation research to streamline the Research and Innovation (R&I) continuum and minimise redundancies, ensuring lasting impact on the quality of life of the people living with a rare disease while strengthening systemic efficiency and cost-effectiveness. To that end, the partnership should mobilise a significant investment to spur innovation, by aligning regional, national and European R&I priorities and improving EU competitiveness in R&I.
- Support research and innovation across key intervention areas (prevention, diagnosis, treatment), and promote the sustainable uptake of existing health innovations in clinical practice through coordinated training, implementation research, and active stakeholder engagement.
- Contribute to and align with the International Rare Disease Research Consortium (IRDiRC)²⁰³ to reinforce Europe's global leadership, ensure policy coherence, and sustain long-term strategic alignment beyond the lifetime of the partnership. To that end, an optimised assessment of the European contribution to IRDiRC would be beneficial to ensure complementarity and avoid overlaps.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

Collaboration with the European Commission's Joint Research Centre (JRC) should be considered to materialise the sharing of (meta)data regarding registries for rare diseases, exchanging data for clinical studies and research based on a unified pseudonymisation tool provided by the European Platform on Rare Disease Registration (EU RD Platform) and related tools and services, as well as in other areas of mutual interest, such as training and capacity building.

HORIZON-HLTH-2026-03-DISEASE-13: European Partnership for Pandemic Preparedness (Phase 2)

Call: Partnerships in Health (2026/2)
Specific conditions

²⁰³ <https://irdirc.org>

Horizon Europe - Work Programme 2026-2027
Health

<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 63.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 63.00 million.
<i>Type of Action</i>	Programme Co-fund Action
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: The proposal must be submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2024-DISEASE-12-01: “European Partnership for Pandemic Preparedness”. This eligibility condition is without prejudice to the possibility to include additional partners. In recognition of the opening of the US National Institutes of Health’s programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. Because the US contribution will be considered for the calculation of the EU contribution to the partnership, the concerned consortium of research funders from eligible EU Members States and Associated Countries must expressly agree to this participation. Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: The evaluation will take into account the existing context and the scope of the initial evaluation as relevant, and related obligations enshrined in the grant agreement. If the proposal is successful, the next stage of the procedure will be grant agreement amendment preparations. If the outcome of amendment preparations is an award decision, the coordinator of the consortium funded under topic HORIZON-HLTH-2024-DISEASE-12-01: “European Partnership for Pandemic Preparedness” will be invited to submit an amendment to the grant

	agreement, on behalf of the beneficiaries.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: This action is intended to be implemented in the form of an amendment of the grant agreement concluded pursuant to Article 24(2) of the Horizon Europe Regulation. For the additional activities covered by this action: • The funding rate is 50% of the eligible costs. This is justified by the pooling of proposers' in-kind contributions and in-house activities and by the nature of activities to be performed: in addition of joint calls, sustain and further develop the EU-wide networks and infrastructures for clinical research, and in particular a network of ever-warm clinical trial sites. • Beneficiaries may provide financial support to third parties (FSTP). The support to third parties can only be provided in the form of grants. As a co-funded European Partnership, providing financial support to third parties is a core activity of this action in order to achieve its objectives. Consequently, the EUR 60 000 threshold laid down in Article 207 of Financial Regulation (EU, Euratom) 2024/2509 does not apply. The maximum amount of FSTP that may be awarded to any single third party for the duration of the partnership is set at EUR 3.00 million. This ceiling is justified by the fact that FSTP is a primary activity of this action, by its expected duration of 7-10 years (exceeding a standard project lifespan), and by the extensive experience gained under predecessor partnerships. This ceiling is also justified by the fact that the FSTP will support infectious diseases research involving multi-country research networks and infrastructures as well as their coordination. However, if the objectives of the action would otherwise be impossible or overly difficult (and duly justified in the proposal) the maximum amount may be higher. • The starting date of the grant awarded under this topic may be as of the submission date of the application. Applicants must justify the need for a retroactive starting date in their application. Costs incurred from the starting date of the action may be considered eligible (and will be reflected in the entry into force date of the amendment to the grant agreement). The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results,

	as set out in the specific provision of Annex 5.
<i>Total indicative budget</i>	The total indicative budget for the topic is EUR 63 million committed in annual instalments over the two years, 2026 and 2027 (EUR 30 million from the 2026 budget and EUR 33 million from the 2027 budget).

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. Proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The EU offers a valued operational network of clinical research sites (both interventional and observational) that have the capacity to implement well-coordinated large-scale multi-country quality clinical studies in different target populations, which are able to smoothly transition to interventions relevant for cross-border health threats in readiness for or response to a public health emergency.
- Key stakeholders, including relevant EU and national entities, the scientific communities, policymakers and funders enhance their collaboration and coordination to strengthen research on pandemic preparedness and response, forming a strong, structured and comprehensive ecosystem with shared evidence, tools and methodologies cutting across sectors.
- Research funders, policymakers, relevant EU and national entities, and the research community recognise and rapidly close relevant research and related infrastructure gaps and break existing silos on pandemic preparedness research and response, adopting a One Health approach.
- Healthcare authorities, regulatory authorities, policymakers and other stakeholders utilise research results to develop evidence-based strategies and policies for pandemic preparedness and response, and deploy good practices to European countries and regions, and beyond whenever relevant.
- The research community benefits from and uses an improved comprehensive knowledge framework integrating the EU, national/regional data and information infrastructures to improve transnational research in the area of pandemic preparedness and response.
- The EU is strengthened as an internationally recognised actor for pandemic preparedness research and response, as such substantially contributing to global cooperation and coordination.

Scope: This topic targets an action under Article 24(2) HE Regulation aiming to add additional activities to existing grant agreements, together with additional partners (if relevant) that would deliver on those activities. The award of a grant to continue the partnership in accordance with this call should be based on a proposal submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2024-DISEASE-12-01:

“European Partnership for Pandemic Preparedness” and the additional activities (which may include additional partners) to be funded by the grant, such as the close coordination with the Clinical Trial Coordination Mechanism (CT-CM)²⁰⁴, should be subject to an evaluation. The partnership should be firmly anchored within the framework of the European Health Union package²⁰⁵ and ensure synergies with the European Health Emergency Preparedness and Response Authority (HERA) and other relevant Commission services. The partnership’s activities are expected to be key enablers of the EU Global Health Strategy²⁰⁶. Taking into account that the present action is a continuation of the topic HORIZON-HLTH-2024-DISEASE-12-01: “European Partnership for Pandemic Preparedness” and foresees an amendment to an existing grant agreement, the proposal should present the additional activities (including additional partners) to be covered by the award primarily in terms of grant agreement revisions. The existing action, the “Be Ready Now - European Partnership for Pandemic Preparedness” (BE READY NOW) can only reasonably be enhanced and enlarged on the basis of the existing consortium²⁰⁷, as the co-funded framework established cannot simply be replaced without significant disruption, given the top-quality, long-term expertise and wide coverage of the beneficiaries comprising this consortium.

The partnership should contribute to the actions proposed in the Joint Communication on the European Preparedness Union Strategy (JOIN(2025) 130 final²⁰⁸) which recognises the essential contribution of research and innovation to allow “continuously adapted, optimised and state-of-the-art responses to crisis”. It should also contribute to the “Strategy for European Life Sciences”²⁰⁹. Synergies with EU programmes such as EU4Health Programme (2021-2027)²¹⁰ or the Digital Europe Programme²¹¹ are encouraged.

The co-funded Partnership for Pandemic Preparedness should enable improved coordination and cooperation on national and European levels (and contributing globally), building on the Strategic Research and Innovation Agenda (SRIA)²¹² established in the first phase of the partnership. The partnership’s implementation is grounded in coordinating and jointly funding transnational research, combined with a strong focus on integrative ‘in-house’ activities, ultimately reinforcing the readiness of Europe’s research ecosystem. As a continuation of an existing action, the proposal should present the additional activities (including additional partners) to be covered by the award primarily in terms of grant agreement revisions.

²⁰⁴ <https://ec.europa.eu/transparency/expert-groups-register/screen/expert-groups/consult?lang=en&groupId=104872&fromMeetings=true&meetingId=59543>

²⁰⁵ https://commission.europa.eu/strategy-and-policy/priorities-2019-2024/promoting-our-european-way-life/european-health-union/health-crisis-preparedness_en

²⁰⁶ EU Global Health Strategy: Better Health for All in a Changing World - European Commission: https://health.ec.europa.eu/publications/eu-global-health-strategy-better-health-all-changing-world_en

²⁰⁷ Consortium which was awarded the grant under topic HORIZON-HLTH-2024-DISEASE-12-01: “European Partnership for Pandemic Preparedness”.

²⁰⁸ <https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=celex:52025JC0130>

²⁰⁹ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;

https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

²¹⁰ https://health.ec.europa.eu/funding/eu4health-programme-2021-2027-vision-healthier-european-union_en, <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32021R0522>

²¹¹ <https://digital-strategy.ec.europa.eu/en/activities/digital-programme>

²¹² <https://beready4pandemics.eu/sria>

The partnership should cover the full scope of preparedness research, ranging from basic and pre-clinical research, to clinical, public health, social sciences and implementation research. The partnership will consider the interplay between environmental, ecological and climatic factors and the emergence and spread of health threats and will adopt a One Health approach to better understand and mitigate the risks of emerging infectious diseases.

Of particular interest is the consolidation and further development of the ever-warm clinical research network, comprising both observational and interventional studies, ensuring continuous clinical research activity across diverse sites, and with the in-built capacity to rapidly respond to public health emergencies. In this regard, the partnership should ensure that provisions are in place for the close coordination with the CT-CM, which i) should facilitate providing scientific advice on the clinical research needs in preparedness and response to public health emergencies, and ii) promote a coordinated approach to the national and EU funding of identified clinical research needs.

The partnership should strengthen the European Research Area by supporting excellence in innovative research and capacity building, widening the engagement of countries not yet involved. As a demonstration of its added value, the partnership should be able to attract the engagement of a broad range of stakeholders beyond European health authorities and research funders, such as private and philanthropic actors and innovators.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)²¹³ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

The relevant European research infrastructures²¹⁴ in the area of health should be exploited for available services, expertise and digital tools for dataset creation, standardisation, data discovery, secure access, management, visualization, harmonization, analysis and other functions as appropriate.

When defining calls for proposals in the context of jointly funded transnational research, the partnership should consider sex and identity-related differences. If relevant, it also needs to consider the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, to produce meaningful and significant effects enhancing the societal impact of the related research activities. The support and involvement of citizens and civil society should be considered.

The partnership will consolidate a suitable health research data ecosystem aligned with the European Health Data Space (EHDS)²¹⁵, and the European Open Science Cloud (EOSC)²¹⁶

²¹³ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

²¹⁴ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

²¹⁵ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

²¹⁶ https://research-and-innovation.ec.europa.eu/strategy/strategy-2020-2024/our-digital-future/open-science/european-open-science-cloud-eosc_en

supporting the harmonisation and standardisation as well as the federated access of FAIR²¹⁷ research data in the context of pandemic preparedness and response. The partnership's work should comply with the appropriate ethical, regulatory and legal frameworks, and should ensure the timely translation of research outcomes into effective clinical and public health policy and innovation.

HORIZON-HLTH-2026-01-DISEASE-15: Scaling up innovation in cardiovascular health

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 1.90 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 1.90 million.
<i>Type of Action</i>	Coordination and Support Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, legal entities established in the United States of America may exceptionally participate as a beneficiary or affiliated entity, and are eligible to receive Union funding. Coordinators of projects must be legal entities established in an EU Member State or Associated Country. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant</i>	The rules are described in General Annex G. The following exceptions apply: Eligible costs will take the form of a lump sum as defined in the

²¹⁷

See definition of FAIR data in the introduction to this Work Programme part.

<i>Agreements</i>	Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ²¹⁸ .
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Healthcare providers, policymakers and researchers benefit from an improved knowledge base and collaboration on the key challenges and gaps on cardiovascular health research and a conceptual framework to develop a roadmap for research and innovation is established.
- Health systems gain improved and standardised evidence to better prevent, diagnose or treat cardiovascular diseases (CVDs) and associated comorbidities, based on the research results on prediction, early detection, screening practices and diagnostic methods and tools, including via personalised and digital approaches.
- Medical and non-medical health professionals and technology developers have an increased knowledge, awareness and capacity to uptake and deliver effective and innovative approaches for risk prediction, early detection, screening and health management strategies, such as Virtual Human Twins (VHT)²¹⁹ or Artificial Intelligence (AI)-based applications. This involves supporting, strategic foresight, improving health literacy and cross-sectoral knowledge exchange and collaboration to drive innovation in personalised prevention and cardiovascular risk prediction.
- Healthcare providers and policymakers have an improved knowledge base to inform future strategies for early detection and prevention of CVDs, with specific attention to women and vulnerable groups, through research on personalised risk prediction approaches that consider multiple and interacting risk factors (e.g. genetic predisposition, environmental pollutants, diet, lifestyle habits, multimorbidity, sex and gender).

Scope: CVDs are the main cause of death in the EU, with over 1.7 million deaths annually, costing about EUR 282 billion, or 11% of the healthcare budget²²⁰. With projections showing

²¹⁸ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

²¹⁹ See the European Virtual Human Twins Initiative: <https://digital-strategy.ec.europa.eu/en/policies/virtual-human-twins>

²²⁰ European Union takes action for the cardiovascular health of its 440 million people - EACH: <https://www.cardiovascular-alliance.eu/european-union-takes-action-for-the-cardiovascular-health-of-its-440-million-people>

a rise in CVD prevalence and mortality by 2050 due to an aging population, the Commission is preparing a comprehensive EU Cardiovascular Health (CVH) plan²²¹ to support Member States in their efforts to reduce the burden of CVDs. The proposal is expected to support prevention, early detection, including via digital and personalised approaches.

Applicants should take stock of research and innovation results to identify gaps and set up a plan for a Strategic Research and Innovation Agenda (SRIA) on CVDs, with the final aim of leveraging research and innovation results to improve risk prediction, early detection and screening practices for CVDs and associated comorbidities, especially obesity and diabetes, across the EU and Associated Countries. This initiative addresses the pressing need to translate existing innovations and promising research results into implementable protocols that enhance prevention, diagnosis, and health outcomes for diverse populations. By fostering collaboration and integrating digital tools and methods, proposals will support the future EU CVH plan, building and aligning with future and ongoing activities, including the European VHT Initiative, the 1+Million Genomes Initiative²²², Tech Foresight/Horizon scanning activities²²³ and actions funded under the EU4Health Programme (2021-2027)²²⁴ and the Digital Europe Programme²²⁵.

Proposals should include all the following activities:

- Conduct a comprehensive review at national, EU, and international levels of existing cardiovascular research and innovative healthcare solutions, potentially linked with associated comorbidities, to identify gaps and areas where future integration into health systems can have the greatest impact. The mapping should build upon existing EU-level reviews and pay particular attention to sex- and gender-related gaps, including under-representation in studies and differences in risk, diagnosis, and treatment. Such insights will inform subsequent policy actions and implementation initiatives under other funding programmes.
- Create a detailed report outlining the barriers to effective personalised prediction, screening and prevention in cardiovascular health, providing recommendations to overcome these challenges.
- Develop a SRIA on CVDs and associated comorbidities aiming to improve personalised prevention, prediction and screening and to inform research funders and stakeholders, including relevant EU and national initiatives. The agenda will include stakeholder validation and adoption pathways. Support the development of personalised prevention and care pathways and the role of digital interventions, based on genomics, VHTs and

²²¹ <https://data.consilium.europa.eu/doc/document/ST-15315-2024-INIT/en/pdf>

²²² <https://digital-strategy.ec.europa.eu/en/policies/1-million-genomes>

²²³ Farinha, J., Nagy, O., Bailey, G., Mochan, A., Polvora, A. et al., *Embodying the Future - Horizon scanning for emerging technologies and breakthrough innovations in the field of human-like AI systems*, Publications Office of the European Union, Luxembourg, 2025, <https://publications.jrc.ec.europa.eu/repository/handle/JRC143535>

²²⁴ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

²²⁵ <https://digital-strategy.ec.europa.eu/en/activities/digital-programme>

AI-driven methods in line with the European Health Data Space (EHDS)²²⁶ Regulation, to enhance precision in early detection and health management. Where applicable, the mapping of existing practices such as biomarkers for diagnosis, monitoring in patients, and stratification of patient groups should be considered.

- Integrate sex and gender-related variables, age, racial or ethnic origin²²⁷, socio-economic, lifestyle and behavioural factors, genetic predisposition into research design, data collection and analysis, to ensure inclusive and generalisable findings enhancing the effectiveness of screening, diagnostic, and prevention strategies across diverse population groups.
- Organise high-impact, targeted events with clear objectives to promote a multi-sectorial approach, fostering collaboration among healthcare providers, researchers, civil society, patients' organisations, and policymakers
- Work with health experts to develop the capacities for implementing standardised screening protocols and methods.
- Develop a comprehensive dissemination strategy and stakeholder engagement plan to share findings and promote the results, utilising online platforms and social media to reach a broad audience. Complement with long-term engagement mechanisms such as policy briefings or partnerships with EU-level dissemination networks.

The applicants should ensure adequate involvement across the project lifespan of all relevant stakeholders and value chain actors including industry, healthcare professionals, scientists, patients' associations to ensure performance and sustainability and maximise the final impact.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts and institutions, in order to meaningfully enhance the societal impact of the related research activities.

HORIZON-HLTH-2027-02-DISEASE-01-two-stage: Innovative healthcare interventions for non-communicable diseases

Call: Cluster 1 - Health (Two stage - 2027)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 7.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.

²²⁶ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

²²⁷ The use of the term 'racial or ethnic origin' does not imply an acceptance of theories that attempt to determine the existence of separate human races.

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<i>Indicative budget</i>	The total indicative budget for the topic is EUR 63.80 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Admissibility conditions</i>	The conditions are described in General Annex A. The following exceptions apply: Applicants submitting a proposal for a blind evaluation (see General Annex F) must not disclose their organisation names, acronyms, logos nor names of personnel in the proposal abstract and Part B of their first-stage application (see General Annex E).
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: For the first stage, the thresholds for each criterion will be 4 (Excellence) and 4 (Impact). The overall threshold applying to the sum of the two individual scores will be set at a level that ensures the total requested budget of proposals admitted to stage 2 is as close as possible to three times the available budget, and not less than two and a half times the available budget. For the second stage, the thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: The first-stage proposals of this topic will be evaluated blindly.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results,

	as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ²²⁸.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Researchers, developers and clinical practitioners have access to state-of-the-art knowledge, data, technologies, tools, methods, best practices, and trainings to develop innovative healthcare interventions aimed at reducing burden of the following specific Non-Communicable Diseases (NCDs): cardiovascular diseases, diabetes, chronic respiratory diseases or chronic kidney diseases.
- Scientific and clinical communities can use innovative healthcare interventions to generate meaningful advances in clinical practice and care for patients with NCDs following validation in late-stage clinical trials.
- Scientific and clinical communities make wide use of relevant databases and/or integrate them with existing infrastructures for storage and sharing of collected data according to FAIR²²⁹ principles, thereby encouraging further use of the data.
- Policymakers, scientific and clinical communities, developers, patient organisations, regulators, and other relevant bodies are informed of the research advances made and the requirements for a widespread implementation of the innovative therapeutic interventions and complementary approaches.
- Patients and caregivers are constructively engaged with the research, ensuring that their needs are catered for, with the aim of tangibly benefitting from the interventions.

Scope: NCDs represent over 80% of the disease burden in Europe and the leading cause of avoidable premature deaths. Innovative and effective healthcare interventions are required to provide treatment and disease management solutions and assure best quality of care for patients suffering from NCDs when prevention strategies have failed.

Proposals should address all the following aspects:

²²⁸ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

²²⁹ See definition of FAIR data in the introduction to this Work Programme part.

- Perform rigorous early stage²³⁰ clinical trial(s) to validate novel or refined healthcare interventions²³¹ for treatment and/or disease management solutions for patients suffering from the following specific NCDs: cardiovascular diseases, diabetes, chronic respiratory diseases or chronic kidney diseases²³². Whenever relevant, existing co- and multimorbidities should be addressed in the trial design.
- Clinical trial(s) should be supported by completed proof-of-concept²³³ of clinical safety and efficacy.
- Both preclinical research and the draft clinical trial protocol should be completed at the time of submission of the proposal. Proposals should also demonstrate evidence of preliminary consultations with ethics and regulatory authorities at the time of submission.
- A sound feasibility assessment, including an appropriate patient selection and realistic recruitment plans, justified by publications or preliminary results should be provided.
- Take into account sex and gender differences in all relevant aspects throughout the research process, and consider stratification criteria such as age, disability, racial or ethnic origin²³⁴, socio-economic status, genetic and epigenetic variations, etc., where relevant.
- Use and/or develop technologies, including digital ones (e.g. (generative) Artificial Intelligence, wearable technologies) to help implement and monitor the long-term efficacy of the intervention(s), as well as manage the disease and/or monitor their progression (e.g. with unobtrusive technologies suitable for patient monitoring at home and in real-world conditions), whilst also ensuring they are bias-free, inclusive, and ethically sound. Hardware and software should be interoperable in line with internationally accepted standards²³⁵. The use of virtual human twins²³⁶ could also be considered, where relevant.
- Exploit existing data, health data infrastructures²³⁷, biobanks, registries and/or cohorts, together with the generation of new data that should be managed in line with the FAIR

²³⁰ For pharmacological interventions: phase 1 and/or phase 2 clinical trials.

²³¹ Applicants may address any mono- or combinatorial pharmacological and/or non-pharmacological interventions.

²³² Other diseases are not within the scope of this topic.

²³³ Comparative effectiveness studies are not within the scope of this topic.

²³⁴ The use of the term 'racial or ethnic origin' does not imply an acceptance of theories that attempt to determine the existence of separate human races.

²³⁵ For digital technologies concerned, appropriate measures for the security of the communications between the intended parties should be considered, in particular based on the use of post-quantum cryptography.

²³⁶ <https://digital-strategy.ec.europa.eu/en/policies/virtual-human-twins>

²³⁷ For instance BBMRI, ELIXIR, EU-OPENSREEN, ECRIN, EATRIS, etc.

principles and contribute to emerging research infrastructures established in the framework of the European Health Data Space (EHDS)²³⁸, when relevant.

- Advance research by leveraging already existing and emerging state-of-the-art research infrastructures as well as results stemming from EU-supported research projects, where applicable.
- Engage all relevant stakeholders (especially patients and patients' representatives, caregivers, clinicians, counsellors, regulators, etc.) to design end-user optimised interventions.
- Engage with national public health authorities and regulators to ensure a robust development pathway and further uptake of the intervention.
- Present a thorough health-economic assessment and Real-World Data (RWD)²³⁹ analysis to enhance sustainability and scalability of novel interventions.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)²⁴⁰ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

All projects funded under this topic are encouraged to participate in networking and joint activities, as appropriate²⁴¹ and explore potential synergies with projects funded under the EU4Health Programme (2021-2027)²⁴² in the area of NCDs.

Applicants invited to the second stage should provide details of their clinical studies²⁴³ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

²³⁸ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

²³⁹ EMA definition: “Real-World Data are routinely collected data relating to patient health status or the delivery of healthcare from a variety of sources other than traditional clinical trials (e.g. claims databases, hospital data, electronic health records, registries, mhealth data, etc.)”.

²⁴⁰ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

²⁴¹ Proposals should include a budget for the attendance to regular joint meetings and may consider covering the costs of any other potential joint activities without the prerequisite to detail concrete joint activities at this stage. The details of these joint activities will be defined during the grant agreement preparation phase.

²⁴² https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

²⁴³ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

HORIZON-HLTH-2027-01-DISEASE-05: Development of novel small molecule antiviral therapeutics for pathogens with epidemic potential

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 9.00 and 11.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 44.20 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: To ensure a balanced project portfolio covering the viruses/groups of viruses targeted in this topic²⁴⁴, grants will be awarded (within available budget) to proposals not only in order of ranking but at least also to those proposals that are the highest ranked within different viruses/groups of viruses targeted, provided that the proposals attain all thresholds.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results,

²⁴⁴ Virus/group of viruses i to vi, as given in the scope of this topic.

	as set out in the specific provision of Annex 5.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental antivirals for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation.
- Candidate antiviral therapies are available to treat patients for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope: Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The availability of antivirals targeting conserved viral or host mechanisms would provide a critical preparedness measure against future health threats caused by (re)emerging infectious disease epidemics or pandemics, due to infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁴⁵.

Antibodies and antibody derived proteins are excluded from the scope of this topic.

Applicants should explicitly state in their proposal which of the following viruses/groups of viruses is targeted and the proposed work should address only this specific virus/group of viruses. The proposed work should aim to advance the development of novel or existing antiviral candidates targeting exclusively one of the following viruses/groups of viruses:

- i. Junin mammarenavirus and/or Lassa mammarenavirus
- ii. Tick-borne encephalitis virus and/or Japanese encephalitis virus
- iii. Andes virus and/or Hantaan virus and/or Sin Nombre virus
- iv. Hendra virus
- v. Enterovirus D68
- vi. Venezuelan equine encephalitis virus

²⁴⁵ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

Identifying a specific virus/group of viruses does not preclude the exploration of these antiviral candidates' effects on other viruses/groups of viruses. Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address some of the following research areas:

- Discovery and selection of candidate antivirals with consideration for intra-family and/or variant-transcending potential.
- Optimisation of selected candidates to improve potency, selectivity, pharmacokinetics, and developability, using Structure-Activity Relationship (SAR) studies or equivalent methodologies.
- In-vitro characterisation of antiviral activity, mechanism of action, and, where appropriate, resistance potential across multiple viruses or strains.
- In-vivo tests in at least one animal model or, if available in human organoid or organotypic models, to demonstrate the protective function of the antiviral candidates and deemed sufficient for moving to first clinical trials.
- If requested by regulators as pre-requisite for clinical studies, in-vivo tests in a non-human primate model.
- Production of batches of the most promising antiviral candidates according to the Good Manufacturing Practices (GMP)²⁴⁶ of the most promising therapeutics solution.
- First in human clinical safety studies demonstrating a clear regulatory pathway for market authorisation. Attention should be paid to critical biological and social factors such as sex, age, ethnicity and disability.

Participation of third countries where viruses addressed in the proposal are endemic or where outbreaks have occurred or are ongoing is encouraged.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)²⁴⁷ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

All projects funded under this topic are expected to engage with regulatory bodies in a timely manner to ensure adequacy of the actions from a regulatory point of view.

Proposals should advance research by leveraging already existing and emerging state-of-the-art research infrastructures²⁴⁸ such as those having contributed to the services developed under the ISIDORE project²⁴⁹.

²⁴⁶ https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en
²⁴⁷ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

The projects funded under this topic should synergise with projects funded by the co-funded European Partnership for Pandemic Preparedness²⁵⁰.

Applicants should provide details of their clinical studies²⁵¹ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-01-DISEASE-06: Development of monoclonal antibodies to prevent and treat infections from Flaviviruses

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 9.00 and 10.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 37.30 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: To ensure a balanced project portfolio covering the Flaviviruses targeted

²⁴⁸ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

²⁴⁹ <https://isidore-project.eu>

²⁵⁰ <https://cordis.europa.eu/project/id/101226682>, <https://beready4pandemics.eu>

²⁵¹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	in this topic ²⁵² , grants will be awarded (within available budget) to proposals not only in order of ranking but at least also to those proposals that are the highest ranked within different Flaviviruses targeted, provided that the proposals attain all thresholds.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental monoclonal antibodies for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation.
- Candidate monoclonal antibody therapies are available to treat patients for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope: Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The capacity to produce antibodies that can target new variants and rapidly increase production would serve as an essential preparedness strategy against future health threats, whether from infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁵³.

Applicants should explicitly state in their proposal which of the following Flaviviruses is targeted and the proposed work should address only this specific Flavivirus. The proposed work should aim to advance the development of existing prophylactic and therapeutic monoclonal antibody candidates targeting exclusively one of the following Flaviviruses:

- Dengue Virus
- Tick-borne Encephalitis Virus

²⁵² Flavivirus i to vi, as given in the scope of this topic.

²⁵³ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

- iii. Japanese Encephalitis Virus
- iv. West Nile Fever Virus
- v. Yellow Fever Virus
- vi. Zika Virus

Proposals should focus on antibodies produced or derived from a single cell clone through recombinant expression, such as B-cell derived antibodies, hybridoma derived antibodies and nanobodies.

Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address all the following research areas:

- If necessary, finalisation of the in-vitro characterisation of existing monoclonal antibody candidates with regard to target specificity, epitope recognised, and their ability to impair or inactivate viral functions.
- In-vivo tests in at least one animal model or, if available in humanised immune system animal models, to demonstrate the protective function of the monoclonal antibodies deemed sufficient for moving to first clinical trials.
- If requested by regulators as pre-requisite for clinical studies, in-vivo tests in a non-human primate model.
- Evaluation of Antibody-Dependent Enhancement (ADE) risk where scientifically relevant.
- Production of batches of the most promising antibody candidates according to the Good Manufacturing Practices (GMP)²⁵⁴.
- First in human clinical safety studies demonstrating a clear regulatory pathway for market authorisation. Attention should be paid to critical biological and social factors such as sex, age, ethnicity and disability.

Participation of third countries where viruses addressed in the proposal are endemic or where outbreaks have occurred or are ongoing is encouraged.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)²⁵⁵ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

²⁵⁴ https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en
²⁵⁵ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

All projects funded under this topic are expected to engage with regulatory bodies in a timely manner to ensure adequacy of the actions from a regulatory point of view.

Proposals should advance research by leveraging already existing and emerging state-of-the-art research infrastructures²⁵⁶ such as those having contributed to the services developed under the ISIDORE project²⁵⁷.

Applicants should provide details of their clinical studies²⁵⁸ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-01-DISEASE-07: Development of monoclonal antibodies to prevent and treat infections from Filo-, Nairo-, Phenui-, Picorna- and Toga viruses

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 9.00 and 10.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 37.30 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

²⁵⁶ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

²⁵⁷ <https://isidore-project.eu>

²⁵⁸ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: To ensure a balanced project portfolio covering the viruses targeted in this topic²⁵⁹, grants will be awarded (within available budget) to proposals not only in order of ranking but at least also to those proposals that are the highest ranked within different viruses targeted, provided that the proposals attain all thresholds.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental monoclonal antibodies for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation.
- Candidate monoclonal antibody therapies are available to treat patients for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope: Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The capacity to produce antibodies that can target new variants and rapidly increase production would serve as an essential preparedness strategy against future health threats, whether from infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁶⁰.

Applicants should explicitly state in their proposal which of the following viruses is targeted and the proposed work should address only this specific virus. The proposed work should aim to advance the development of existing prophylactic and therapeutic monoclonal antibody candidates targeting exclusively one of the following viruses:

²⁵⁹ Virus i to vi, as given in the scope of this topic.

²⁶⁰ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

- i. Ebola Virus
- ii. Marburg Virus
- iii. Crimean-Congo Hemorrhagic Fever Virus
- iv. Rift Valley Fever Virus
- v. Enterovirus D68
- vi. Chikungunya Virus

Proposals should focus on antibodies produced or derived from a single cell clone through recombinant expression, such as B-cell derived antibodies, hybridoma derived antibodies and nanobodies.

Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address all the following research areas:

- If necessary, finalisation of the in-vitro characterisation of existing monoclonal antibody candidates with regard to target specificity, epitope recognised, and their ability to impair or inactivate viral functions.
- In-vivo tests in at least one animal model or, if available in humanised immune system animal models, to demonstrate the protective function of the monoclonal antibodies deemed sufficient for moving to first clinical trials.
- If requested by regulators as pre-requisite for clinical studies, in-vivo tests in a non-human primate model.
- Evaluation of Antibody-Dependent Enhancement (ADE) risk where scientifically relevant.
- Production of batches of the most promising antibody candidates according to the Good Manufacturing Practices (GMP)²⁶¹.
- First in human clinical safety studies demonstrating a clear regulatory pathway for market authorisation. Attention should be paid to critical biological and social factors such as sex, age, ethnicity and disability.

Participation of third countries where viruses addressed in the proposal are endemic or where outbreaks have occurred or are ongoing is encouraged.

²⁶¹ https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)²⁶² is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

All projects funded under this topic are expected to engage with regulatory bodies in a timely manner to ensure adequacy of the actions from a regulatory point of view.

Proposals should advance research by leveraging already existing and emerging state-of-the-art research infrastructures²⁶³ such as those having contributed to the services developed under the ISIDORE project²⁶⁴.

Applicants should provide details of their clinical studies²⁶⁵ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-01-DISEASE-08: Development of innovative antimicrobials against pathogens resistant to antimicrobials

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 8.00 and 10.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 44.20 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).

²⁶² <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

²⁶³ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

²⁶⁴ <https://isidore-project.eu>

²⁶⁵ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: To ensure a balanced project portfolio covering the pathogens targeted in this topic²⁶⁶, grants will be awarded (within available budget) to proposals not only in order of ranking but at least also to those proposals that are the highest ranked within different pathogens targeted, provided that the proposals attain all thresholds.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to new and innovative products for the treatment of antimicrobial resistant bacteria and fungi, as well as for further clinical investigation.
- Candidate therapies are available to treat patients for antimicrobial resistant bacteria and fungi, increasing therapeutic options for clinical deployment in the fight against Antimicrobial Resistance (AMR).

Scope: The rapid rise of AMR presents a formidable threat to public health, challenging our ability to treat infections that were once easily managed with standard antimicrobials. As pathogens continually adapt and develop resistance to existing drugs, the efficacy of these treatments diminishes, leading to more severe and prolonged illnesses, increased healthcare costs and productivity losses, and higher mortality rates. This escalating crisis underscores an urgent need for viable therapeutic alternatives required to reduce the burden of diseases caused by antibiotic resistance. Innovative solutions are crucial to maintaining effective disease management and safeguarding public health.

Proposals should pursue the development of innovative and effective antibacterial and antifungal agents, including antibody-based therapies, which meet at least one of the four

²⁶⁶ Pathogen i to vi, as given in the scope of this topic.

World Health Organization (WHO) innovation criteria²⁶⁷, namely: i) new chemical class, ii) new target, iii) new mode of action and iv) no evidence of cross-resistance.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁶⁸ and the forthcoming Critical Medicines Act²⁶⁹.

Proposals under this topic should not pursue the development of phage-therapies.

Applicants should explicitly state in their proposal which of the following pathogens is targeted and the proposed work should address only this specific pathogen. The proposed work should pursue the development of existing therapeutic candidates targeting exclusively one of the following pathogens:

- i. Carbapenem resistant *Acinetobacter baumannii* (CRAB)
- ii. Carbapenem-resistant Enterobacterales (CRE) and third-generation cephalosporin-resistant Enterobacterales (C3GRE)
- iii. Carbapenem resistant *Pseudomonas aeruginosa*
- iv. Methicillin-Resistant *Staphylococcus aureus* (MRSA)
- v. (Drug)-resistant *Aspergillus fumigatus*
- vi. (Drug)-resistant *Candida* spp

Identifying a specific pathogen does not preclude the exploration of these candidates' effects on other bacteria or fungi. Proposals should thus aim to accelerate testing of novel candidates in human trials, diversify and accelerate the global prophylactic and therapeutic research and development portfolio for AMR bacterial and fungal infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address all the following areas:

- If necessary, finalisation of in-vivo tests in at least one animal model or, if available in humanised immune system animal models, to demonstrate the protective function of the therapeutics deemed sufficient for moving to first clinical trials.
- If requested by regulators as pre-requisite for clinical studies, in-vivo tests in a non-human primate model.
- Production of batches of the most promising antimicrobials candidates according to the Good Manufacturing Practices (GMP)²⁷⁰.

²⁶⁷ <https://www.who.int/publications/i/item/9789240093461>

²⁶⁸ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

²⁶⁹ https://health.ec.europa.eu/publications/proposal-critical-medicines-act_en

²⁷⁰ https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

- In human clinical safety and efficacy studies, demonstrating a clear regulatory pathway for market authorisation. Attention should be paid to critical biological and social factors such as sex, age, ethnicity, disability and vulnerability.

Participation of third countries where AMR bacteria and fungi in the proposal are endemic or where outbreaks have occurred or are ongoing is encouraged.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)²⁷¹ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

All projects funded under this topic are expected to engage with regulatory bodies in a timely manner to ensure adequacy of the actions from a regulatory point of view.

Proposals should advance research by leveraging already existing and emerging state-of-the-art research initiatives such as the co-funded European Partnership on One Health Anti-Microbial Resistance (EUP OHAMR)²⁷².

Applicants should provide details of their clinical studies²⁷³ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-01-DISEASE-10: Prevention and management of chronic non-communicable diseases in children and young people (GACD)

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 3.00 and 4.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 11.80 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding.

²⁷¹ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

²⁷² <https://cordis.europa.eu/project/id/101217154>, <https://ohamr.eu>

²⁷³ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ²⁷⁴.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to some of the following expected outcomes:

- Researchers, healthcare practitioners and providers in low- and middle-income countries (LMICs) ²⁷⁵ and/or those in high-income countries (HICs) serving disadvantaged populations have access to improved insights and evidence on how to equitably promote the early prevention, risk reduction, and timely diagnosis of Non-Communicable Diseases (NCDs) in children and/or young people.
- Policymakers, public health managers and authorities, parents and their children, and young adults have access to evidence and recommendations for national programmes and policies to improve quality of life in children and/or young people and extend healthy life expectancy.
- Researchers, clinicians, policymakers, public health managers and authorities have an improved understanding how to effectively adapt and/or scale up interventions for

²⁷⁴ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

²⁷⁵ As defined by the World Bank, <https://www.worldbank.org>

prevention and management of chronic NCDs in children and/or young people at local, regional, and national levels.

- Communities, parents and their children, young adults, local stakeholders and authorities are fully engaged in implementing and taking up interventions that tackle NCDs in children and/or young people.

Scope: The Commission is a member of the Global Alliance for Chronic Diseases (GACD)²⁷⁶. The GACD specifically addresses NCDs and supports implementation research²⁷⁷ to improve health outcomes. This topic is launched in concertation with the other GACD members (international funding agencies) and aligned with the 12th GACD call.

Chronic NCDs that begin in childhood have an impact on both quality of life and life expectancy. Onset of many NCDs diseases occurs at younger ages in LMICs, and this is further accompanied by a longer duration of disease and a higher rate of complications, including multimorbidity. The conditions in which people are born, grow and live (the social determinants of health) including access to good nutrition, education, housing, and healthcare are major contributors to health and ill health²⁷⁸.

Up to 70% of preventable adult deaths from NCDs are linked to risk factors originating in childhood and adolescence²⁷⁹, and interventions that can successfully control or prevent chronic disease in young people can dramatically improve health outcomes later in life. Childhood and adolescence are critical periods, when behaviours associated with NCD risk are adopted including tobacco use, alcohol use, substance abuse, unhealthy diets and sedentary lifestyles and children and young people are often targeted by commercial marketing of unhealthy products.

The aim of this topic is to fund implementation research, exploring strategies, evidence-based program and policy interventions across prevention, diagnosis, screening and management of chronic NCDs, centred on the critical life stages spanning early childhood to young adulthood (1-24 years of age) living in LMICs, and/or underserved populations in HICs.

In this regard, proposals focused on implementation research should explore implementation strategies on evidence-based interventions, adaptations of interventions and tailored interventions, or initiatives including (though not limited to) those focussed on one or more of the following:

- Policy evaluation to tackle childhood- and/or youth-relevant social, economic, political, structural or commercial determinants of chronic NCD conditions.

²⁷⁶ <https://www.gacd.org>

²⁷⁷ https://iris.who.int/bitstream/handle/10665/91758/9789241506212_eng.pdf

²⁷⁸ <https://www.taylorfrancis.com/chapters/oa-edit/10.4324/9781003306689-20/social-determinants-health-ncds-ruth-bell-jaime-miranda-jean-woo-michael-marmot>

²⁷⁹ <https://data.unicef.org/topic/child-health/noncommunicable-diseases/>

- Prevention of NCDs using children and/or young people targeted implementation strategies (e.g. educational strategies, vaccination strategies, promotion of behavioural and lifestyle changes).
- Screening and diagnosis of NCDs (or risk factors) in children and/or young people (in particular use of digital tools).
- Cost effective and patient-centred management of NCDs in children and/or young people (including access to medicines and equipment; integrated care pathways; continuity of care for adolescents with existing non-communicable diseases who "age out" of paediatrics, caregiver health and support, citizen science approaches).

Multiple interventions focus on prevention of NCDs in children and young people, yet relatively few have focussed on strategies for management of chronic conditions in these critical life stages, and a limited number of studies have been carried out to study implementation of these in LMIC contexts or with underserved communities. In this instance it would be anticipated that proposals should explore implementation strategies using the appropriate hybrid design study incorporating effectiveness and implementation research outcomes. Therefore, it may be important to undertake formative research as a part of the proposal to support readiness for implementation.

The proposed implementation research should be focused on one or more evidence-based interventions (or complex interventions), providing existing evidence of the intervention's effectiveness, cost-effectiveness, sustainability, scalability and potential for long-term health and other impacts (and in what context this evidence has been generated).

Applicants should provide rationale and explore the implementation of proposed intervention(s) for a selected study population(s) based in one or more LMICs, and/or underserved populations experiencing health disparities, including Indigenous populations, in HICs, considering the unique social, political, economic, and cultural context(s) in which the study will take place²⁸⁰. Applicants should justify why any adaptation will not compromise the known effectiveness of the selected intervention(s).

Proposals should address all the following implementation research activities²⁸¹:

- Clearly describe the implementation research methodology, including the statistical design.

²⁸⁰ Focus on populations facing extreme vulnerabilities, such as individuals or communities living in informal settlements, post-disaster settings, or in situations of homelessness is encouraged (though not required).

²⁸¹ The following types of proposals are not in the scope of this topic: i) proposals with the primary aim of informing the development and/or selection of an intervention for a given context, where the implementation component will be explored in a future project (i.e. standalone feasibility projects); ii) epidemiological cohorts; iii) etiological work, mechanistic, or epidemiological research, unless an essential component of a focused study to develop implementation research approaches; iv) clinical trials, validation studies, or intervention efficacy studies for a new or established pharmacological agent or behavioural intervention.

- Have an appropriate strategy for measuring implementation research outcomes and real-world effectiveness outcomes and indicators.
- Specifically address issues of equitable implementation to ensure interventions reach the populations that need them the most.
- Engage an appropriately expert and skilled research team which can ensure a suitable multidisciplinary approach and that demonstrates equitable partnership and shared leadership between HIC-LMIC, and/or non-Indigenous-Indigenous members of the project team and external stakeholders through a clear governance strategy.
- Provide a stakeholder engagement strategy with evidence of support/engagement from key stakeholders for delivering the intervention and a pathway to sustain the proposed intervention (if proven effective) after the funding from the GACD grant ends.
- Provide opportunities for NCD-focused implementation research capacity building for early career researchers and team members from lower resourced environments, such as LMICs or disadvantaged communities.
- Ensure meaningful involvement of early career team members, including at least one early career member as a co-investigator.

The study population may include children and/or young people in the general population, with one or more existing NCDs, those currently without NCDs, or a combination of any of the above. Applicants may propose implementation research focused on interventions that are implemented at the individual, family, community (e.g. work or school), population, and/or structural level. With regard to NCDs, applicants are encouraged to explore any chronic non-communicable condition (or combination of conditions), including mental health disorders, autoimmune conditions, musculoskeletal conditions, neurological disorders and sleep disorders and/or any risk factor (or combination of risk factors). Additionally, whenever relevant, applicants are also encouraged to take a life course approach, adapting interventions for particular life stages with the goal of promoting life-long health.

Proposals should use an appropriate implementation research design and framework²⁸², before and after studies, and additional implementation science classifications of study designs (e.g. hybrid designs²⁸³), noting that applicants are not limited to any particular design.

Proposals would be expected to generate evidence that is of direct relevance to policymakers, communities and practitioners. Proposed work should identify and engage all key stakeholders necessary and relevant to the development, undertaking and knowledge translation phases of the project, including meaningful collaboration with young people

²⁸² Examples of frameworks include (this list is not exclusive): i) Consolidated Framework for Implementation Research (CFIR); ii) the context enhanced (RE-AIM) Reach, Effectiveness, Adoption, Implementation, Maintenance); iii) Practical Robust Implementation and Sustainability Model (PRISM) frameworks; iv) Framework for Developing and Evaluating Complex Interventions (MRC & NIHR).

²⁸³ <https://pmc.ncbi.nlm.nih.gov/articles/PMC3731143> and <https://pmc.ncbi.nlm.nih.gov/articles/PMC6779135>

themselves (and their families). Proposals should also consider using co-development and co-design approaches, involving policymakers, local authorities, community groups, educators, healthcare providers, and other individuals or organisations necessary to the delivery and sustainability of the study outcomes. Project partners should be engaged from the beginning to contribute to the sustainability of the intervention after the end of project. Proposals should demonstrate sustainability of the strategy, beyond the lifespan of the project.

Poverty, discrimination based on sex, racial or ethnic origin²⁸⁴, religion or belief, disability, age, and other inequities are directly associated with reduced potential for equitable access to quality care. Proposals should consider relevant determinants of health (e.g. social, structural, commercial, economic) and discuss their potential impact on the effective implementation of the intervention(s). If there is a focus on a particular population (e.g. gender, racial or ethnic origin²⁸⁵, etc.), then the reason for this should be justified.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

All projects funded under this topic are strongly encouraged to participate in networking and joint activities, including internationally, as appropriate. These activities could, for example, involve the participation in joint workshops, the Annual Scientific Meetings of the GACD, the exchange of knowledge, the development and adoption of best practices, or joint communication activities. Therefore, proposals are expected to include a budget for such activities and may consider covering the costs of any other potential joint activities without the prerequisite to detail concrete joint activities at this stage. The details of these joint activities will be defined during the grant agreement preparation phase.

Applicants should provide details of their clinical studies²⁸⁶ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-02-DISEASE-14-two-stage: Clinical trials for advancing innovative interventions for neurodegenerative diseases

Call: Cluster 1 - Health (Two stage - 2027)	
Specific conditions	
<i>Expected EU contribution per</i>	The Commission estimates that an EU contribution of around EUR 10.00 million would allow these outcomes to be addressed

²⁸⁴ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

²⁸⁵ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

²⁸⁶ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

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<i>project</i>	appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 39.30 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Admissibility conditions</i>	The conditions are described in General Annex A. The following exceptions apply: Applicants submitting a proposal for a blind evaluation (see General Annex F) must not disclose their organisation names, acronyms, logos nor names of personnel in the proposal abstract and Part B of their first-stage application (see General Annex E).
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: For the first stage, the thresholds for each criterion will be 4 (Excellence) and 4 (Impact). The overall threshold applying to the sum of the two individual scores will be set at a level that ensures the total requested budget of proposals admitted to stage 2 is as close as possible to four times the available budget, and not less than three and a half times the available budget. For the second stage, the thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: The first-stage proposals of this topic will be evaluated blindly.
<i>Legal and financial set-up of the Grant</i>	The rules are described in General Annex G. The following exceptions apply:

<i>Agreements</i>	The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities make effective use of state-of-the-art knowledge, data, technologies, tools, methods, best practices, and trainings to underpin and complement the development of innovative interventions aimed at more effective treatments for neurodegenerative diseases.
- The scientific and clinical communities benefit from the exchange of data, knowledge and best practices, thereby strengthening their collaboration in the EU, the Associated Countries and beyond.
- The scientific and clinical communities make wide use of relevant databases and/or integrate them with existing infrastructures for storage and sharing of collected data according to FAIR²⁸⁷ principles, thereby encouraging further use of the data.
- Policymakers, funders, scientific and clinical communities, patient organisations, regulators, and other relevant bodies are informed of the research advances made and the requirements for a widespread implementation of the innovative therapeutic interventions and complementary approaches.
- Patients and caregivers are constructively engaged with the research, ensuring that their needs are catered for, with the aim of tangibly benefitting from the interventions.

Scope: Neurodegenerative diseases are a high burden for patients, caregivers, health systems and society. Given the limitations with current therapeutic solutions, including that they primarily address symptoms rather than underlying causes and can have serious side effects, together with the increasing prevalence of neurodegenerative diseases in an aging population, there is a huge need to develop more innovative, safer and more effective therapeutic solutions for these diseases. To further enhance their safety and effectiveness, the therapeutic solution based on an active substance should be combined/complemented with another multidisciplinary approach (e.g. lifestyle changes, cognitive training, rehabilitation therapies). Together this innovative intervention should lead to an improved quality of life and reduce the societal impact of these diseases.

Rare neurodegenerative diseases are excluded²⁸⁸.

²⁸⁷

See definition of FAIR data in the introduction to this Work Programme part.

Proposals should address most of the following aspects:

- Perform rigorous early-stage²⁸⁹ clinical trials into the safety and efficacy of the innovative interventions and their mode of administration, ensuring adequate cohorts/sample sizes with adequate representation of the patient population, including in terms of age, sex and ethnicity.
- Through the clinical trials and to the extent possible of additional studies, gain further insight into the potentially novel mechanism(s) of action of the innovative therapies and complementary approaches. This could entail analyses of imaging (e.g. MRI, ultrasound, nuclear imaging), as well as physiological, molecular, biochemical or omics signatures revealing potential perturbations prior to the intervention and recovery/improvement thereafter, and it could lead to the development of surrogate endpoints. This insight should open the path to more personalised interventions and approaches.
- Use and/or develop technologies, including digital ones (e.g. (generative) Artificial Intelligence - AI²⁹⁰, wearable technologies) to help implement and monitor the long-term efficacy of the intervention(s), as well as manage the disorder and/or monitor their progression (e.g. with unobtrusive technologies suitable for patient monitoring at home and in real-world conditions), whilst also ensuring they are bias-free, inclusive, and ethically sound.
- Utilise existing data, biobanks, registries and/or cohorts, together with the generation of new data that should be managed in line with the FAIR principles.
- Engage all relevant stakeholders (especially patients and patients' representatives for the disease, caregivers, clinicians, counsellors, regulators, etc.) to design end-user optimised interventions, applying gender-sensitive and intersectional approaches.
- Advance research by leveraging already existing and emerging state-of-the-art research infrastructures (e.g. EuroBioImaging²⁹¹, European Genomic Data Infrastructure²⁹², ECRIN²⁹³, EATRIS²⁹⁴, EBRAINS²⁹⁵, BBMRI²⁹⁶, etc.), as well as results stemming from EU-supported research projects, where applicable²⁹⁷.

²⁸⁸ Rare diseases, as defined by the European Union Regulation on Orphan Medicinal Products (1999), being a disease that affects not more than 1 person per 2000 in the European population (<https://www.orpha.net/>).

²⁸⁹ For pharmacological-based interventions: phase 1 and/or phase 2 clinical trials.

²⁹⁰ Generative AI is a type of AI technology that can generate various forms of new content such as text, images, sounds, and even code, such as for programming or gene sequencing (<https://ec.europa.eu/newsroom/dae/redirection/document/101621>).

²⁹¹ <https://www.eurobioimaging.eu>

²⁹² <https://gdi.onemilliongenomes.eu>

²⁹³ <https://ecrin.org>

²⁹⁴ <https://eatris.eu>

²⁹⁵ <https://www.ebrains.eu>

²⁹⁶ <https://www.bbmri-eric.eu>

²⁹⁷ Consult databases e.g. CORDIS (<https://cordis.europa.eu>) & the JPND Research Database (<https://neurodegenerationresearch.eu/search-our-database>).

- Engage with national public health authorities and regulators to ensure a robust development pathway and further uptake of the intervention.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)²⁹⁸ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

Funded projects should liaise with the co-funded European Partnership for Brain Health²⁹⁹, covered by topic HORIZON-HLTH-2025-02-DISEASE-01: “European Partnership for Brain Health”.

The topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the involvement of SSH experts, institutions as well as the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

All projects funded under this topic are strongly encouraged to participate in networking and joint activities, as appropriate. Therefore, proposals should include a budget for the attendance to regular joint meetings and may consider covering the costs of any other potential joint activities without the prerequisite to detail concrete joint activities at this stage. The details of these joint activities will be defined during the grant agreement preparation phase.

Applicants should provide details of their clinical studies³⁰⁰ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

²⁹⁸ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

²⁹⁹ <https://cordis.europa.eu/project/id/101259512>, <https://www.brainhealth-partnership.eu>

³⁰⁰ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

Destination - Ensuring equal access to innovative, sustainable, and high-quality healthcare

Topics under this destination are directed towards the Key Strategic Orientation 2 “*The Digital transition*” and Key Strategic Orientation 3 “*A more resilient, competitive, inclusive, and democratic Europe*” of Horizon Europe’s strategic plan 2025-2027³⁰¹.

Research and Innovation supported under this destination should contribute to the following expected impact, set out in the strategic plan impact summary for the Health Cluster: *“healthcare systems provide equal access to innovative, sustainable and high-quality healthcare thanks to the development and uptake of safe, cost-effective and people-centred solutions. This is to be accompanied by management models focusing on population health, health systems resilience, and health equity and patient safety, and also improved evidence-informed health policies”*.

Health systems are affected by limitations in sustainability and resilience, and face inequalities in access to high-quality and acceptable healthcare services. Health systems need to become more effective, efficient, accessible, fiscally and environmentally sustainable, and resilient in order to cope with public health emergencies, support healthcare workforce, adapt to environmental challenges like climate change, and contribute to social justice and cohesion. The transformation and modernisation of health systems will remain an important challenge for many years to come, but it also holds a significant opportunity to generate evidence, leverage existing and emerging solutions, implement digital and data-driven innovation and develop more accessible, cost-effective, flexible and equitable health systems.

Research and Innovation under this destination should aim to support the transformation of healthcare systems ensuring fair and inclusive access to high-quality, acceptable, sustainable healthcare for all. Funded activities will focus on developing innovative, practical, scalable and financially sound solutions, that improve governance, provide decision-makers with new evidence, tools, and technologies, and ensure long-term fiscal, environmental and climate sustainability. A patient-centred approach should be adopted, improving patients’ health outcomes, empowering patients, fostering active dialogue among stakeholders (e.g. citizens, patients, caregivers, healthcare providers), and encouraging social innovation. Research and Innovation actions should prioritise supporting healthcare professionals and providers, ensuring they have the resources and tools needed to meet the diverse needs and preferences of citizens. Research and Innovation should facilitate scalable and transferable solutions that can be applied across different healthcare systems and national, regional, and local contexts. This should include generating knowledge that supports the transfer of solutions between countries, including measures to address health inequalities. Research and Innovation activities under this destination will contribute to, among other things, the European Care Strategy³⁰², the digital transformation of health and care in the EU³⁰³, the European Pillar of

³⁰¹ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/strategic-plan_en

³⁰² Communication from the European Commission on the European care strategy, COM(2022) 440, 7.9.2022

Social Rights^{304, 305}, the EU strategy on adaptation to climate³⁰⁶, the Pharmaceutical Strategy for Europe³⁰⁷, the European Health Data Space (EHDS)³⁰⁸, the Strategy for European Life Sciences³⁰⁹ and the European Green Deal³¹⁰. They align with the Commission's Political Guidelines for 2024-2029³¹¹, which include efforts to complete the European Health Union by promoting access for all to high-quality and affordable healthcare, fostering a resilient and innovative health ecosystem, and strengthening the competitiveness of the European Union³¹².

In this Work Programme part, the focus of this destination will be on public procurement of innovative solutions for integrated or personalised care, aiming to develop and test solutions that improve access to and provision of healthcare. It will also support personalised medicine approaches to reduce adverse drug reactions due to the administration of multiple medication, and research to identify and address low-value care in health and care systems, improving healthcare outcomes, efficiency, and fiscal sustainability.

To increase the impact of EU investments under Horizon Europe, the Commission encourages and supports cooperation among EU-funded projects to foster cross-fertilisation and synergies. This includes networking, joint activities such as workshops, knowledge exchange, best practices development, and joint communication activities. Synergies can be explored not only between projects funded under the same topic, but also between projects funded under other topics, Clusters or Pillars of Horizon Europe. For instance, collaborations may arise between projects related to European health research infrastructures (under Pillar I), the EIC³¹³ strategic challenges on health (under Pillar III), or across the Clusters of Pillar II such as Cluster “Culture, Creativity and Inclusive Society” focusing e.g. on the long-term sustainability of public health systems (e.g. economic and organisational models and measures for cost effectiveness and fiscal sustainability), or Cluster “Digital, Industry and Space” focusing on the digitalisation of the health sector, including the use of Artificial Intelligence (AI).

The Commission aims to foster synergies between Horizon Europe and other EU programmes. To this end, applicants are encouraged to explore the funding opportunities

³⁰³ Communication from the European Commission on enabling the digital transformation of health and care in the Digital Single Market; empowering citizens and building a healthier society, COM(2018) 233, 25.4.2018

³⁰⁴ https://employment-social-affairs.ec.europa.eu/policies-and-activities/european-pillar-social-rights-building-fairer-and-more-inclusive-european-union_en

³⁰⁵ Commission Communication on Artificial Intelligence for Europe; COM(2018) 237 final: <https://digital-strategy.ec.europa.eu/en/policies/european-approach-artificial-intelligence>; <https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=COM:2018:237:FIN>

³⁰⁶ https://climate.ec.europa.eu/eu-action/adaptation-climate-change/eu-adaptation-strategy_e

³⁰⁷ https://health.ec.europa.eu/medicinal-products/pharmaceutical-strategy-europe_en

³⁰⁸ https://health.ec.europa.eu/ehds-digital-health-and-care/european-health-data-space-regulation-ehds_en

³⁰⁹ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;

https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

³¹⁰ https://commission.europa.eu/strategy-and-policy/priorities-2019-2024/european-green-deal_en

³¹¹ https://commission.europa.eu/about/commission-2024-2029_en

³¹² https://commission.europa.eu/topics/eu-competitiveness_en

³¹³ <https://eic.ec.europa.eu>

available through the EU4Health Programme (2021-2027)³¹⁴, the EU's public health programme, as a means of capitalising on potential collaborations and maximising impact.

Expected impacts:

Proposals for topics under this destination should set out a credible pathway to contributing to ensuring access to innovative, sustainable, inclusive and high-quality healthcare, and more specifically to one or several of the following impacts:

- Health and social care services and systems have improved governance mechanisms, making them more effective, efficient, accessible, resilient, trusted and sustainable, from fiscal, organisational and environmental perspectives. This includes shifting from hospital-centred to community-based, people-centred and integrated healthcare structures, embedding technological innovations and prioritising health promotion and disease prevention and management.
- Healthcare providers are trained and equipped with the skills and competences needed for future healthcare systems that are modernised, digitally transformed and equipped with safe innovative tools, technologies and digital solutions for healthcare. This will involve better patient management, improved patient engagement and health outcomes, reorganised workflows, and improved resource management.
- Citizens play a key role in managing their own healthcare, informal carers (including unpaid carers) are fully supported (e.g. by preventing overburdening and economic stress) and the specific needs of groups in a vulnerable situation are recognised and addressed. This includes improved access to healthcare services, financial risk protection, timely access to quality healthcare services including essential medicines and vaccines.
- Health policy and systems adopt a holistic approach -considering individuals, communities, organisations, society- in evaluating health outcomes, public health interventions, healthcare organisation, and decision-making. They benefit from evidence based, scalable and transferable healthcare solutions (e.g. between countries and healthcare settings) including for addressing health inequalities and ensuring environmental and climate sustainability in the health sector.

The actions resulting from the topics under this destination will also create strong opportunities for synergies with actions stemming from the EU4Health programme, in particular contributing to the goals under the general objective “*protecting people in the Union from serious cross-border threats to health*” and specific objective 4 “*to strengthen health systems, their resilience and resource efficiency*”.

Legal entities established in China are not eligible to participate in both Research and Innovation Actions (RIAs) and Innovation Actions (IAs) falling under this destination. For

³¹⁴ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

additional information please see “Restrictions on the participation of legal entities established in China” found in the Annex B of the General Annexes of this Work Programme.

The protection of European communication networks has been identified as an important security interest of the Union and its Member States. Entities that are assessed as high-risk suppliers³¹⁵ of mobile network communication equipment (and any entities they own or control) are not eligible to participate as beneficiaries, affiliated entities and associated partners to topics identified as “subject to restrictions for the protection of European communication networks”. Please refer to the Annex B of the General Annexes of this Work Programme for further details.

Proposals are invited against the following topic(s):

HORIZON-HLTH-2026-01-CARE-01: Public procurement of innovative solutions for improving citizens' access to healthcare through integrated or personalised approaches

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 3.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 24.50 million.
<i>Type of Action</i>	Public Procurement of Innovative Solutions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health’s programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The specific conditions for actions with PCP/PPI procurements in section H of the General Annexes apply to grants funded under this

³¹⁵ Entities assessed as “high-risk suppliers”, are currently set out in the second report on Member States’ progress in implementing the EU toolbox on 5G cybersecurity of 2023 (NIS Cooperation Group, Second report on Member States’ progress in implementing the EU Toolbox on 5G Cybersecurity, June 2023) and the related Communication on the implementation of the 5G cybersecurity toolbox of 2023 (Communication from the Commission: Implementation of the 5G cybersecurity Toolbox, Brussels, 15.6.2023 C(2023) 4049 final).

	topic. Subject to restrictions for the protection of European communication networks.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Beneficiaries may provide financial support to third parties to ensure the deployment and impact of the project outcomes. The support to third parties can only be provided in the form of grants. The maximum amount to be granted to each third party is EUR 60 000. The specific conditions are described in General Annex H. PPI procurement costs are eligible. The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to several of the following expected outcomes:

- Patients and their carers, health authorities and health professionals will benefit from the deployment of innovative solutions, designed around actual clinical needs, that facilitate identification, integration or coordination of care, allowing for personalised, more accessible, inclusive and higher quality of health and care.
- Patients will benefit from personalised approaches, improved care experiences and health outcomes or are more engaged in their care and better equipped to make informed decisions on their health, in collaboration with health professionals.
- Health professionals will be better equipped with, and thus benefit from, improved means for diagnosis, care delivery and/or coordination, with multi-disciplinary approaches and closer patient engagement, thanks to new technologies.
- Health systems will improve their accessibility, coordination mechanisms, effectiveness, inclusivity and resilience, thanks to innovative solutions, with a better use of resources, thus stimulating organisational innovation, cultural transformation within hospitals, and European-level collaboration.

Scope: Public Procurement of Innovative Solutions (PPI)³¹⁶ can boost the wider market uptake of high impact innovations in health systems, while enhancing the tools available to

³¹⁶ <https://research-and-innovation.ec.europa.eu/strategy/support-policy-making/shaping-eu-research-and-innovation-policy/new-european-innovation-agenda/innovation-procurement/horizon-europe-funding->

providers and improving access to healthcare for citizens. This supports enhancement of social rights³¹⁷ and the European economic competitiveness by providing business opportunities and thus incentives to innovate. By acting as early adopters of innovative solutions, procurers can open up new growth markets for the European industry and small and medium-sized enterprises (SMEs)³¹⁸. Joint/collaborative demand-driven initiatives can help create economies of scale and facilitate the wider adoption of innovations in the health sector for the benefits of patients in need.

PPI actions target consortia of procurers with a similar need that want to procure together the deployment of innovative solutions for supporting integration of care or diagnostics for personalised medicine. This topic does not provide direct funding to developers, industry or research organisations to perform research and development. They will be able to respond to the call for tenders launched by consortia of procurers funded under this topic.

Proposals should specify which segment of the patient population they target, the specific organisational and/or technological innovations to be procured, and why the proposed innovative solutions would be fit for purpose adhering, when relevant, to the principles of integrated care³¹⁹ or personalised medicine³²⁰.

Examples of target groups that could be covered by this action are: patients at risk of vulnerability such as children and older/frail people with complex needs for health and social care; people with multi-morbidities or non-communicable diseases of high burden; people with both physical and mental health conditions; people living with rare diseases or cancer; persons with disabilities; other groups of patients in need of highly integrated and coordinated care. Proposals should pay attention to how gender and intersectional factors (e.g. caregiving responsibilities, work-related health disparities etc.) affect healthcare access and outcomes.

Proposals should demonstrate, with qualitative and quantitative indicators, how they contribute to the above expected outcomes, clearly describe the application of the principles of integrated care and personalised medicine in the deployed solutions, when relevant. This would also include embedding the innovation in the existing health systems, addressing gaps and avoiding overlaps, while fostering change management across organisations, professions and sectors.

³¹⁷ pcp-and-ppi_en For PPI executed by a group of procurers, the lead procurer should coordinate the preparation and implementation of one joint or several coordinated public procurements of innovative solutions, based on common specifications defined jointly by the buyers' group. Each PPI should focus on one concrete need identified as a common challenge that requires the deployment of innovative solutions. Projects that aim to implement a PPI should contain a preparation and execution stage.

³¹⁸ European Pillar of Social Rights: https://employment-social-affairs.ec.europa.eu/european-pillar-social-rights-20-principles_en

³¹⁹ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>
<https://integratedcarefoundation.org/nine-pillars-of-integrated-care> See also diagram:
https://cordis.europa.eu/docs/results/h2020/634/634288_PS/001-eur-selfie2020-infographic-implementation.png and relevant corresponding article:
<https://www.sciencedirect.com/science/article/pii/S0277953621000605?via%3Dihub>

³²⁰ https://health.ec.europa.eu/medicinal-products/personalised-medicine_en

Solutions envisaged within this action are for example digital solutions³²¹, including Artificial Intelligence (AI) elements, to facilitate delivery of integrated care across hospitals, primary care, Long-Term Care (LTC) facilities and home settings, or technologies that improve routine diagnosis and lead to personalised medicine approach with the health and care setting.

The actions should target first deployment of innovative solutions across different health and care jurisdictions in Europe by engaging public and/or private procurers from each participating country (at national, regional or local level) that have deployment responsibilities and budget control in the provision of health and care services. Procurers will specify, purchase and deploy solutions addressing their relevant and shared unmet needs, while engaging together in a supply and demand side dialogue. Proposals should be based on clearly identified user needs and well-structured deployment plans, explaining how the procurement of the innovative solutions will contribute to the expected outcomes and improve current practice. In addition, cost-effectiveness analyses as well as estimates of the wider economic impact are highly desirable.

Activities covered should include cooperation with policymakers to reinforce national/regional policy frameworks and policies, to raise awareness, for technical assistance and/or capacity building beyond the project, to mainstream PPI implementation and remove obstacles to introduce innovative solutions to the market.

A wide variety of settings are potentially relevant for the implementation of such innovative solutions, for example primary healthcare settings, hospitals, specialised centres, long-term care facilities and home settings. The involvement of end-users (including for analysing the impact of the deployed solutions on health professionals and patients across the care continuum) and the use of cross-sectorial approaches are necessary. When relevant, linkage with ongoing work at national level for the implementation of the European Health Data Space (EHDS)³²² is encouraged. Proposals are encouraged to consider, where relevant, the data, expertise and services offered by European research infrastructures³²³ in the health domain.

Transfer and adaptation of solutions and/or interventions from other sectors to healthcare is possible. The topic is open both to innovations bringing improvements mainly based on one specific solution/technology field, as well as to innovations delivering end-to-end solutions that need combinations of different types of innovative elements. Proposals are strongly encouraged to build upon past work and build synergies with ongoing EU-funded initiatives, for example the Joint Actions JADECARE³²⁴ and Xt-EHR³²⁵, the project

³²¹ For digital technologies concerned, appropriate measures for the security of the communications between the intended parties should be considered, in particular based on the use of post-quantum cryptography.

³²² https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

³²³ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

³²⁴ <https://www.jadecare.eu>

³²⁵ <https://www.xt-ehr.eu>

MyHealth@Myhands³²⁶ and the three co-funded European Partnerships on Transforming Health and Care Systems³²⁷, on Personalised Medicine³²⁸ and on Rare Diseases³²⁹, as well as with actions supported under the Technical Support Instrument and the Cohesion Policy Funds.

HORIZON-HLTH-2026-01-CARE-03: Identifying and addressing low-value care in health and care systems

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 10.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 38.00 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action,

³²⁶ <https://myhealthmyhands.eu>

³²⁷ <https://cordis.europa.eu/project/id/101095654>, <https://www.thcspartnership.eu>

³²⁸ <https://cordis.europa.eu/project/id/101137129>, <https://www.epparmed.eu>

³²⁹ <https://cordis.europa.eu/project/id/101156595>, <https://erdera.org>

	object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Healthcare providers and policymakers make use of evidence-based indicators and methodologies to identify low-value care³³⁰ practices, as well as opportunities for improvement and tools to monitor such improvements.
- Healthcare professionals are equipped with the knowledge and tools to implement guidelines for reducing or discontinuing low-value care activities and maintaining effective and patient-centred practices that ensure quality of care.
- Patients and citizens benefit from more effective healthcare, by understanding and endorsing measures that reduce low-value care, recognising the potential to achieve higher-quality healthcare and better health outcomes overall.
- Health and care systems benefit from a reduction of low-value care practices, which enables enhanced patient safety and quality of care, while contributing to their efficiency as well as fiscal and environmental sustainability.
- Healthcare organisations can, by identifying low-value care practices, reallocate valuable healthcare resources to other areas of need.

Scope: Low-value care, as defined in the footnote, can have widespread negative consequences for patients, caregivers, healthcare professionals, the health and care system, and the broader environment. A 2017 OECD report³³¹ estimated that “wasteful healthcare spending is common” and that “up to one-fifth of healthcare spending could be redirected towards better uses”. Low-value care represents a significant challenge, contributing to waste, costs, misuse of resources, and inefficiencies. Addressing low-value care can free up and allow reallocation of valuable healthcare resources to other areas of need, thereby maximising health outcomes, improving health and care systems resilience, and reducing their environmental impact. In this context, a recent report³³² by the Expert Group on Health

³³⁰ Definition of low-value care from the Report by the Expert Group on Health Systems Performance Assessment: “From a health system perspective, low-value care encompasses overuse, misuse and underuse of healthcare services (for example, prevention, diagnostics, treatment, medication). Overuse and/or misuse comprise the delivery of harmful, ineffective, inappropriate, or not cost-effective healthcare services. Underuse refers to healthcare services not provided or used despite being necessary. Low-value care can lead to negative consequences for patients, their caregivers, the healthcare workforce, the health system as a whole and the wider environment.”

³³¹ OECD (2017), Tackling Wasteful Spending on Health, OECD Publishing, Paris. <http://dx.doi.org/10.1787/9789264266414-en>

³³² Report by the Expert Group on Health Systems Performance Assessment: “Identifying, Measuring And Reducing Low-Value Care In The Context Of Health System Performance Assessment”.

Systems Performance Assessment (HSPA)³³³ establishes the methodological basis and metrics to identify, measure and reduce low-value care.

Research activities under this topic should adopt a patient-centred approach that considers the needs and preferences of patients and citizens. They should promote socially acceptable solutions, taking into account relevant ethical, social and legal aspects and foster dialogue and collaboration between policymakers, healthcare providers, healthcare professionals, and patients/citizens. Proposals should engage citizens and civil society organisations in the development of their actions to ensure acceptability of solutions. By doing so the projects will contribute to better use of healthcare resources -including time and personnel- in ways that significantly improve patient outcomes and alleviate the increasing burden on healthcare professionals and health systems. Implementation research and multidisciplinary approaches should be considered to foster adoption and ensure effective interventions and long-term sustainability.

Proposed activities may³³⁴ include clinical studies³³⁵ to provide evidence on the value of any interventions or processes and, therefore, facilitate justified removal of any type of low value care. Proposed activities may also include data models, digital and artificial intelligence-based analysis, models and/or tools to identify and/or address low-value care. Proposed activities may examine the design and impact of healthcare payment systems, that could unintentionally incentivise low-value care and evaluate alternative financing models that better align incentives with patient outcomes and high-value care. Proposed activities may also facilitate or implement collaboration among registries (disease registries such as cancer registries, primary healthcare visits registries, prescription and drug purchase registries, reimbursement and medical devices registries, screening databases, socio-economic and census databases, etc.) across regions or countries, to enable or improve the assessment and comparison of different levels of care and their value to patients. Additionally, activities that facilitate learning and best practice transfers between countries or regions may also be considered as element of the proposal (for instance, to leverage best practice-sharing initiatives from international platforms such as the Knowledge Hub of the co-funded European Partnership on Transforming Health and Care Systems³³⁶ or any other relevant European or global initiatives). Additionally, proposals may include or support international comparisons of low-value care practices and strategies for their reduction across countries, if and where deemed valuable.

Research actions should address all the following objectives:

- Develop a deeper understanding of how low-value care can be identified and measured throughout the healthcare process, including testing related indicators and producing

https://health.ec.europa.eu/publications/identifying-measuring-and-reducing-low-value-care-context-health-system-performance-assessment_en

³³³ https://health.ec.europa.eu/health-systems-performance-assessment_en

³³⁴ Some proposals may not need to conduct clinical studies to achieve the objectives.

³³⁵ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

³³⁶ <https://cordis.europa.eu/project/id/101095654>, <https://www.thcspartnership.eu>

evidence-based methodologies that enable the pursuit of improved efficiency and quality of care.

- Identify instances of overuse, misuse, underuse and unwarranted variation in specific healthcare contexts across different stages of the healthcare process. This analysis should provide actionable insights for policymakers, healthcare providers and healthcare professionals to evaluate the potential of possible strategies for reducing low-value care, allowing for more informed decision-making and improved care practices.
- Develop and/or pilot innovative strategies for effective reduction of low-value care in specific settings across the care pathway. These pilots should demonstrate scalability and transferability across diverse health and care systems in Europe.

Proposals should consider how gender norms and roles influence utilisation patterns, ensuring that strategies to reduce low-value care do not inadvertently exacerbate existing gender and social inequalities in healthcare access and outcomes. In addition, attention should be paid to intersectional factors that may further affect healthcare access and outcomes. If handling data and indicators, sex- and gender-disaggregated data should be collected and analysed, incorporating intersectional factors where feasible.

Proposals should consider the work and output of any EU level initiatives (e.g. the Expert Group on Health Systems Performance Assessment, the co-funded European Partnership on Transforming Health and Care Systems, relevant projects or Joint Actions funded under the EU4Health Programme (2021-2027)³³⁷ and under EU Research & Innovation Framework Programmes, etc.) or other international initiatives (e.g. the 2017 OECD report mentioned above) in this area.

Applicants envisaging³³⁸ to include clinical studies³³⁹ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

HORIZON-HLTH-2026-04-CARE-04: Enhancing and enlarging the European Partnership on Personalised Medicine (EP PerMed) (Top-up)

Call: Partnerships in Health (2026/3)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 9.80 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 9.80 million.

³³⁷ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

³³⁸ Some proposals may not need to conduct clinical studies to achieve the objectives.

³³⁹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

<i>Type of Action</i>	Programme Co-fund Action
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: The proposal must be submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine”. This eligibility condition is without prejudice to the possibility to include additional partners. In recognition of the opening of the US National Institutes of Health’s programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. Because the US contribution will be considered for the calculation of the EU contribution to the partnership, the concerned consortium of research funders from eligible EU Members States and Associated Countries must expressly agree to this participation.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: The evaluation will take into account the existing context and the scope of the initial evaluation as relevant, and related obligations enshrined in the grant agreement. If the proposal is successful, the next stage of the procedure will be grant agreement amendment preparations. If the outcome of amendment preparations is an award decision, the coordinator of the consortium funded under topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine” will be invited to submit an amendment to the grant agreement, on behalf of the beneficiaries.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: This action is intended to be implemented in the form of an amendment of the grant agreement concluded pursuant to Article 24(2) of the Horizon Europe Regulation. For the additional activities covered by this action: • The funding rate is 30% of the eligible costs.

	- Beneficiaries may provide financial support to third parties (FSTP). The support to third parties can only be provided in the form of grants. As a co-funded European Partnership, providing financial support to third parties (FSTP) is a core activity of this action in order to achieve its objectives. Consequently, the EUR 60 000 threshold laid down in Article 207 of Financial Regulation (EU, Euratom) 2024/2509 does not apply. The maximum amount of FSTP that may be awarded to any single third party for the duration of the partnership is set at EUR 10.00 million. This ceiling is justified by the fact that FSTP is a primary activity of this action, by its expected duration of 7-10 years (exceeding a standard project lifespan), and by the extensive experience gained under predecessor partnerships. This ceiling is also justified by the fact that personalised medicine being at the forefront of medical approaches, requires the use of state-of-the-art technologies and tailored clinical trials which are usually expensive. However, if the objectives of the action would otherwise be impossible or overly difficult (and duly justified in the proposal) the maximum amount may be higher. - The starting date of the grant awarded under this topic may be as of the submission date of the application. Applicants must justify the need for a retroactive starting date in their application. Costs incurred from the starting date of the action may be considered eligible (and will be reflected in the entry into force date of the amendment to the grant agreement). The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.
<i>Total indicative budget</i>	The total indicative budget for the duration of the co-funded partnership is EUR 109.8 million.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- European countries and regions, along with international partners, are engaged in enhanced collaborative research efforts for the development of innovative personalised medicine approaches regarding prevention, diagnosis and treatment.

- Healthcare authorities, policymakers and other stakeholders develop evidence-based strategies and policies for the uptake of personalised medicine in national or regional healthcare systems.
- Health industries, policymakers and other stakeholders have access to efficient measures and investments to allow swift transfer of research and innovation into market.
- Health industries and other stakeholders can accelerate the uptake of personalised medicine through the adoption of innovative business models.
- Healthcare authorities, policymakers and other stakeholders use improved knowledge and understanding of the health and costs benefits of personalised medicine to optimise healthcare and make healthcare systems more sustainable.
- Healthcare providers and professionals improve health outcomes, prevent diseases and maintain population health through the implementation of personalised medicine.
- Stronger and highly connected local/regional ecosystems of stakeholders, including innovators, are in place and facilitate the uptake of successful innovations in personalised medicine, thus improving healthcare outcomes and strengthening European competitiveness.
- Citizens, patients and healthcare professionals have a better knowledge of personalised medicine and are better involved in its implementation.
- Stakeholders cooperate better and establish a network of national and regional knowledge hubs for personalised medicine.

Scope: This topic targets an action under Article 24(2) HE Regulation aiming to add additional activities to existing grant agreements, together with additional partners that would deliver on those activities. The award of a grant to continue the partnership in accordance with this call should be based on a proposal submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine” and the additional activities and additional partners to be funded by the grant should be subject to an evaluation. Taking into account that the present action is a continuation of the topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine” and foresees an amendment to an existing grant agreement, the proposal should present the additional activities (including additional partners) to be covered by the award primarily in terms of grant agreement revisions. Partners from countries recently associated to Horizon Europe from 2024 onwards (2024 included) are particularly welcome. The existing action, the “European Partnership for Personalised Medicine” (EP PerMed) can only reasonably be enhanced and enlarged on the basis of the existing consortium³⁴⁰, as the co-funded framework established cannot simply be replaced without significant disruption,

³⁴⁰ Consortium which was awarded the grant under topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine”.

given the top-quality, long-term expertise and wide coverage of the beneficiaries comprising this consortium.

The additional activities to be performed by applicants under this topic should consist of several of the following:

- Organisation of activities or tools according to their expertise and interests, e.g.:
 - o Personalised Medicine (PM) Innovation related activities and tools, business and entrepreneur relations and support, case studies and guides.
 - o PM public health and social care, people's engagement, activities to support health system's ability to turn scientific discoveries into new or improved treatments and services, support the scientific community to tackle complex health and social care challenges, international outreach.
 - o PM and diversity, underrepresented populations, gender aspects, health data and knowledge mobilisation activities, PM and rare diseases.
 - o PM related genomics, expert and societal exchange on genomics, opportunities by genomics for innovations and economic growth.
- Contribution to the design and implementation of the specific topics and features of the Transnational Joint Calls as of 2026 to which new partners will contribute national commitments.
- Specific, tailored contributions to other EP PerMed calls such as: Fast Track, Venture Creation Programme, Networking, Twinning calls, Call for surveys, Education calls, etc.
- Organisation of specific EP PerMed events, such as in-situ visits (Work Package 5 - WP5), summer schools (WP3/4).
- Contribution to the development and dissemination of strategic documents in additional geographical areas, for example the Strategic Research and Innovation Agenda (SRIA) updates.
- Development and implementation of other new PM tailored activities within the related WPs.

HORIZON-HLTH-2027-01-CARE-02: Personalised approaches to reduce risks from Adverse Drug Reactions due to administration of multiple medications

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per</i>	The Commission estimates that an EU contribution of between EUR 8.00 and 10.00 million would allow these outcomes to be addressed

<i>project</i>	appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 38.00 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health’s programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Patients benefit from decreased incidence of Adverse Drug Reactions (ADRs) caused by the administration of multiple medications (three or more medicinal products³⁴¹) and enhanced health outcomes by ensuring safer and more effective use of medication.
- Healthcare professionals can adopt adverse drug reactions prevention and reduction strategies to integrate genetic and other biomarker information into clinical decision-making to optimise the use of medication, especially in situations of comorbidities.

³⁴¹

<https://www.ema.europa.eu/en/glossary-terms/medicinal-product>

- Healthcare systems benefit from cost savings thanks to reduced hospital admissions and other costs associated with ADRs related to the intake of multiple medicines.
- Clinical and regulatory guidelines and policies for medication management in case of multiple medications can be revised supported by robust evidence.
- Educational programs for healthcare providers and patients benefit from improved awareness and management of polypharmacy and ADRs.

Scope: While medicinal products contribute considerably to the health of EU citizens, they can also have adverse reactions. It is estimated that around 5% of all hospital deaths are due to an adverse drug reaction. On average, 16% of hospitalised older³⁴² patients experience significant ADRs, varying in severity and mostly preventable, with commonly prescribed drug classes (such as diuretics, anti-bacterials, antithrombotic agents, analgesics, antineoplastics, etc.) accounting for most ADRs³⁴³. Overall, ADRs increase morbidity, mortality, hospitalisations, and healthcare costs.

ADRs from multiple medications contribute significantly to healthcare costs due to increased hospitalisations and treatments, making this an area of focus to achieve cost efficiency.

Initial failure to recognise ADRs can generate inappropriate prescription cascades, in which the side effects of drugs are misdiagnosed as symptoms of new problems, resulting in further prescriptions and further side effects that tend to accumulate, confusing and complicating the diagnostic while aggravating the evolution. Therefore, there is a distinct need for research to help identify and prevent such prescription cascades, possibly by maximising the use of technology, as well as to improve multiple drug management in order to reduce patient harm. Furthermore, it is also possible that aside from the ADRs specific to individual drugs taken in combination, new ADRs can emerge as results from the drug combinations themselves.

Research activities under this topic should make use of the constantly improving health technologies and data analytics that provide new opportunities to address these issues more effectively, by better integrating medication management into healthcare practices, including into Electronic Health Records (EHR) and decision support systems.

Identifying and validating relevant biomarkers for better patient stratification can contribute to significantly decreasing the risk of adverse drug reactions. Biomarkers can also help to detect adverse drug reactions early before occurrence of clinical symptoms and enable early countermeasures. Generating knowledge on the interaction and complexity of biochemical pathways can improve the understanding of patients' response to ADRs and thus provide better tailored treatments and early responses to adverse reactions.

For this purpose, any biomedical strategy that allows a better stratification of patients to identify drug response patterns in well-defined patient groups could be used, including in-

³⁴² Old age is often defined as starting around 60 or 65 years of age.

³⁴³ Emma L. M. Jennings et al., In-hospital adverse drug reactions in older adults; prevalence, presentation and associated drugs - a systematic review and meta-analysis, *Age and Ageing* 2020; 49: 948-958 doi: 10.1093/ageing/afaa188

vitro or in-silico models for adverse drug reactions, imaging biomarkers, drug-drug/drug-gene/drug-food interactions, therapeutic dose reduction and pharmaco-exposomics, nutrition and beverage interference, smoking, vaping, pollution etc. De-escalation studies in view of improving multiple drug management can be also considered. Proposals should be sufficiently robust to examine differences across various populations, and also consider sex difference in drug reactions.

The further use of results generated by the projects funded under this topic should be ensured through data sharing with the relevant stakeholders and the European Medicines Agency (EMA), in view of possible adoption of deprescribing or adjusted-prescribing guidelines by relevant authorities at EU and national levels.

Where applicable, applicants are strongly encouraged to follow all relevant guidelines in the relevant scientific fields, including but not limited to:

- Joint EMA/Heads of Medicines Agencies (HMA)/EC Workshop recommendations on pharmacogenomics in medicines regulation and on implementation into clinical practice³⁴⁴.
- Pharmaceutical development of medicines for use in the older population, Scientific guideline from the EMA³⁴⁵.
- Guidelines from the Clinical Pharmacogenetics Implementation Consortium ('CPIC guidelines')³⁴⁶.

Proposals funded under this topic should address all the following aspects:

- Leverage the role of pharmacogenomics, pharmacokinetics and pharmacodynamics in predicting and preventing adverse drug reactions in situations of multiple medications (three or more drugs administered concomitantly), and propose personalised medicine approaches, such as targeted therapies and biomarker-driven treatment strategies, to reduce the rate of adverse drug reactions and limit multiple medications.
- Maximise the use of technology, such as electronic health records, artificial intelligence and clinical decision support systems, to support safe medication use and prevent adverse drug reactions.
- Address the ethical, regulatory, and implementation challenges associated with integrating personalised medicine into clinical practice to address adverse drug reactions due to the administration of multiple medications.

³⁴⁴ https://www.ema.europa.eu/en/documents/report/report-joint-ec-hma-ema-multi-stakeholder-workshop-pharmacogenomics-24-september-2024_en.pdf

³⁴⁵ <https://www.ema.europa.eu/en/pharmaceutical-development-medicines-use-older-population-scientific-guideline>

³⁴⁶ <https://cpicpgx.org/guidelines>

- Generate evidence on the clinical utility and cost-effectiveness of treatment guided by pharmacogenomics and other relevant biomarkers-based approach, for single drugs and for combinations of drugs.
- Develop and implement strategies, including regulatory science approaches, for efficient integration of project results into daily healthcare.
- Align with similar work in other EU-funded projects or partnerships, such as the co-funded European Partnership for Personalised Medicine³⁴⁷, the co-funded European Partnership on Transforming Health and Care System³⁴⁸ etc. while avoiding any potential overlaps.

The participation of start-ups, micro, small and medium-sized enterprises (SMEs)³⁴⁹ is encouraged with the aim of strengthening their scientific and technological foundations, enhancing their innovation potential, and exploring possibilities for commercial exploitation.

Applicants should provide details of their clinical studies³⁵⁰ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

³⁴⁷ <https://cordis.europa.eu/project/id/101137129>, <https://www.eppermed.eu>

³⁴⁸ <https://cordis.europa.eu/project/id/101095654>, <https://www.thcspartnership.eu>

³⁴⁹ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

³⁵⁰ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

Destination - Developing and using new tools, technologies and digital solutions for a healthy society

Topics under this destination are directed towards the Key Strategic Orientation 2 “*The Digital Transition*” and Key Strategic Orientation 3 “*A More Resilient, Competitive, Inclusive, and Democratic Europe*” of Horizon Europe’s strategic plan 2025-2027³⁵¹.

Research and Innovation supported under this destination should contribute to the following expected impact, set out in the strategic plan impact summary for the Health Cluster: “*Health technologies, data, new tools, and digital solutions are applied effectively thanks to their inclusive, ethically sound, secure and sustainable delivery, integration and deployment in health policies and in health and care systems.*”

The Health Cluster will continue to drive the development and adoption of innovative technologies and digital solutions to improve healthcare and health systems. This will ensure that the EU remains at the forefront of breakthrough health and medical technologies and can achieve open strategic autonomy in essential medical supplies and digital innovations.

In line with the Commission's Political Guidelines for 2024-2029³⁵², this destination will support research and innovation in tools and technologies strengthening the competitiveness of European health industry and reinforcing EU autonomy. This effort will contribute to the completion of the European Health Union which aims to enhance the resilience of healthcare systems, facilitate access to innovative and affordable healthcare solutions, and ensure that all citizens have access to high-quality, equitable, inclusive and sustainable healthcare.

The development and use of innovative tools and technologies for biomedical research are the basis for prevention, early diagnosis, efficacious therapy and patient monitoring, essential components of efficient healthcare. These include enabling technologies, not least innovative biotechnological approaches, and emerging technologies like synthetic biology, digital tools including those based on Machine-Learning/Artificial Intelligence (ML/AI) and other data-driven approaches which will enable the development of more personalised medicine. Hence the combination of innovative tools, high-quality health data (incl. Real-World Data - RWD³⁵³), digital technologies, modelling and AI tools holds great potential not only for advancing biomedical Research and Innovation but for developing health technologies that improve healthcare.

However, the implementation of these tools and technologies faces specific barriers such as scalability, regulatory frameworks and public acceptance and trust. To overcome these challenges cross-sectoral cooperation among stakeholders including researchers, regulatory bodies, policymakers, industry, healthcare providers and patients, is necessary. This collaboration will facilitate the design and development of innovative health products and

³⁵¹ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/strategic-plan_en

³⁵² https://commission.europa.eu/about/commission-2024-2029_en

³⁵³ EMA definition: “Real-World Data are routinely collected data relating to patient health status or the delivery of healthcare from a variety of sources other than traditional clinical trials (e.g. claims databases, hospital data, electronic health records, registries, mhealth data, etc.)”.

services, tailored to specific population groups, ultimately improving patient outcomes and reducing health inequalities.

By taking a comprehensive and inclusive approach, this destination will prioritise the development of novel tools and technologies that address key considerations such as the rights of the individual, safety, effectiveness, appropriateness, accessibility, comparative value-added and fiscal sustainability while also ensuring ethical, legal and regulatory compliance.

In this Work Programme part, Destination “*Developing and using new tools, technologies and digital solutions for a healthy society*” is driven mainly by three key Commission policies, the “Biotechnology and Biomanufacturing Communication”³⁵⁴ the “Artificial Intelligence Strategy”³⁵⁵ and the “Strategy for European Life Sciences”³⁵⁶ and focuses on developing and applying innovative technologies to improve human health and healthcare systems. The topics under this destination cover efforts to develop AI based predictive biomarkers for disease prognosis and treatment response, advancing bio-printing of living cells for regenerative medicine, and integrating New Approach Methodologies (NAMs) to advance biomedical research, as well as developing virtual human twins for integrated clinical decision support.

To increase the impact of EU investments under Horizon Europe, the Commission encourages cooperation between EU-funded projects to enable cross-fertilisation and other synergies. For example, this cooperation could take the form of networking, to joint activities, such as the participation in joint workshops, exchange of knowledge, development and adoption of best practices, or joint communication activities. Opportunities for such activities and potential synergies exist between projects funded under the same topic but also between other projects funded under different topics, Clusters or Pillars of Horizon Europe. Specifically, this could involve projects related to European health research infrastructures (under Pillar I of Horizon Europe), the EIC³⁵⁷ strategic challenges on health (under Pillar III of Horizon Europe) or with projects on themes that cut across the Clusters of Pillar II such as with Cluster “Digital, Industry and Space” on digitalisation of the health sector or key enabling technologies.

Expected Impacts:

Proposals for topics under this destination should set out a credible pathway towards the development and use of new tools, technologies and digital solutions for a healthy society, and more specifically to one or several of the following impacts:

³⁵⁴ Commission Communication on Building the future with nature: Boosting Biotechnology and Biomanufacturing in the EU; COM(2024) 137 final: https://research-and-innovation.ec.europa.eu/document/download/47554adc-dffc-411b-8cd6-b52417514cb3_en

³⁵⁵ Commission Communication on Artificial Intelligence for Europe; COM(2018) 237 final: <https://digital-strategy.ec.europa.eu/en/policies/european-approach-artificial-intelligence>; <https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=COM:2018:237:FIN>

³⁵⁶ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;

³⁵⁷ https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686
<https://eic.ec.europa.eu>

- Europe's scientific and technological expertise and know-how, its capabilities for innovation in new tools, technologies and digital solutions, and its ability to take-up, scale-up and integrate innovation in healthcare is world-class.
- Citizens benefit from targeted and faster research resulting in safer, more sustainable, efficient, cost-effective, accessible and affordable tools, technologies and digital solutions for improved (personalised) disease prevention, diagnosis, treatment and monitoring for better patient outcome and wellbeing, in particular through increasingly shared health resources (interoperable data, infrastructure, expertise, citizen/patient driven co-creation)³⁵⁸.
- The EU gains high visibility and leadership in terms of health technology development, including through international cooperation.
- The burden of diseases in the EU and worldwide is reduced through the development and integration of innovative diagnostic and therapeutic approaches, personalised medicine approaches, digital and other people-centred solutions for healthcare.
- Both the productivity of health Research and Innovation, and the quality and outcome of healthcare is improved thanks to the use of health data and innovative analytical tools, such as AI supported decision-making, in a secure, ethical and inclusive manner, respecting individual integrity and underpinned with public acceptance and trust.
- Citizens trust and support the opportunities offered by innovative technologies for healthcare, based on expected health outcomes and potential risks involved.

Legal entities established in China are not eligible to participate in both Research and Innovation Actions (RIAs) and Innovation Actions (IAs) falling under this destination. For additional information please see "Restrictions on the participation of legal entities established in China" found in the Annex B of the General Annexes of this Work Programme.

The protection of European communication networks has been identified as an important security interest of the Union and its Member States. Entities that are assessed as high-risk suppliers³⁵⁹ of mobile network communication equipment (and any entities they own or control) are not eligible to participate as beneficiaries, affiliated entities and associated partners to topics identified as "subject to restrictions for the protection of European communication networks". Please refer to the Annex B of the General Annexes of this Work Programme for further details.

Proposals are invited against the following topic(s):

³⁵⁸ Commission Communication on the digital transformation of health and care; COM(2018) 233 final
³⁵⁹ Entities assessed as "high-risk suppliers", are currently set out in the second report on Member States' progress in implementing the EU toolbox on 5G cybersecurity of 2023 (NIS Cooperation Group, Second report on Member States' progress in implementing the EU Toolbox on 5G Cybersecurity, June 2023) and the related Communication on the implementation of the 5G cybersecurity toolbox of 2023 (Communication from the Commission: Implementation of the 5G cybersecurity Toolbox, Brussels, 15.6.2023 C(2023) 4049 final).

HORIZON-HLTH-2026-01-TOOL-03: Integrating New Approach Methodologies (NAMs) to advance biomedical research and regulatory testing

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 5.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 49.00 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B. Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: In order to maximise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities (and in determining modalities for their implementation and the specific responsibilities of projects). These activities will be included in a dedicated work package, having sufficient budget allocated to it (around

	2% of the total requested budget). Depending on the scope of proposals selected for funding, these activities may include: • Attendance of regular joint meetings (e.g. common kick-off meeting and annual meetings). • Periodic report of joint activities (delivered at each reporting period). • Common dissemination and communication activities (which may include, for example: a common dissemination and communication strategy, web portal and visual identity, brochure, newsletters). • Common Data Management Strategy and Common Policy Strategy (including joint policy briefs). • Thematic workshops/trainings on issues of common interest. • Working groups on topics of common interest (e.g. data management and exchange, communication and dissemination, science-policy link, scientific synergies). The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025)³⁶⁰.
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Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to several of the following expected outcomes:

- Researchers are in possession of improved human-relevant New Approach Methodologies (NAMs) platforms that capture the genetic, phenotypic, age-related, immune, microbiome, and environmental exposure variability of the human population.

³⁶⁰ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

These innovations support more equitable healthcare solutions and personalised treatment strategies across diverse life stages.

- Industry gets access to platforms that allow a faster pace of innovation for the development of more cost-effective targeted therapeutic interventions and improvement of the safety assessment of chemicals, other medicinal products, and medical devices.
- Patients benefit from innovative platforms and strategies that improve prediction, prevention and treatment of diseases, in particular through enhanced understanding of disease pathways and mechanisms.
- The general population is better protected through a safer environment, as these platforms enhance the detection and mitigation of risks posed by chemicals and other potentially harmful substances.
- Regulatory bodies gain confidence and trust in NAMs, supporting their integration into product development, risk assessment, and approval processes.
- Fewer live animals are used in biomedical research and regulatory testing.

Scope: This topic aims to support the ongoing paradigm shift in biomedical research and safety assessment of chemical compounds by fully integrating NAMs across the entire research and regulatory spectrum, from basic discovery phase to clinical application, and regulatory testing of medicinal products and medical devices, and/or industrial and environmental chemicals.

NAMs include a wide range of innovative and human-relevant technologies such as in-vitro or human ex-vivo assays, organoids, Organ-on-Chip (OoC) systems, human tissue models, induced Pluripotent Stem Cell (iPSC) applications, virtual twin tools, in-silico methods, and Artificial Intelligence (AI)-driven modelling.

Although the Commission and several Member States have supported the development of NAMs for over two decades, primarily in the context of chemical risk assessment, regulatory uptake remains limited. There is a need to address this situation by delivering validated NAMs solutions that can be adopted by industry and accepted by the regulators for the safety assessment of chemicals. In parallel, there is a growing readiness to expand the development and application of NAMs across the entire biomedical research spectrum, from early discovery through to clinical translation and regulatory testing of medicinal products and medical devices.

Proposals should bring together stakeholders from academia, health-related infrastructures, SMEs³⁶¹, industry, and regulators to develop new NAMs platforms or improve existing ones that could be used for biomedical applications and/or regulatory testing. For biomedical applications, these platforms should enhance disease modelling precision, especially in areas where current animal models are of limited human relevance, and where NAMs could

³⁶¹ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

effectively complement or replace animal studies. For proposals addressing regulatory use, in particular the safety assessment of chemicals, other medicinal products and medical devices, the intended context(s) of use should be clearly defined, with validation strategies and methodologies aligned with current OECD and/or European Medicines Agency (EMA) guidance. Early, proactive, and sustained engagement with regulators should also be demonstrated.

Proposals should develop or optimise scalable and reproducible platforms based on one or more of the following:

- Advanced in-vitro assays.
- iPSC-based models, organoid or complex OoC systems derived from patients and/or healthy donors.
- Human tissues that closely replicate physiological and pathological conditions.

In order to enable real-time monitoring of physiological responses, proposals should consider integration of embedded sensors. They should also address biological diversity, reflecting variations in genetics, phenotype, age, immune status, and microbiome across the population.

Moreover, proposals may incorporate one or both of the following complementary approaches to enhance predictive power and clinical relevance:

- AI-driven predictive modelling trained on high-quality, curated, bias-minimised datasets to predict outcomes of biomedical interventions, or risk assessment.
- Virtual twin technology to simulate disease progression, responses to interventions, and support the optimisation of clinical trials.

To maximise scientific impact, interoperability, and reuse, all data generated should comply with FAIR³⁶² principles. Proposals should describe how data will be curated, standardised, and shared within or linked to the European Health Data Space (EHDS)³⁶³ or other repositories, and/or relevant ESFRI³⁶⁴ research infrastructures.

In order to optimise synergies and increase the impact of the projects, all proposals selected for funding from this topic will form a cluster and be required to participate in common networking and joint activities.

Proposals should consider involving the European Commission's Joint Research Centre (JRC), including its EU Reference Laboratory for alternatives to animal testing (EURL

³⁶² See definition of FAIR data in the introduction to this Work Programme part.

³⁶³ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

³⁶⁴ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

ECVAM)³⁶⁵, to take advantage of its expertise and relevant activities in bridging research and application communities and facilitating uptake of NAMs in biomedical research and regulatory testing. In that respect, the JRC should collaborate with any successful proposal and this collaboration should be established after the proposal's approval.

Applicants envisaging to include clinical studies³⁶⁶ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

HORIZON-HLTH-2026-01-TOOL-05: Pilot actions for follow-on funding: Leveraging EU-funded collaborative research in regenerative medicine

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 6.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 29.50 million.
<i>Type of Action</i>	Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: The proposals must be based on results generated within a prior multi-beneficiary project funded under Horizon 2020 or Horizon Europe Framework Programme. This project must have been completed maximum 3 years before the submission deadline. Applicants must explicitly state in their proposal the prior multi-beneficiary project concerned. Projects funded under Marie Skłodowska-Curie Actions are not considered eligible. Projects funded under co-funded European Partnerships or ERANETs are considered eligible. Ongoing projects³⁶⁷ are not considered eligible. In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of

³⁶⁵ https://joint-research-centre.ec.europa.eu/projects-and-activities/reference-and-measurement/european-union-reference-laboratories/eu-reference-laboratory-alternatives-animal-testing-eurl-ecvam_en

³⁶⁶ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

³⁶⁷ Projects that have not been completed before the submission deadline.

	Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). In line with the “<i>restriction on control in innovation actions in critical technology areas</i>” delineated in General Annex B of the General Annexes, entities established in an eligible country but which are directly or indirectly controlled by China or by a legal entity established in China are not eligible to participate in the action.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ³⁶⁸.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The overall competitiveness of the EU biotechnology sector is strengthened through the further development of closer-to-deployment health innovations.
- The EU benefits from greater impact of the EU’s Research and Innovation (R&I) Framework Programmes through successful leveraging of previous EU funding in the field of regenerative medicine.
- EU innovators secure further funding to finalise the last stages of development.
- Patients benefit faster from solutions that improve their health and wellbeing.

³⁶⁸ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

Scope: R&I is essential for economic growth and boosting the competitiveness of the EU's life sciences sector. Through the Horizon 2020 and Horizon Europe Framework Programmes, the EU has supported projects that significantly impact our health by fostering scientific discoveries and developing new solutions. Transformational health innovations, such as mRNA vaccines, highlight the importance of collaboration among businesses, research institutions, and healthcare providers. Furthermore, sustained funding throughout the entire value chain is crucial for maximising impact and ensuring more products reach patients faster. The main aim of this topic is to pilot a follow-on funding mechanism, supporting the stepwise development of biotech innovations through collaboration, resulting from previously supported EU R&I actions in the field of health. This topic contributes to strengthening the R&I ecosystem within the EU and supports the implementation of the “Strategy for European Life Sciences”³⁶⁹. Given the importance of biotechnology as a critical technology³⁷⁰, this topic aims to ensure that promising research results are efficiently taken further along the value chain, speeding the time to market or patient through stepwise funding and increasing the EU's competitiveness. The chosen area of focus is regenerative medicine as it has the potential to heal or replace tissues and organs damaged by age, disease, or trauma, as well as to normalise congenital defects. Proposals should focus on prototyping, demonstrating and validating health innovations from TRL 5, moving beyond early-stage research to clinical development, testing, or eventual large-scale manufacturing. The previously funded EU research on which the proposal is built should be applicable to the field of regenerative medicine and should have clear exploitation potential and/or socio-economic benefits for the patients.

Applicants are expected to:

- Demonstrate in their proposal that the health product, therapy or service, has been successfully validated at preclinical level in the prior EU funded project and provide justification of the innovation potential with qualitative and quantitative data (e.g. publications, patent/trademark/design applications, spin-out/start-up track record, regulatory procedures, venture capital pitches, funds raised etc).
- Justify the proposed composition of the consortium and explain how this differs from the previous grant, and demonstrate how the health product, therapy or service to be developed further qualifies as regenerative medicine.
- Demonstrate adequate protection of the idea or Intellectual Property Rights or ensure freedom to operate until full deployment.
- Have a clear vision on the intended pathway to patients and/or route to market, including regulatory compliance. This includes defining specific milestones together with concrete

³⁶⁹ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

³⁷⁰ Commission Recommendation on critical technology areas for the EU's economic security for further risk assessment with Member States: https://defence-industry-space.ec.europa.eu/system/files/2023-10/C_2023_6689_1_EN_annexe_acte_autonome_part1_v9.pdf

and verifiable Key Performance Indicators (KPIs) to assess progress towards the market or healthcare settings.

- Identify the target patient group(s) (how many patients to be treated during the project and the potential patient population that could benefit) and product development milestones including a financial plan (for each milestone).

In the case of innovations with commercial potential, proposals should present the investor and market readiness towards commercialisation and deployment (market research, value proposition, business case and business model, prospects for growth, intellectual property protection, competitor analysis etc.) as well as aspects of regulation, certification and standardisation and reimbursement.

In the case of innovations with evidenced limited commercial potential but high patient benefit, proposals should contain a deployment and sustainability plan including aspects related to regulations, certification and standardisation and patient access through healthcare providers.

Proposals should take into account sex, gender, age and other relevant socio-demographic variables to ensure the scientific robustness, clinical value and applicability of the targeted regenerative medicine innovation.

Applicants should provide details of their clinical studies³⁷¹ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2026-01-TOOL-06: Support to European Research Area (ERA) action on accelerating New Approach Methodologies (NAMs) to advance biomedical research and testing of medicinal products and medical devices

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 2.90 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 2.90 million.
<i>Type of Action</i>	Coordination and Support Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply:

³⁷¹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	In recognition of the opening of the US National Institutes of Health's programmes to European researchers, legal entities established in the United States of America may exceptionally participate as a beneficiary or affiliated entity, and are eligible to receive Union funding. Coordinators of projects must be legal entities established in an EU Member State or Associated Country. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ³⁷².

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Member States and relevant stakeholders identify priority areas where New Approach Methodologies (NAMs) and infrastructures are most needed and expected to have the highest short- to medium-term impact.

³⁷² This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

- Member States and other stakeholders jointly support the validation and qualification of a limited set of NAMs that are intended to be accepted and implemented in regulatory testing of medicinal products and medical devices.
- Member States and other stakeholders develop common education and training programmes based on best practices identified in European and non-European countries to better inform researchers and regulators on NAMs and on the application of the 3Rs principles³⁷³.
- Member States and other stakeholders implement a harmonised NAM openness and awareness programme that improves open access to NAMs protocols and results of animal experiments. It also provides guidance to harmonise the awareness of NAMs for ethical committee members, reviewers, and regulators, based on best practices in the participating Member States. The programme should propose concrete actions to increase the confidence of regulators in NAMs including a better understanding of the potential and limitations of NAMs.

Scope: This topic aims to coordinate and develop the new European Research Area (ERA) policy action to accelerate, through an aligned and coordinated approach across Member States and Associated Countries, the development, validation/qualification, acceptance, and uptake of NAMs in biomedical research and regulatory testing of medicinal products and medical devices as part of the ERA Policy Agenda 2025-2027³⁷⁴.

The ERA action should establish an EU-wide forum that brings together relevant ministries, regulatory agencies, research funding organisations, academia, industry (pharmaceutical and medical technology), Contract Research Organisations (CROs), small and medium-sized enterprises (SMEs)³⁷⁵, and startups to harmonise policies and strategies for NAMs development and implementation.

The selected proposal should be coordinated by any active participant to the ERA action to ensure consistency with ERA action policy objectives. It should contribute to the implementation of the following themes of the four thematic Working Groups (WGs) of the ERA action:

WG1: Development of NAMs and common European infrastructures. This WG identifies opportunities for the development and integration of NAMs and the establishment of supporting infrastructures. Its focus spans specific disease or biological areas and safety, quality, and efficacy assessment endpoints for medicinal products and medical devices. The WG provides insight and suggests priorities to governments and industry for the further coordinated efforts to leverage promising development of NAMs, taking into consideration

³⁷³ Replacement, Reduction, Refinement: <https://nc3rs.org.uk/who-we-are/3rs>

³⁷⁴ Proposal for a Council Recommendation on the European Research Area Policy Agenda 2025-2027: <https://european-research-area.ec.europa.eu/documents/proposal-council-recommendation-european-research-area-policy-agenda-2025-2027>

³⁷⁵ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

the complementarity of scientific strengths, funding priorities and available expertise in the different Member States and regions.

WG2: Validation, acceptance, and uptake of NAMs. The WG defines optimal criteria for NAMs to facilitate their uptake in the contexts of basic and applied biomedical research, and their acceptance for the regulatory assessment and eventual approval of medicinal products and medical devices within defined contexts of use. It proposes priorities for the validation and qualification of NAMs. Member States and pharma/MedTech industry take the decision to jointly support the validation/qualification of certain NAMs that are sufficiently mature for acceptance and uptake in regulatory testing of medicinal products and medical devices.

WG3: Education and training. The WG maps existing education and training programmes on NAMs and the 3Rs principles and assesses their quality and outreach. The WG makes suggestions to Member States based on the best practices identified for the joint development of high-quality education and training modules on NAMs and the application of the 3Rs principles in close partnership with education directors at knowledge institutes.

WG4: Openness and awareness. The WG develops common policies to improve the openness and quality of research, including open access to available protocols on NAMs, and facilitating the publication of results from NAMs and animal experiments, even if these are negative or neutral (or historic, if feasible and appropriate), to avoid unnecessary duplication of animal testing or development of non-valid NAMs. It considers strategies for sharing best practices to make sure that different ethical committees, funding assessment committees, reviewers, and regulators have a similar level of awareness regarding the latest scientific advancements in available NAMs. It proposes actions to enhance the confidence of regulators in validated and qualified NAMs. The WG also identifies opportunities for raising awareness among civil society and patients regarding the biomedical research, drug discovery and development process.

The European Commission's Joint Research Centre (JRC) may contribute to the proposal selected for funding, particularly with activities on innovative in vitro biotechnologies.

HORIZON-HLTH-2026-01-TOOL-07: Establishing a European network of Centres of Excellence (CoEs) for Advanced Therapies Medicinal Products (ATMPs)

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 3.90 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 3.90 million.
<i>Type of Action</i>	Coordination and Support Actions

<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, legal entities established in the United States of America may exceptionally participate as a beneficiary or affiliated entity, and are eligible to receive Union funding. Coordinators of projects must be legal entities established in an EU Member State or Associated Country. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: Eligible proposals submitted under this topic and exceeding all the evaluation thresholds will be awarded a STEP Seal ³⁷⁶.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ³⁷⁷.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools,

³⁷⁶ https://strategic-technologies.europa.eu/about/step-seal_en

³⁷⁷ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- In line with the objectives of the “Strategy for European Life Sciences”³⁷⁸, the EU successfully translates a strategic priority into an implementable action plan for building technological and innovation leadership on a global stage.
- Europe, including regions, profits from an increased capacity, accessibility, coordination of its Advanced Therapies Medicinal Products (ATMP) infrastructures.
- The European economy benefits from more ATMP innovations being developed and commercialised in the EU.
- Patients across the EU gain faster access to innovative ATMPs thanks to increased and focused public Research and Development (R&D) investment, harmonised policies and strategies for ATMP development and uptake in healthcare systems.

Scope: ATMPs represent a frontier in medicine, offering groundbreaking treatments such as gene therapies, cell therapies, and tissue-engineered products that hold the promise of addressing complex and previously untreatable conditions. The European ATMP landscape is dynamic and promising, with 28 products having received marketing authorisation and many more in the pipeline³⁷⁹. The development of specialised infrastructures in Europe for these cutting-edge therapies is crucial for fostering innovation and ensuring fast and efficient delivery to patients in an equitable way. The report by Mario Draghi ³⁸⁰ on EU competitiveness, highlights that the EU’s share of the fast-growing global ATMP market is small, suggesting that to remain competitive, increased and focused public R&D investment is needed to complement ongoing efforts to streamline regulations and ensure faster pricing and reimbursement. The report recommends building on existing innovation hubs and expanding the capacity of the EU to conduct ATMPs R&D by the consolidation of EU public funds. By strengthening this budding innovation ecosystem, Europe can position itself as a leader in the ATMP sector, ultimately improving patient access to life-saving treatments and stimulating economic growth.

The EU has several scattered ATMP centres with divergent capacities and capabilities³⁸¹, limiting its attractiveness for scaling up R&D in the field, calling for a coherent and coordinated approach. The aim of this topic is to establish a European network of Centres of Excellence (CoEs) for ATMPs, building on existing centres and coordinating their further

³⁷⁸ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

³⁷⁹ Committee for Advanced Therapies (CAT) quarterly highlights and approved ATMPs Feb-May 2025: https://www.ema.europa.eu/en/documents/committee-report/cat-quarterly-highlights-approved-atmps-may-2025_en.pdf

³⁸⁰ The future of European competitiveness, Mario Draghi: https://commission.europa.eu/topics/eu-competitiveness/draghi-report_en

³⁸¹ Advanced Therapy Medicinal Products - EATRIS: <https://eatris.eu/atmp>

development in synergy with national strategies. The creation of such a Network is key for strengthening the EU's capacity to develop, scale up and deploy ATMPs across Europe, increasing Europe's attractiveness for clinical research in the field and supporting the implementation of the "Strategy for European Life Sciences". The CoEs should directly support the development and manufacturing capacity of ATMP-related biotechnologies, such as cell and gene therapy platforms, manufacturing infrastructure, and scale-up processes.

Each potential CoE should be an existing centre embedded within a vibrant biocluster (i.e. within proximity to pharmaceutical companies and research institutes) and should already benefit from critical infrastructure and services necessary to advance from lab to patient such as knowledge transfer support, state-of-the-art GMP³⁸² and clinical trials facilities. In addition, the centre should be performing the full spectrum of life sciences research, from discovery to clinical trials and should have demonstrated leadership in the field through a stand-alone research programme. Each CoE is expected to become world-class by further embedding itself in the full value chain and seeking additional political and financial support at regional, national and European level. The network should be limited to no more than 10 CoEs across the EU, with complementary expertise in the various ATMP technologies.

The proposed European network of CoEs for ATMPs should include multiple stakeholders beyond the research community and/or established academic centres, including Member State ministries, regional representatives, funders, regulators and healthcare payers, industry actors, patient organisations and policymakers. The European Commission's Joint Research Centre (JRC) may participate as a member of the consortium selected for funding, bringing its expertise in pre-normative research, standardisation, regulatory advice and access to its research infrastructure.

To align with STEP eligibility, activities supported under this topic should demonstrate how they contribute to either: i) bringing innovative, cutting-edge technologies with strong economic potential to the internal market, or ii) reducing or preventing the Union's strategic dependencies in the field of advanced therapies.

Proposals should be of limited duration (2027-2030) and cover at a minimum the following activities:

- Identify common needs and challenges related to ATMP R&D, as well as develop relevant policy recommendations related to clinical trials, manufacturing, logistics, regulatory (including harmonisation of market access authorisation and reimbursement procedures), public acceptance, policies for transnational care, and coordination with national/regional healthcare systems etc.
- Develop a roadmap to ensure that Europe becomes the global leader for ATMP R&I by 2035, with clearly defined milestones, targets and Key Performance Indicators (KPIs). The roadmap should align with national R&D plans and include a long-term funding

³⁸² Good Manufacturing Practices: https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

strategy. Appropriate measures should be considered to expand or widen the network during the roadmap's implementation.

- Create an advisory board with diverse stakeholders, as a forum to provide guidance and advice for ensuring maximum utility of the generated outputs.
- Develop common education and training programmes for the next generation of scientists including outreach activities to better inform i) the public and patients on the benefits of ATMPs and ii) the stakeholders about access to the CoEs facilities and support.

Proposals should build on the experiences and outcomes of previous or ongoing actions such as RESTORE³⁸³, Join4ATMP³⁸⁴, PRECISEEU³⁸⁵ and T2EVOLVE³⁸⁶, and liaise with the relevant partnerships such as the co-funded European Partnership on Rare Diseases³⁸⁷, the co-funded European Partnership for Personalised Medicine³⁸⁸ and the Innovative Health Initiative Joint Undertaking (IHI-JU)³⁸⁹ as appropriate.

HORIZON-HLTH-2027-02-TOOL-01-two-stage: Development of predictive biomarkers of disease progression and treatment response by using AI methodologies for chronic non-communicable diseases

Call: Cluster 1 - Health (Two stage - 2027)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 6.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 44.20 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Admissibility conditions</i>	The conditions are described in General Annex A. The following exceptions apply: Applicants submitting a proposal for a blind evaluation (see General Annex F) must not disclose their organisation names, acronyms, logos nor names of personnel in the proposal abstract and Part B of their first-

³⁸³ <https://cordis.europa.eu/project/id/820292>

³⁸⁴ <https://ec.europa.eu/info/funding-tenders/opportunities/portal/screen/how-to-participate/org-details/999999999/project/101137206/program/43108390/details>,
<https://cordis.europa.eu/project/id/101137206>

³⁸⁵ <https://cordis.europa.eu/project/id/101161301>

³⁸⁶ <https://cordis.europa.eu/project/id/945393>

³⁸⁷ <https://cordis.europa.eu/project/id/101156595>, <https://erdera.org>

³⁸⁸ <https://cordis.europa.eu/project/id/101137129>, <https://www.epprmed.eu>

³⁸⁹ <http://www.ihj.europa.eu>

	stage application (see General Annex E).
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: For the first stage, the thresholds for each criterion will be 4 (Excellence) and 4 (Impact). The overall threshold applying to the sum of the two individual scores will be set at a level that ensures the total requested budget of proposals admitted to stage 2 is as close as possible to four times the available budget, and not less than three and a half times the available budget. For the second stage, the thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: The first-stage proposals of this topic will be evaluated blindly.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Clinical researchers and developers have access to novel predictive biomarkers to guide a more accurate assessment of disease progression and treatment response and tackle the unmet clinical needs of non-communicable chronic diseases.
- Clinicians and healthcare professionals use clinically validated predictive biomarkers for implementing more effective clinical research and personalised medicine with better health outcomes in chronic non-communicable diseases.
- Key stakeholders have access to trustworthy Artificial Intelligence (AI) tools to guide the development of multimodal predictive biomarkers of higher accuracy and clinical value when compared to the established practice.
- The citizens benefit of better health outcomes thanks to improved clinical guidelines and the implementation of effective biomarker-guided clinical research and personalised healthcare.

Scope: Biomarkers³⁹⁰ are invaluable tools for improving patient outcomes, guiding treatment decisions, accelerating personalised medicine, more effective clinical research and the development of better medicines.

However, despite the scientific discoveries of many clinically relevant biomarkers, estimated on the scale of tens of thousands, only a few biomarkers have been implemented in clinical practice. The traditional ‘one biomarker’ paradigm is insufficient for addressing the unmet clinical needs of chronic, progressive and multifactorial diseases, due to the complexity of the clinical phenotypes characterised by broad inter-and intra-patient heterogeneity. The established biomarkers have limitations in their use as prognostic and predictive indicators, for the assessment of the disease progression and the choices of the optimal therapeutic interventions tailored to the patients’ characteristics.

Therefore, the topic focuses on the clinical development of predictive biomarkers of disease progression and treatment response for chronic non-communicable diseases (excluding cancer) by using established AI methodologies able to combine data of clinically used and candidate biomarkers, with available data from relevant clinical studies, longitudinal and Real-World Data (RWD)³⁹¹. This topic is expected to support collaborative projects paving the way for future innovations in personalised medicine and enabling more timely and effective therapeutic interventions.

The proposals should address all the following research and innovation activities:

- Set-up a multidisciplinary collaboration to map and evaluate the available information and data on biomarkers currently used in the clinical setting, candidate biomarkers from

³⁹⁰ See definition as in the Strategic Research and Innovation Agenda of the Innovative Health Initiative Joint Undertaking: http://www.ih.europa.eu/sites/default/files/uploads/Documents/About/IHI_SRIA_ApprovedJan22.pdf

³⁹¹ EMA definition: “Real-World Data are routinely collected data relating to patient health status or the delivery of healthcare from a variety of sources other than traditional clinical trials (e.g. claims databases, hospital data, electronic health records, registries, mhealth data, etc.)”.

past and ongoing clinical studies, which are scientifically proven as clinically relevant to the disease progression and treatment response for the chronic non-communicable diseases under study. This should include stratification by biological sex, and where feasible, integration of gender-related variables and sociodemographic determinants that may modulate disease trajectories or treatment efficacy.

- Adapt and apply of established AI methods rather than developing novel ones from scratch, to deliver novel predictive biomarkers of disease progression and treatment response, by integrating data of currently used and candidate biomarkers, with suitable data from available longitudinal and other relevant clinical studies, including RWD, as necessary. To guarantee a solid and fast optimisation and training of the AI tools, the applicants should provide information in their proposal that the appropriate high-quality clinical data are readily available, and when necessary generate small-scale new data for the AI optimisation needs. The biomarkers under study should be multimodal, covering for instance molecular, cellular, physiological, imaging, behavioural and digital markers, and/or their combinations. The applicants should justify why the development of the biomarkers proposed is imperative to tackle the unmet clinical needs of the chronic non-communicable diseases under study.
- Use AI and, where needed, other relevant data and knowledge integration methods, to describe the relationships among different biomarkers and support the robust prioritisation of predictive biomarkers tailored to the characteristics of the patients' and their disease stage and treatment response. Proposals should have strong emphasis on the AI trustworthiness³⁹² and develop the adequate performance metrics to assess their accuracy, reliability, reproducibility, including the assessment of possible inherent bias. Use of AI and dataset should comply with existing privacy-preserving legislation. Moreover, proposals should consider the development of user-friendly and fit-for-purpose visualisation and decision-support tools to guide clinicians in evaluating the clinical plausibility of the biomarkers under study across diverse patient groups.
- Establish a biomarker validation platform to assess the clinical utility of the predictive biomarkers identified. To this end, the applicants should implement clinical validation studies in independent disease cohorts, RWD and exploratory clinical studies, as appropriate, to demonstrate their clinical value as prognostic and predictive indicators for more effective clinical research and better patient health outcomes as compared to the established clinical practice of chronic non-communicable diseases. Prospective clinical studies are expected to be led by entities in the EU/EFTA and/or Associated Countries.

³⁹²

See introduction to this Work Programme part as well as the Ethics Guidelines for Trustworthy AI, published by the European Commission's High Level Expert Group on Artificial Intelligence <https://digital-strategy.ec.europa.eu/en/library/ethics-guidelines-trustworthy-ai> and the Ethics by Design and Ethics of Use Approaches for Artificial Intelligence https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ethics-by-design-and-ethics-of-use-approaches-for-artificial-intelligence_he_en.pdf

- Develop a comprehensive exploitation plan for the valorisation of the research outputs and a regulatory strategy to support the alignment to the regulatory requirements for the qualification of the biomarkers and/or AI tools and engage with the regulators in a timely manner. The applicants should prioritise the exploitation of their research results in the EU. Participation of small and medium-sized enterprises (SMEs)³⁹³ is encouraged with the aim to strengthen the scientific and technological basis of SMEs and valorise their health innovations.

Proposals should apply good practices for GDPR³⁹⁴ compliant personal data protection.

Proposals are encouraged, where relevant, to exploit the available data services, expertise and digital tools offered by the relevant European research infrastructures³⁹⁵ and/or data infrastructures³⁹⁶ in the area of health funded under the Digital Europe Programme.

All proposals selected for funding under this topic will be strongly encouraged to participate in networking and joint activities (e.g. participation in joint workshops, development of best practices, or joint communication activities), which may also involve networking with projects funded under Horizon Europe, or other EU programmes (e.g. the Digital Europe Programme³⁹⁷). The proposals should allocate a sufficient budget for networking and joint activities, without the prerequisite to detail such activities at the proposal stage.

Applicants should provide details of their clinical studies³⁹⁸ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-03-TOOL-02: Advancing bio-printing of living cells for regenerative medicine

Call: Cluster 1 - Health (Single stage - 2027/2)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 7.00 and 10.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 39.30 million.

³⁹³ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

³⁹⁴ General Data Protection Regulation: https://commission.europa.eu/law/law-topic/data-protection_en, <https://gdpr-info.eu>

³⁹⁵ The catalogue of European Strategy Forum on Research Infrastructures (ESFRI) research infrastructures portfolio can be browsed on the ESFRI website: <https://ri-portfolio.esfri.eu>

³⁹⁶ <https://digital-strategy.ec.europa.eu/en/policies/artificial-intelligence-health>

³⁹⁷ <https://digital-strategy.ec.europa.eu/en/activities/digital-programme>

³⁹⁸ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). The Joint Research Centre (JRC) may participate as member of the consortium selected for funding as a beneficiary with zero funding, or as an associated partner. The JRC will not participate in the preparation and submission of the proposal - see General Annex B.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Biomedical scientists from academia and industry will gain access to entire bio-printing units designed to regenerate human tissue.
- Healthcare professionals acquire information on the safe and effective use of equipment enabling advanced therapies with bio-printed human tissue.
- Healthcare providers dispose of tools enabling them to treat conditions of unmet medical need.
- Individual patients will benefit from a personalised approach to their respective medical condition thanks to the bio-printed regenerative medicine solution.

Scope: Tissue-specific functional 3D bio-printing of living cells has made significant progress as a new approach for transplantation applications in regenerative medicine. There are currently several types of bio-printing technologies under development for the repair of different targeted tissues or organs. To fully unleash the potential of bio-printed cell constructs for regenerative medicine several bottlenecks still need to be overcome. Various studies in pre-clinical models have shown that bio-printed cell constructs or tissues hold great promise for regenerative medicine, by allowing autologous tissue grafts being printed thus avoiding adverse graft-host reactions. However, translation of such approaches into clinical settings (i.e. humans) and their application to internal organs still needs to be investigated and demonstrated. Depending on the actual target site (i.e. the defect tissue or organ in the human body) different bio-printing approaches may be preferable. For printing complex tissues and especially entire organs an in-vitro approach followed by transplantation is the preferred way. “In-situ” bio-printing, sometimes also referred to as “in-vivo”, or as “intraoperative”, reflects a bioprinting process performed on a live subject in a surgical setting and has in certain instances (e.g. tissue repair) advantages over an in-vitro bio-printing technique followed by transplantation. In-situ bio-printing involves direct patterning of bio-inks onto a patient’s body at the target site, allowing for precise construction of a site-matching tissue-structure within the actual physiological location where regeneration or repair is needed. As such, in-situ bio-printing allows for high adaptability, reduced risk of contamination, improved cell viability, function and host integration. The high cell densities present in the human vital organs underscore the importance of bio-inks which contain less additional biomaterials as matrix. Hence the bio-printing of cell constructs that comprise native tissue-like cell densities may facilitate repair and/or regeneration of defective complex tissues or internal organs. For such approaches meticulous engineering of the bio-printing equipment is necessary, involving sophisticated micro-surgical instrumentation and medical imaging platforms. To achieve the desired function and to mimic the natural cues in native tissues for in-vitro printed bio-constructs, the use of additional stimuli is needed, whereas in-situ approaches normally rely on the body as natural bioreactor providing the necessary extracellular cues. Recently, combinations of in-situ bio-printing with real-time stimuli have been investigated, even for the repair of internal organs. However, there remain bottlenecks that need to be overcome, like the integration with existing imaging modalities and surgical procedures or the long-time stability and functionality of the created bio-constructs.

To address these challenges, researchers should work in multidisciplinary teams with engineers, biomedical scientists, cell biologists and medical doctors. Proposals should be based on the use of human cells and address all the following activities:

- Develop or improve existing bioprinting equipment that comprises all steps of the bio-printing suite to print bio-constructs with high cell-density for improved vascularisation and faster repair of the defect in the body.

- Scale-up the chosen bio-printing technology to a Good Manufacturing Practices (GMP)³⁹⁹ conform/compliant manufacturing process.
- Perform all necessary regulatory work enabling the conduct of clinical studies and assess the clinical value of the developed bio-printing technology in first in-human studies.

Priority should be given to bio-printing approaches that either target vital internal organs followed by surgical grafting or employ in-situ approaches depositing the cell-laden bioink directly from the printhead or endoscope on the defect target site in the body.

Regulatory knowledge of the field is desired and should be documented through contacts with relevant national or international European regulatory authorities. A good understanding of the different steps involved and the inherent risks in each of these steps will be a basis to identify appropriate safety and quality requirements. Requirements from the different established EU frameworks on Substances of Human Origin (SoHO), medical devices and pharmaceuticals including Advanced Therapy Medicinal Products (ATMPs) should be considered for manufacturing/preparation as well as for clinical outcome monitoring. A combination of requirements from different frameworks might be most appropriate to allow for responsible and fast uptake.

Proposals under this topic may address any therapeutic area, i.e. any disease, dysfunction or defect. Sex differences at the cellular level should be taken into consideration.

Preclinical stage and clinical development are eligible. The involvement of small and medium-sized enterprises (SMEs)⁴⁰⁰ is encouraged.

The European Commission's Joint Research Centre (JRC) may contribute to the proposals selected for funding with work on strategic technologies for economic security and innovative industrial ecosystems, particularly activities on innovation in vitro biotechnologies.

Applicants should provide details of their clinical studies⁴⁰¹ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-03-TOOL-04: Virtual Human Twins (VHTs) for integrated clinical decision support in prevention and diagnosis

Call: Cluster 1 - Health (Single stage - 2027/2)	
Specific conditions	
<i>Expected EU contribution per</i>	The Commission estimates that an EU contribution of between EUR 10.00 and 12.00 million would allow these outcomes to be addressed

³⁹⁹ https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

⁴⁰⁰ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

⁴⁰¹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

<i>project</i>	appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 39.30 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Healthcare professionals have access to multi-scale⁴⁰², multi-organ models of individual patients that aim to improve prevention and diagnosis in high disease burden areas.
- Health professionals benefit from enhanced knowledge of complex diseases and co-morbidities by recourse to multi-scale, multi-organ models.

⁴⁰² In the context of this topic, multi-scale refers to modelling at different levels of human anatomy, e.g. at (sub-)cellular, tissue, organ or organ system level.

- Patients with diverse characteristics (e.g. of any sex, age group, racial or ethnic origin⁴⁰³) benefit from improved, integrated and personalised prevention and diagnostics tools.
- Health professionals and patients benefit from the use of “Virtual Human Twin” (VHT) models which enable integration of other preventive and diagnostic tools and modalities.

Scope: VHTs are digital representations and in-silico models of an individual’s health and disease state at different levels of anatomy. Multi-scale, multi-organ VHT solutions have a potential for tailored prevention and diagnosis, particularly in areas of high disease burden, and can significantly benefit citizens' health and the efficiency of EU health systems.

Proposals should take into account the work of projects funded under topic HORIZON-HLTH-2023-TOOL-05-03: “Integrated, multi-scale computational models of patient pathophysiology (‘virtual twins’) for personalised disease management”, which had a predominant focus on disease management, and focus on high-potential multi-disciplinary approaches at greater complexity (multiscale, multiorgan, longitudinal), strengthening their deployment in health and care, including the integration into care pathways and links with other decision support tools.

The proposals should address all the following activities:

- Select clinical use cases to deliver multi-disciplinary, high impact solutions requiring multi-organ, multi-scale approaches to modelling complex pathophysiology over time, as a basis from where prevention and diagnosis of diseases with high morbidity and mortality could be enhanced. Proposals can put forward use cases in any areas of high disease burden; examples include co-morbidities, chronic cardiovascular conditions, infection and (auto)immunity, inflammation and cancer, diabetes and related conditions, rare diseases, degenerative diseases (including their interaction with mental health conditions), the exposome and its impact on human health and disease.
- Building on current approaches, standards, data repositories (e.g. biobanks, environmental data, others) and modelling assets (e.g. those of the EDITH CSA⁴⁰⁴ and the Platform for Advanced VHT Models⁴⁰⁵), and new data if relevant, design, develop, extend and validate multi-organ, multi-scale, dynamic computational models that accurately simulate a person’s health and disease states, as necessary.
- Evaluate, select, extend and validate diverse modelling methodologies, resulting in integrated, advanced, interoperable, patient-specific VHT models that can integrate diverse data sources and methodologies, addressing the chosen clinical use case requirements. Methodologies may include and are not limited to biophysics-based

⁴⁰³ The use of the term ‘racial or ethnic origin’ does not imply an acceptance of theories that attempt to determine the existence of separate human races.

⁴⁰⁴ See the “European Virtual Human Twin” Coordination and Support Action EDITH, funded under the Digital Europe Programme: <https://www.edith-csa.eu>

⁴⁰⁵ Funded under the Digital Europe Programme, procedure identifier EC-CNECT/LUX/2024/OP/0014: <https://ec.europa.eu/info/funding-tenders/opportunities/portal/screen/opportunities/tender-details/16cc3c6a-844a-42d4-9746-dcc7444b8001-CN>

modelling, artificial intelligence (AI) that should be interpretable or allow explainability of outcomes, generative AI and in-silico modelling, agent-based and network physiology approaches. Evaluation, selection and extension of these should be documented during the design phase. Availability and integration of the multi-modal data should be documented, and the ethical and sex dimensions be investigated.

- Demonstrate integration of these models with other advanced preventive and diagnostic modalities, tools and techniques enabling integration across pathways.
- Generate evidence, including clinical validation, that the solutions deliver clinically meaningful decision support, addressing use case requirements. Document lessons-learned for broader application. Gather evidence via health economic and/or feasibility studies in real-world healthcare settings confirming cost-effectiveness vis-à-vis current practice (e.g. cost-effectiveness analysis). Produce an exploitation plan on regulatory compliance⁴⁰⁶ and intellectual property.

Proposals should be multidisciplinary; solution design and development should be end-user-focused and draw on user and non-user input. Best practice in VHT software development including responsible AI development should be followed (e.g. risk assessment and management, requirements definition process).

Participation of small and medium-sized enterprises (SMEs)⁴⁰⁷ is encouraged.

Proposals should contribute to the objectives of the European VHT Initiative⁴⁰⁸ and to the Platform for Advanced VHT Models, with project assets made available on the Platform and interoperable with its technical specifications⁴⁰⁹; relevant consortia members should join its User Community. Budget should be reserved for these activities. Projects are expected to collaborate with other EU-funded projects on VHTs⁴¹⁰ and align with relevant EU initiatives funded under Horizon Europe, the Digital Europe Programme⁴¹¹ and the EU4Health Programme (2021-2027)⁴¹², e.g. European Cancer Imaging Initiative⁴¹³, 1+Million Genomes Initiative⁴¹⁴, Intensive Care Unit Data Space⁴¹⁵, co-funded European Partnership for Personalised Medicine⁴¹⁶, and projects on advancing AI in health where relevant.

⁴⁰⁶ For example with Regulation (EU) 2017/745 on medical devices: <http://data.europa.eu/eli/reg/2017/745/oj>, Regulation (EU) 2017/746 on in vitro diagnostic medical devices: <http://data.europa.eu/eli/reg/2017/746/2025-01-10>, Regulation (EU) 2024/1689 laying down harmonised rules on artificial intelligence: <http://data.europa.eu/eli/reg/2024/1689/oj>

⁴⁰⁷ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

⁴⁰⁸ <https://digital-strategy.ec.europa.eu/en/policies/virtual-human-twins>

⁴⁰⁹ No contact with the developer of the Platform is required at proposal stage.

⁴¹⁰ Including the projects funded under topic HORIZON-HLTH-2023-TOOL-05-03: “Integrated, multi-scale computational models of patient patho-physiology (‘virtual twins’) for personalised disease management”

⁴¹¹ <https://digital-strategy.ec.europa.eu/en/activities/digital-programme>

⁴¹² https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

⁴¹³ <https://digital-strategy.ec.europa.eu/en/policies/cancer-imaging>

⁴¹⁴ <https://digital-strategy.ec.europa.eu/en/policies/1-million-genomes>

⁴¹⁵ <https://ec.europa.eu/info/funding-tenders/opportunities/portal/screen/opportunities/topic-details/digital-2023-cloud-ai-04-icu-data>

⁴¹⁶ <https://cordis.europa.eu/project/id/101137129>, <https://www.epppermed.eu>

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

Applicants should provide details of their clinical studies⁴¹⁷ in the dedicated annex using the template provided in the submission system. As proposals under this topic are expected to include clinical studies, the use of the template is strongly encouraged.

HORIZON-HLTH-2027-03-TOOL-08: Towards the next generation of frontier Artificial Intelligence models for healthcare

Call: Cluster 1 - Health (Single stage - 2027/2)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of around EUR 2.90 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 2.90 million.
<i>Type of Action</i>	Coordination and Support Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In order to achieve the expected outcomes, and safeguard the Union's strategic assets, interests, autonomy, or security, it is important to avoid a situation of technological dependency on a non-EU source, in a global context that requires the EU to take action to build on its strengths, and to carefully assess and address any strategic weaknesses, vulnerabilities and high-risk dependencies which put at risk the attainment of its ambitions. For this reason, participation is limited to legal entities established in Member States and Associated Countries. Proposals including entities established in countries outside the scope specified in the topic will be ineligible. For the duly justified and exceptional reasons listed in the paragraph above, in order to guarantee the protection of the strategic interests of the Union and its Member States, entities established in an eligible country listed above, but which are directly or indirectly controlled by a non-eligible country or by a non-eligible country entity, may not participate in the action unless it can be demonstrated, by means of guarantees positively assessed by their eligible country of establishment,

⁴¹⁷ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

	that their participation to the action would not negatively impact the Union's strategic assets, interests, autonomy, or security. Entities established in an eligible country, but which are directly or indirectly controlled by China or by a legal entity established in China cannot submit guarantees.⁴¹⁸ If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used).
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ⁴¹⁹.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

⁴¹⁸ The guarantees shall in particular substantiate that, for the purpose of the action, measures are in place to ensure that: a) control over the applicant legal entity is not exercised in a manner that retrains or restricts its ability to carry out the action and to deliver results, that imposes restrictions concerning its infrastructure, facilities, assets, resources, intellectual property or know-how needed for the purpose of the action, or that undermines its capabilities and standards necessary to carry out the action; b) access by a non-eligible country or by a non-eligible country entity to sensitive information relating to the action is prevented; and the employees or other persons involved in the action have a national security clearance issued by an eligible country, where appropriate; c) ownership of the intellectual property arising from, and the results of, the action remain within the recipient during and after completion of the action, are not subject to control or restrictions by non-eligible countries or non-eligible country entity, and are not exported outside the eligible countries, nor is access to them from outside the eligible countries granted, without the approval of the eligible country in which the legal entity is established.

⁴¹⁹ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

- Researchers and innovators benefit from an improved understanding of how to develop and use the next generation of frontier Artificial Intelligence (AI) models for healthcare, including how to leverage AI Factories and how to combine and expand the capabilities of existing foundation models towards inclusive and personalised medicine.
- Researchers and innovators benefit from an improved understanding of how to leverage highly heterogeneous and multimodal health data spanning a range of anatomical scales (i.e. the micro to the macro level).
- Multidisciplinary stakeholders have access to a collaboratively created roadmap for developing the next generation of frontier AI models for healthcare.

Scope: The AI Continent Action Plan⁴²⁰ identifies the health sector, encompassing life sciences, medical devices and healthcare delivery, as one of the key strategic sectors. The action will contribute to making European life sciences⁴²¹ and healthcare more impactful and productive by fostering the full integration of advanced AI in the health sector and biomedical research, along the objectives of the AI in Science strategy⁴²² and Apply AI strategy⁴²³.

Healthcare typically involves the combining of multimodal data, ranging from electronic health records through imaging and laboratory to molecular and omics data. This information combination is performed by specialists and is often challenging towards optimised patient care. In Europe, the growing amount of accessible multimodal health data, including via the forthcoming European Health Data Space (EHDS)⁴²⁴, combined with the increasing availability of high-performance computing facilities (e.g. AI Factories), presents a unique opportunity to develop the next generation of frontier AI models for healthcare. This action anticipates and operationalises the use of such federated infrastructures for research and innovation. Moreover, regulations such as the EHDS regulation and AI Act⁴²⁵ steer the direction into building an ecosystem fostering ethical and safe innovation on AI in healthcare.

AI models are becoming increasingly complex and able to tackle increasingly challenging tasks. This topic will lay the foundation for the development of the next generation of frontier AI models, paving the way for new, advanced AI-powered solutions to increase efficiency and efficacy in the health sector towards improved patient outcomes. It will leverage results, methodologies, data etc. of other relevant EU-funded projects.

⁴²⁰ <https://digital-strategy.ec.europa.eu/en/library/ai-continent-action-plan>

⁴²¹ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

⁴²² https://research-and-innovation.ec.europa.eu/research-area/industrial-research-and-innovation/artificial-intelligence-ai-science_en

⁴²³ <https://digital-strategy.ec.europa.eu/en/consultations/commission-launches-public-consultation-and-call-evidence-apply-ai-strategy>

⁴²⁴ https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en

⁴²⁵ <https://digital-strategy.ec.europa.eu/en/policies/regulatory-framework-ai>,
<https://eur-lex.europa.eu/eli/reg/2024/1689/oj>

<https://eur-lex.europa.eu/eli/reg/2024/1689/oj>

Proposals should include all the following coordination and support activities, ensuring multidisciplinary approaches and a broad representation of stakeholders in the consortium (e.g. healthcare professionals, patients, biomedical scientists, AI developers, data engineers, ethics experts):

- Community building: build a large-scale and diverse pan-European community of stakeholders with the multidisciplinary expertise united as required to develop the next generation of frontier AI models for healthcare, with a view to leveraging as a community the potential of AI Factories. Where relevant, this should build on and strengthen existing EU-funded communities and networks, and could pave the way for a formalised long-term collaboration under one of the available EU instruments.
- Roadmap creation: review previous research to identify the most promising AI models and model development approaches. In addition, risk assess and review evidence on safety and efficacy of existing AI models with reference to the AI Act, related regulatory provisions (including any jurisprudence) and ethical and security considerations, so that frontier AI model development can proceed on a well-informed basis. Finally, create a roadmap for developing the next generation of frontier AI models.
- Dataset identification, curation, expansion and use: i) identification: identify the most suitable existing datasets for the development of frontier AI models for healthcare, ii) curation: identify how to validate the datasets, ensure dataset interoperability, and convert datasets into formats suitable for frontier AI model development, iii) expansion: identify additional datasets and/or annotations required for frontier AI model development, especially to ensure that datasets are representative and iv) use: identify methods and required infrastructure to allow privacy-preserving use and further expansion of the datasets in alignment with and through the EHDS.
- Frontier AI model preparatory activities: mapping approaches for training and evaluating frontier AI models (e.g. approaches to combine foundation models for life sciences and healthcare delivery in order to develop more advanced and multidisciplinary models towards personalised medicine). The approaches should cover all trustworthy AI aspects⁴²⁶.

This action should take into account the results of other relevant projects on AI in health, in particular in the two GenAI4EU topics HORIZON-HLTH-2025-01-CARE-01: “End user-driven application of Generative Artificial Intelligence models in healthcare (GenAI4EU)” and HORIZON-HLTH-2025-01-TOOL-03: “Leveraging multimodal data to advance Generative Artificial Intelligence applicability in biomedical research (GenAI4EU)”, and leverage the AI Factories and specialised health data infrastructures funded under the Digital Europe Programme⁴²⁷, biobanks, relevant ERICs⁴²⁸, as well as the data resources accessible

⁴²⁶ <https://digital-strategy.ec.europa.eu/en/library/ethics-guidelines-trustworthy-ai>

⁴²⁷ <https://digital-strategy.ec.europa.eu/en/activities/digital-programme>

⁴²⁸ European Research Infrastructure Consortia: <https://www.eric-forum.eu/the-eric-landscape>

through the EHDS infrastructure starting in 2029, funded under EU4Health Programme (2021-2027)⁴²⁹.

This topic requires the effective contribution of social sciences and humanities (SSH) disciplines and the inclusion of relevant SSH expertise, in order to produce meaningful and significant effects enhancing the societal impact of the related research activities.

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⁴²⁹ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

Destination - Maintaining an innovative, sustainable, and competitive EU health industry

Topics under this destination are directed towards the Key Strategic Orientation 3 “*A more resilient, competitive, inclusive, and democratic Europe*” of Horizon Europe’s strategic plan 2025-2027⁴³⁰. In addition, Key Strategic Orientation 2 “*The Digital Transition*” and Key Strategic Orientation 1 “*The Green Transition*” are supported.

Research and Innovation supported under this destination should contribute to the following expected impact, set out in the strategic plan impact summary for the Health Cluster: “*the EU health industry is innovative, sustainable, and globally competitive thanks to improved uptake of breakthrough technologies and innovations (including social innovations) that make the EU with its Member States and Associated Countries more resilient and less reliant on imports of critical health technologies*”.

The health industry is a key driver for growth and has the capacity to provide health technologies to the benefit of patients and providers of healthcare services. The relevant value chains involve a broad variety of key players from supply, demand and regulatory sides. In addition, the path of innovation in health is long and complex. The development of novel health technologies is generally associated with uncertainties and market barriers due to expensive and risky development (e.g. high attrition rate in pharmaceutical development), high quality and security requirements (e.g. clinical performance, safety, data privacy and cybersecurity) and market specificities (e.g. strong regulation, pricing and reimbursement issues). In addition, the growing concern about environmental issues is putting more pressure on this industry. Therefore, there is a need for Research and Innovation integrating various stakeholders to facilitate market access of innovative health technologies (medical technologies, pharmaceuticals, biotechnologies, digital health technologies).

In line with the Commission's Political Guidelines for 2024-2029⁴³¹, and building on the recommendations of the reports by Mario Draghi⁴³² and Enrico Letta⁴³³, as well as the “Strategy for European Life Sciences”⁴³⁴, this destination will support research and innovation to enhance the competitiveness of the European health industry, thereby reinforcing EU autonomy, consolidating its Single Market, and empowering Europe to effectively address the burden of both communicable and non-communicable diseases. In this Work Programme part, Destination “*Maintaining an innovative, sustainable, and competitive EU health industry*” focuses on collaborative efforts to advance cell-free protein synthesis platforms, ready-to-use point-of-care diagnostics, and regulatory science to support

⁴³⁰ https://research-and-innovation.ec.europa.eu/funding/funding-opportunities/funding-programmes-and-open-calls/horizon-europe/strategic-plan_en

⁴³¹ https://commission.europa.eu/about/commission-2024-2029_en

⁴³² The future of European competitiveness, Mario Draghi: https://commission.europa.eu/topics/eu-competitiveness/draghi-report_en

⁴³³ Much more than a market, Enrico Letta: <https://www.consilium.europa.eu/media/ny3j24sm/much-more-than-a-market-report-by-enrico-letta.pdf>

⁴³⁴ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

translational development of patient-centred health technologies. The results will support the EU Industrial Policy, with a focus on strengthening the resilience of the single market, addressing the EU's strategic dependencies, gaining technological sovereignty and accelerating the green and digital transitions. The results will further strengthen the single market, by providing evidence and guidelines for stakeholders and regulators to ensure adoption of innovations, supporting environmental, fiscal and socio-economic sustainability and at the same time fostering healthcare access and reducing health inequities. The results will also support the implementation of the relevant Regulations like those on Medical Devices (MDR) and In-Vitro Medical Devices (IVDR) as well as the general uptake of innovative health technologies by health systems, with a special view to aspects related to ensuring industry competitiveness, fostering innovation and sustainability, while maintaining the high level of quality, safety and efficacy of these health technologies.

In view of increasing the impact of EU investments under Horizon Europe, the Commission welcomes and supports cooperation between EU-funded projects to enable cross-fertilisation and other synergies. This could range from networking to joint activities such as the participation in joint workshops, the exchange of knowledge, development and adoption of best practices, or joint communication activities. All topics are open to international collaboration to address global environment and health challenges.

Expected impacts:

Proposals for topics under this destination should set out a credible pathway to contributing to maintaining an innovative, sustainable and competitive EU health industry, and more specifically to one or several of the following expected impacts:

- Health industry in Europe and Associated Countries is more competitive and sustainable, assuring European leadership in breakthrough health technologies and open strategic autonomy in essential medical supplies and (digital) technologies, contributing to job creation and economic growth, in particular with small and medium-sized enterprises (SMEs)⁴³⁵.
- Health industry is supported by cross-sectoral Research and Innovation in the context of convergence of health technologies (integrating medical technologies, pharmaceuticals, biotechnologies, digital health, and e-health technologies) while strengthening key market positions.
- Health industry is working more efficiently along the value chain from the identification of needs to the scale-up and take-up of solutions at national, regional or local level, including through early engagement with patients, healthcare providers, health authorities and regulators ensuring suitability and acceptance of solutions.
- Citizens, healthcare providers and health systems benefit from a swift uptake of innovative health technologies and services through the provision of evidence and guidelines for stakeholders, policymakers and regulators. These efforts offer significant

⁴³⁵ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

improvements in health outcomes, also potentially strengthening access to healthcare for all and reducing health inequities while health industry benefits from decreased time-to-market.

- Citizens, healthcare providers and health systems benefit from increased health security in Europe and Associated Countries due to reliable access to key manufacturing capacity, including timely provision of essential medical supplies and technologies of particularly complex or critical supply and distribution chains.

Legal entities established in China are not eligible to participate in both Research and Innovation Actions (RIAs) and Innovation Actions (IAs) falling under this destination. For additional information please see “Restrictions on the participation of legal entities established in China” found in the Annex B of the General Annexes of this Work Programme.

The protection of European communication networks has been identified as an important security interest of the Union and its Member States. Entities that are assessed as high-risk suppliers⁴³⁶ of mobile network communication equipment (and any entities they own or control) are not eligible to participate as beneficiaries, affiliated entities and associated partners to topics identified as “subject to restrictions for the protection of European communication networks”. Please refer to the Annex B of the General Annexes of this Work Programme for further details.

Proposals are invited against the following topic(s):

HORIZON-HLTH-2026-01-IND-03: Regulatory science to support translational development of patient-centred health technologies

Call: Cluster 1 - Health (Single stage - 2026)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 4.00 and 6.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 19.60 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply:

⁴³⁶ Entities assessed as “high-risk suppliers”, are currently set out in the second report on Member States’ progress in implementing the EU toolbox on 5G cybersecurity of 2023 (NIS Cooperation Group, Second report on Member States’ progress in implementing the EU Toolbox on 5G Cybersecurity, June 2023) and the related Communication on the implementation of the 5G cybersecurity toolbox of 2023 (Communication from the Commission: Implementation of the 5G cybersecurity Toolbox, Brussels, 15.6.2023 C(2023) 4049 final).

	In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ⁴³⁷.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Maintaining an innovative, sustainable, and competitive EU health industry”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to several of the following expected outcomes:

- Policymakers and regulators will get accelerated access to improved evidence driven methodologies to evaluate the impact and efficiency of novel health technologies, facilitating decision-making for their use in humans and uptake in clinical practice.
- Patients and the health systems will benefit from the more targeted and efficient uptake of safe and effective health innovations in clinical practice, supporting more personalised approaches and improved care and public health.

⁴³⁷ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

Scope: The development, uptake and impact of health technologies typically results from a long product development process that is based on a 'life cycle approach' which typically involves several iterations of defined stages, i.e. from development, assessment to post-market surveillance and post-market clinical follow-up.

While health technologies are governed by comprehensive legal frameworks aiming to ensure that health technologies are safe and effective, the regulatory science underlying these legal frameworks needs to be updated. This concerns *inter alia* i) more precise delineation of specific requirements (e.g. closing existing gaps concerning sufficiency of clinical evidence) and ii) the consideration of novel biomedical approaches, data and digital solutions (e.g. artificial intelligence - AI, virtual human twin, new approach methodologies as well as methods that cut through these domains) which model and predict relevant biological parameters and exploit relevant end-points and novel (bio)markers for clinical diagnostic and prognostic predictions. Such update of the regulatory science of health technologies should aim at supporting an effective adoption and uptake into routine use by health systems and end-users (healthcare providers, citizens), while maintaining guardrails to ensure that innovative health technologies are backed up by evidence of sufficient quality and relevance to the human situation.

Proposals can cover all types of health technologies, aiming to define improved and novel sources of evidence with proven relevance for regulatory decision-making with a focus on safety and performance throughout their lifecycle, i.e. throughout the continuous process of clinical evaluation. To this end, proposals should address either, or a combination of the following: i) the improvement of existing methodologies and their fitness to specific types or classes of health technologies, including methodology for regulatory assessment and ii) explore and examine to which extent novel information sources as indicated above can be considered as evidence that is satisfactory in view of regulatory needs concerning safety and performance.

Proposals should support the update and refinement of regulatory science on health technologies and contribute actionable information that can be used for improved or novel regulatory policies, rules, guidance documents and other tools with a view to ensuring that European patients and healthcare professionals have access to safe and effective innovative health technologies. Proposals should ultimately contribute to a regulatory environment that makes use of the full spectrum of novel biomedical and bio-digital approaches for clinical investigation and evaluation, while promoting a patient-centred approach to health technology innovation, facilitating the timely entry to market of performant and effective innovations and support their uptake in the health systems and clinical workflows without compromising patient safety.

Applicant consortia should reflect a broad representation of stakeholders, notably clinical societies, academia, notified bodies, industry, patients and regulators and the proposed work should address one or more of the following elements:

- Data and analyses on how existing approaches in regulatory science can be refined and improved in view of closing existing gaps of clarity, sufficiency of clinical evidence, generated on the basis of clinical studies and clinical investigations.
- Data and analyses on whether and to which extent novel information sources from biomedicine including new approach methods and digital and AI-enabled models and approaches can contribute to the clinical evaluation of innovative health technologies, e.g.:
 - o By providing information on relevant biophysical, anatomical, physiological and other disease-relevant aspects.
 - o By supporting information integration through the use and aggregation of already existing data, including clinical ones, from similar types or groups of technologies (e.g. retrospective information in registries, data collections, including Real-World Data (RWD)⁴³⁸ from using technologies that have characteristics that are relevant for innovative technologies).
 - o By supporting improved planning and design of first-in-man clinical studies, with a view of enhancing the effectiveness and the safety of such studies and rationalising the use of resources of all involved actors by focusing the generation and assessment of clinical data on health technologies for which those data are indispensable.
- Data and analyses that examine to which extent the above-mentioned points can support the development and uptake of innovative technologies for unmet medical needs and for special patient populations (e.g. paediatric and rare conditions) via dedicated regulatory pathways and/or within a structured framework enabling their development and testing in a real-world environment under regulatory supervision (“regulatory sandbox”).

The actual conduct of clinical studies⁴³⁹ is not in scope of this topic.

The activities should cover and draw on all the relevant healthcare innovation related frameworks other than pharmaceutical products, i.e. medical devices, in-vitro diagnostics, AI, and Substances of Human Origin (SoHO).

The starting point is a good understanding of the innovative technology and of its inherent risks, so that appropriate safety and quality requirements can be applied for monitoring the outcome in the relevant healthcare setting. As the number of hybrid or combinations of health technologies increases and technology integration becomes rather the norm than an exception in health innovation, the current segregated, technology-specific, frameworks may not provide a clear path forward for the health technology that is targeted. To that end, when considering

⁴³⁸ EMA definition: “Real-World Data are routinely collected data relating to patient health status or the delivery of healthcare from a variety of sources other than traditional clinical trials (e.g. claims databases, hospital data, electronic health records, registries, mhealth data, etc.)”.

⁴³⁹ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

an innovation, it is important to consider all relevant legislative frameworks including MDR⁴⁴⁰ and IVDR⁴⁴¹, the proposed SoHO-Regulation⁴⁴², and AI Act⁴⁴³ among others.

Proposals are encouraged to consider, where relevant, the data, expertise and services offered by European research infrastructures especially those active in the health domain, such as EATRIS ERIC⁴⁴⁴, and also the findings of previous EU-projects (e.g.: CORE-MD⁴⁴⁵).

HORIZON-HLTH-2027-01-IND-01: Development of cell-free protein synthesis platforms for discovery and/or production of biologicals

Call: Cluster 1 - Health (Single stage - 2027/1)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 6.00 and 8.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 24.50 million.
<i>Type of Action</i>	Research and Innovation Actions
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). Subject to restrictions for the protection of European communication networks.
<i>Award criteria</i>	The criteria are described in General Annex D. The following

⁴⁴⁰ Regulation (EU) 2017/745 of the European Parliament and of the Council of 5 April 2017 on medical devices: <https://eur-lex.europa.eu/eli/reg/2017/745/oj>

⁴⁴¹ Regulation (EU) 2017/746 of the European Parliament and of the Council of 5 April 2017 on in vitro diagnostic medical devices: <https://eur-lex.europa.eu/eli/reg/2017/746/oj>

⁴⁴² Regulation (EU) 2024/1938 of the European Parliament and of the Council of 13 June 2024 on standards of quality and safety for substances of human origin intended for human application: <https://eur-lex.europa.eu/eli/reg/2024/1938/oj>

⁴⁴³ <https://digital-strategy.ec.europa.eu/en/policies/regulatory-framework-ai>, <https://eur-lex.europa.eu/eli/reg/2024/1689/oj>

⁴⁴⁴ European Infrastructure for Translational Medicine: <https://www.eatris.eu>

⁴⁴⁵ Improved methods for clinical investigation and evaluation of high-risk medical devices: <https://www.core-md.eu>

	exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Maintaining an innovative, sustainable, and competitive EU health industry”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Biopharmaceutical industries get access to streamlined development and production processes for peptide- or protein-based biologicals.
- Health systems benefit from the availability of enhanced or decentralised production systems for innovative health technologies that involve peptides or proteins, and which improve health and care.
- Citizens and patients will benefit from better access, availability and affordability of pharmaceuticals based on biologicals.

Scope: Cell-Free Protein Synthesis (CFPS) has been employed in fundamental biological research for decades, however, interest for the approach as a viable means for drug development and production has only emerged in recent years. The advantages that CFPS provides in terms of efficiency, simplicity, flexibility, cost- and time savings outweigh the hurdles that are still to be overcome for CFPS to become a routine manufacturing system for peptide- or protein-based biologicals.

Currently, there are several CFPS systems used that are either based on prokaryotic or eukaryotic cell lysates (including mammalian) or fully synthetic systems consisting of all the molecular machinery necessary to create functional proteins. The choice of a specific lysate is dictated by the target protein and the end-use application. Proteins that require post-translational modification are generally produced using lysates of mammalian cells. Hence systems based on mammalian cells are of particular interest as they combine properties inherent to eukaryotic cells and their ability to produce human-like glycosylated proteins with the advantages of cell-free synthesis. These proteins include antibody fragments, antigens, virus-like particles, cytokines, enzymes, antimicrobial peptides and proteins containing non-natural amino acids. The benefits of CFPS are manifold, from ease of handling and scalability, on-demand launch of production, ability to rapidly switch products, simplified purification to facilitated standardisation and quality control. CFPS needs less energy

resources, the manufacturing footprint is less complex and smaller than in cell cultivation and it enables production of proteins that have toxic effects on cells. In addition, CFPS has the potential as an enabling technology for personalised medicines and is amenable to decentralised manufacturing. CFPS has gained even more interest in the recent past owing to advances in synthetic biology and thanks to the rise of Machine-Learning/Artificial Intelligence (ML/AI). The use of generative deep learning and artificial intelligence has high potential in the *de-novo* design of biomolecules with specific properties of therapeutic and/or preventive nature. CFPS offers here great opportunities to increase the throughput in screening of the *de-novo* created biomolecules.

The application of synthetic biology, potentially also combined with generative AI, and cell-free biosynthesis open up new avenues for the design, discovery and manufacture of therapeutics not only against infectious diseases, but also non-communicable diseases and equally for vaccines.

Proposals should address at least two of the following elements:

- Address the bottlenecks that currently hamper the large-scale deployment of CFPS, i.e. the lack of a quality-by-design approach, the need to fully characterise the underlying cell lysates and their critical quality attributes and the need for better understanding of the correlations between specific cell lysate properties and CFPS process parameters, specific product quality attributes (such as protein folding), and CFPS platform performance.
- Use synthetic biology techniques for the design of *de-novo* biomolecules with specific desired properties (antimicrobial, immunogenic, angiogenic, etc.) and develop suitable cell-free systems for the high-throughput screening of the designed biomolecules.
- Develop novel or optimise existing CFPS platforms for the production of the targeted biomolecule to a Good Manufacturing Practices (GMP)⁴⁴⁶ conform process, producing clinical-grade material that can be tested in clinical trials.

The demonstration of the superiority of the developed CFPS platform as compared to the current state-of-the art production system for a specific therapeutic peptide or protein would be an asset and participation of start-ups, micro, small and medium-sized enterprises (SMEs)⁴⁴⁷ is encouraged.

Applicants envisaging to include clinical studies⁴⁴⁸ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

⁴⁴⁶ https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

⁴⁴⁷ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

⁴⁴⁸ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

HORIZON-HLTH-2027-02-IND-02-two-stage: Portable and versatile Point-of-care diagnostics

Call: Cluster 1 - Health (Two stage - 2027)	
Specific conditions	
<i>Expected EU contribution per project</i>	The Commission estimates that an EU contribution of between EUR 5.00 and 7.00 million would allow these outcomes to be addressed appropriately. Nonetheless, this does not preclude submission and selection of a proposal requesting different amounts.
<i>Indicative budget</i>	The total indicative budget for the topic is EUR 39.30 million.
<i>Type of Action</i>	Innovation Actions
<i>Admissibility conditions</i>	The conditions are described in General Annex A. The following exceptions apply: Applicants submitting a proposal for a blind evaluation (see General Annex F) must not disclose their organisation names, acronyms, logos nor names of personnel in the proposal abstract and Part B of their first-stage application (see General Annex E).
<i>Eligibility conditions</i>	The conditions are described in General Annex B. The following exceptions apply: In recognition of the opening of the US National Institutes of Health's programmes to European researchers, any legal entity established in the United States of America is eligible to receive Union funding. If projects use satellite-based earth observation, positioning, navigation and/or related timing data and services, beneficiaries must make use of Copernicus and/or Galileo/EGNOS (other data and services may additionally be used). In line with the "restriction on control in innovation actions in critical technology areas" delineated in General Annex B of the General Annexes, entities established in an eligible country but which are directly or indirectly controlled by China or by a legal entity established in China are not eligible to participate in the action.
<i>Award criteria</i>	The criteria are described in General Annex D. The following exceptions apply: For the first stage, the thresholds for each criterion will be 4 (Excellence) and 4 (Impact). The overall threshold applying to the sum of the two individual scores will be set at a level that ensures the total requested budget of proposals admitted to stage 2 is as close as possible to four times the available budget, and not less than three and a half

	times the available budget. For the second stage, the thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.
<i>Procedure</i>	The procedure is described in General Annex F. The following exceptions apply: The first-stage proposals of this topic will be evaluated blindly.
<i>Legal and financial set-up of the Grant Agreements</i>	The rules are described in General Annex G. The following exceptions apply: The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5. Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025) ⁴⁴⁹.

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Maintaining an innovative, sustainable, and competitive EU health industry”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Healthcare professionals dispose of diagnostic tools at the point of care that accelerate therapeutic decision making.
- Patients benefit from fast and accurate diagnosis leading to improved health outcomes.
- Thanks to more efficient diagnosis, health systems will get better evidence for disease control and prevention strategies.

Scope: Point-of-Care (PoC) medical testing has made great technical progress (e.g. improved extraction, microfluidics, miniaturisation, and data processing techniques) with PoC test accuracies nearly matching those of lab-based tests. PoC tests may thus be an alternative to laboratory testing methods, enabling faster diagnostic results and therapeutic decision making. However, PoC testing is not always achieving a completely accurate diagnosis and one of the major issues with PoC diagnostics is the occurrence of false results during testing, another one

⁴⁴⁹ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

is the often-cumbersome sample preparation. Hence there is a need for PoC diagnostics that are more sensitive, selective and easy-to-use allowing for improved clinical practice.

The World Health Organization (WHO) has defined a set of criteria for PoC diagnostics in primary care which, in the advent of digital technologies, has been completed with two additional features and is represented by the acronym REASSURED: RReal-time connectivity, EEase of specimen collection and environmental friendliness, AAffordable, SSensitive, SSpecific, UUser-friendly, RRapid and robust, EEquipment-free (or equipment-modest) and DDeliverable to end users. To these criteria adds the feature of “sample-to-answer” (sometimes also called “sample-to-result”) and more challenges like: Miniaturisation, power supply, versatility (nature and origin of the human sample), biocompatibility of the used materials and their suitability for mass production, readiness for high-throughput testing, quality control, regulatory compliance, environmental footprint and, last but not least, cost, which is of particular concern in resource-limited settings. All these challenges are not only valid for PoC diagnostics developed for infectious diseases, they equally apply to those that are designed to detect non-communicable diseases as well as their continuous monitoring on patients. Mobile technologies are playing an important role, especially since around 70% of the globally 7.4 billion cell phone users live in developing countries, which are the areas in direct need of advanced and more accessible PoC diagnostics (lower density of relevant health infrastructure, e.g. hospitals and laboratory medicine testing facilities). Mobile phones have not only been proposed and tested for data acquisition and readout of assays, images and other results but also for sample processing (e.g. for heating step), as have been Machine-Learning/Artificial Intelligence (ML/AI) powered algorithms that are integrated in the diagnostic devices to analyse complex biological data and detect patterns that might be missed by human analysis.

The selection of the PoC device to be developed or optimised should be based on an objectively conducted clinical needs assessment, which includes -next to clinicians’ perspectives- the complete care pathway and system-level needs. Moreover, a value-based concept should be applied in the choice and development of the PoC device, taking into account its Health Technology Assessment (HTA) by the relevant HTA bodies, in order to facilitate their decisions for adoption.

Proposals should be driven by a clear clinical need, integrate a value-based concept and include all the following activities:

- The optimisation of (the) targeted PoC diagnostic device(s) that take(s) the above-mentioned criteria, challenges and aspects into consideration.
- The elaboration of a comparative study clearly demonstrating the added value and improved performance of the optimised PoC diagnostic device(s) as compared to the current state of the art for the targeted diagnostic application.
- The conduct of clinical studies of (the) optimised PoC diagnostic medical device(s) as a preferred information source for their clinical validation; subsequent conformity

assessment in agreement with requisite EU's In-Vitro Medical Device (IVDR) or Medical Device (MDR) regulatory requirements.

In general, priority should be given to approaches that are suitable for resource-limited settings. In case of targeting infectious diseases, priority should be given to approaches enabling the distinction between viral, bacterial or fungal infections. In case of targeting non-communicable diseases, priority should be given to approaches that are used in emergency cases where decisions can have life-saving character.

Applicants invited to the second stage and envisaging to include clinical studies⁴⁵⁰ should provide details of their clinical studies in the dedicated annex using the template provided in the submission system.

⁴⁵⁰ Please note that the definition of clinical studies (see introduction to this Work Programme part) is broad and it is recommended that you review it thoroughly before submitting your application.

Other Actions not subject to calls for proposals

Grants to identified beneficiaries

1. Contribution to the activities of the Coalition for Epidemics Preparedness Initiative (CEPI) in 2026

The Coalition for Epidemic Preparedness Innovations (CEPI) is a global initiative focused on vaccine development for pathogens causing epidemic threats. It has played a crucial role in the Union's response to COVID-19. This funding will enable CEPI to issue competitive calls to develop medical countermeasures for diseases with epidemic potential. The grants will support research on new vaccines to prevent future epidemics.

Expected Outcome: Proposals should set out a credible pathway to contributing to one or several expected impacts of destination: “Tackling diseases and reducing disease burden”. Project results under this action are expected to contribute to all the following expected outcomes:

- Healthcare providers have access to newly developed medical countermeasures against prioritised pathogens with epidemic potential.
- Citizens benefit from improvements in prevention and containment of epidemics.
- Research funders, policymakers and the research community will have better tools for achieving Sustainable Development Goal (SDG) 3.3⁴⁵¹ “to combat communicable diseases” and to implement SDG Target 3.b⁴⁵² “to support the research and development of vaccines and medicines for the communicable and non-communicable diseases”.

Scope: CEPI is an international non-profit association established under Norwegian Law. Its objective is to finance and coordinate the development of new vaccines to prevent and contain infectious diseases that have epidemic potential. The Horizon Europe funding will be used to enhance and expand CEPI’s activities. This action will also contribute to the implementation of the Union’s strategy for international cooperation in research and innovation and the EU’s development policy.

Accordingly, the proposals should cover all the following activities:

- Vaccine research and development for emerging pathogens.
- Development of adaptable vaccine technologies.
- Collaboration with stakeholders in epidemic preparedness.

⁴⁵¹ https://www.who.int/data/gho/data/themes/topics/sdg-target-3_3-communicable-diseases

⁴⁵² <https://www.who.int/data/gho/data/themes/topics/indicator-groups/indicator-group-details/GHO/sdg-target-3.b-development-assistance-and-vaccine-coverage>

This action is expected to engage with other relevant initiatives, such as the co-funded European Partnership for Pandemic Preparedness⁴⁵³ and the European Vaccine Hub⁴⁵⁴. Where appropriate, this action can also consider One Health approaches and engage with relevant stakeholders in the field, considering the increased risks for animal-to-human contacts and for human-to-human transmission of disease on a global scale in a globalised world.

With the grant from the European Union, CEPI will be able to award one or several grants to third parties through competitive calls for proposals. The call(s) will be issued to fund advanced pre-clinical as well as clinical research on new vaccines for the prevention of emerging and re-emerging infectious diseases. For this purpose this action is also expected to engage with HERA, and support the EU policy goals of scientific leadership in pharmaceutical research and development and strategic autonomy. Therefore, in its financing of third parties, this action should favour activities conducted in Member States or Associated Countries. Such activities may include early-phase clinical trials, and/or enabling activities, for instance the manufacturing of GMP-grade⁴⁵⁵ batches of investigational products. The action is expected to incentivise the development of local expertise and strengthen regional scientific capacities in Member States and Associated Countries, in balance with the overarching need to prioritise scientific excellence as well as logistical and strategic considerations.

The expected recipients of the grant(s) issued by CEPI include research institutes, universities, small and medium-sized enterprises (SMEs)⁴⁵⁶ as well as large companies, all active in research and innovation on new and improved vaccines.

This grant will be awarded without a call for proposals according to Article 198(e) of the Financial Regulation and Article 24(3)(b) of the Horizon Europe Regulation to CEPI, as CEPI has been a key partner for implementing the common Union response to the COVID-19 epidemic.

Award criteria:

The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

Legal and financial set-up of the Grant Agreements:

The funding rate will be 70%.

The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

⁴⁵³ <https://cordis.europa.eu/project/id/101226682>, <https://beready4pandemics.eu>

⁴⁵⁴ https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1306

⁴⁵⁵ Good Manufacturing Practices: https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

⁴⁵⁶ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

Financial support provided by CEPI to third parties is one of the primary activities of this action in order to be able to achieve its objectives as CEPI does not have the capacity to develop new medical countermeasures itself. The maximum amount to be granted to a third party is EUR 15 million. This is justified by the high cost of development for new vaccines, that reaches tens of millions of Euros⁴⁵⁷. However, if the objectives of the action would otherwise be impossible or overly difficult (and duly justified in the proposal) the maximum amount may be higher.

The financial support to third parties (FSTP) provided by CEPI shall comply with the conditions set in above and in Annex B of the General Annexes. Notwithstanding, by way of derogation, in the context of emergence or development of a serious cross-border threat to health within the meaning of Regulation (EU) 2022/2371⁴⁵⁸, and with the prior agreement of the granting authority, CEPI may, when necessary to achieve underlying emergency response objectives in a quick and efficient manner given the exceptional circumstances, apply shortened submission windows, publish calls through equivalent means to the Funding and Tenders Portal provided wide dissemination is ensured, and/or provide FSTP to targeted entities. The conditions governing any such derogation will be set out in the proposal.

Legal entities:

Coalition for Epidemic Preparedness Innovations, Marcus Thranes gate 2, 0473 Oslo, Norway

Form of Funding: Grants not subject to calls for proposals

Type of Action: Grant to identified beneficiary according to Financial Regulation Article 198(e) - Programme co-fund action

The general conditions, including admissibility conditions, eligibility conditions, award criteria, evaluation and award procedure, legal and financial set-up for grants, financial and operational capacity and exclusion, and procedure are provided in parts A to G of the General Annexes

Indicative timetable: Fourth Quarter of 2026

Indicative budget: EUR 39.30 million from the 2026 budget

2. Presidency event - Ireland. Bridging Worlds - Climate Change and Health through the lens of the One Health Agenda - Research, Innovation and Problem Solving

This action will cover the organisation of a conference by the Irish Presidency, focusing on the impacts of climate change on Health and the research needs within the wider context of the One Health agenda.

⁴⁵⁷ Gouglas, D. et al.: Estimating the cost of vaccine development against epidemic infectious diseases: a cost minimisation study. *Lancet Global Health* Vol. 6 (12) E1386-E1396. DOI: [https://doi.org/10.1016/S2214-109X\(18\)30346-2](https://doi.org/10.1016/S2214-109X(18)30346-2),
[https://www.thelancet.com/journals/langlo/article/PIIS2214-109X\(18\)30346-2/fulltext](https://www.thelancet.com/journals/langlo/article/PIIS2214-109X(18)30346-2/fulltext)

⁴⁵⁸ Regulation (EU) 2022/2371 of the European Parliament and of the Council of 23 November 2022 on serious cross-border threats to health and repealing Decision No 1082/2013/EU.

The impacts of climate change present significant threats to public health, including extreme heat, exacerbated health effects of pollution, antimicrobial resistance, zoonotic transfer of pathogens, shifting diseases vectors, and food and water availability and quality.

In recent years, the importance of adopting a One Health approach has become increasingly evident, particularly in Research and Innovation. These efforts are vital to safeguard human, animal, and plant health, alongside their shared environments and economic competitiveness.

The conference should be informed by key EU initiatives such as:

- The Commission's Scientific Advice Mechanism (SAM) opinion on One Health⁴⁵⁹ supporting an integrated approach and emphasising the importance of interdisciplinary and cross-sectoral collaboration, including in research and innovation.
- Actions and initiatives focusing on optimising the health of people, animals, and ecosystems, such as the co-funded European Partnerships on One Health Anti-Microbial Resistance⁴⁶⁰ and on Animal Health and Welfare⁴⁶¹ and the co-funded European Partnership for Pandemic Preparedness⁴⁶², and including the European Climate and Health Observatory⁴⁶³ and the EU Global Health Strategy⁴⁶⁴.
- The Strategic Research and Innovation Agenda (SRIA) on Health and Climate change, reflecting key concepts in One Health such as inter- and transdisciplinary research, or the role of vectors in the climate related spread of infectious diseases.

This conference will champion the One Health agenda as the foundational framework for building climate-resilient health systems across Europe. By fostering cross-disciplinary collaboration the complex health threats posed by climate change can effectively be addressed and mitigated, moving from reactive responses to proactive prevention and sustainable adaptation.

The conference will create an opportunity for policymakers, national health ministries, research institutions, experts, academics public health agencies, environmental organisations, agricultural sector representative and civil society to share innovative research and solutions to solve climate change-related health risks.

The conference should aim to address the following goals:

- Highlighting and promoting innovative research and evidence-based solutions from across EU Member States being developed under One Health agenda, focusing on:

⁴⁵⁹ One Health governance in the European Union - Scientific Advice Mechanism: <https://scientificadvice.eu/advice/one-health-governance-in-the-european-union>

⁴⁶⁰ <https://cordis.europa.eu/project/id/101217154>, <https://ohamr.eu>

⁴⁶¹ <https://cordis.europa.eu/project/id/101136346>, <https://www.eupahw.eu>

⁴⁶² <https://cordis.europa.eu/project/id/101226682>, <https://beready4pandemics.eu>

⁴⁶³ European Climate and Health Observatory: <https://climate-adapt.eea.europa.eu/en/observatory>

⁴⁶⁴ EU Global Health Strategy: Better Health for All in a Changing World - European Commission: https://health.ec.europa.eu/publications/eu-global-health-strategy-better-health-all-changing-world_en

- o Climate Change Impacts on Health and Ecosystems: i) emerging and re-emerging zoonotic diseases, antimicrobial resistance and climate change, ii) food and water security challenges in a changing climate and their health implications, iii) supporting healthcare provision during the green transition.
- o Advancing Research and Innovation for Climate-Resilient One Health Systems: i) cutting-edge data surveillance and early warning systems for climate-sensitive health threats, ii) sustainable agriculture and food systems for health and climate mitigation.
- o Policy, Governance, and Implementation: i) integration of One Health policies and datasets into national and European climate adaptation strategies and health policies, ii) guidance that has been developed -or is under development- to strengthen interdisciplinarity in research and innovation, along with mechanisms to assess the effectiveness of One Health implementation, iii) public engagement, communication, and education for One Health and climate action.
- Facilitating knowledge sharing and networking across a wide range of disciplines together to and spark inspiration on climate and health challenges.
- Building on the “Strategy for European Life Sciences”⁴⁶⁵ aiming to strengthen life sciences research and innovation in Europe, supporting wide ranging green transitions.
- Building on the Strategic Research Agenda on health and climate change, identifying research gaps and data infrastructure requirements relevant to progressing climate and health under a One Health agenda.

This event would result in a proceedings paper to underpin a call for action to foster inter- and transdisciplinary collaboration in research and innovation at the intersection between climate and health and the One Health Agenda so as to help inform relevant policies being developed at both EU and Member State level.

This grant will be awarded without a call for proposals according to Article 198(e) of the Financial Regulation and Article 24(3)(b) of the Horizon Europe Regulation to the legal entity identified below.

Award criteria:

The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

Procedure:

⁴⁶⁵ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

The evaluation committee will be composed fully by representatives of EU institutions.

Legal and financial set-up of the Grant Agreements:

Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025)⁴⁶⁶.

The funding rate will be 100%.

Subcontracting is not restricted to a limited part of the action.

Legal entities:

Department of Health, Government of Ireland, 50 - 58, Block 1, Miesian Plaza, Baggot Street Lower, Dublin 2, D02 XW14

Form of Funding: Grants not subject to calls for proposals

Type of Action: Grant to identified beneficiary according to Financial Regulation Article 198(e) - Coordination and support action

The general conditions, including admissibility conditions, eligibility conditions, award criteria, evaluation and award procedure, legal and financial set-up for grants, financial and operational capacity and exclusion, and procedure are provided in parts A to G of the General Annexes

Indicative timetable: First Quarter of 2026

Indicative budget: EUR 0.30 million from the 2026 budget

3. Presidency event - Lithuania. Strengthening the European clinical research ecosystem for advanced therapy medicinal products and substances of human origin

This action will cover the organisation of a high-level conference by the Lithuanian presidency, focusing on strengthening the European clinical trials ecosystem with a particular emphasis on Advanced Therapy Medicinal Products (ATMPs) and Substances of Human Origin (SoHO). These innovative therapies which offer the potential for regeneration or repair, involve complex trials but are central to Europe's ambition to become a global leader in cutting-edge medical research and patient-centred care. The decline of clinical research activity in Europe not only impacts economic competitiveness but also limits patient access to transformative therapies, especially in areas such as rare diseases, oncology, and regenerative medicine.

⁴⁶⁶ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under 'Simplified costs decisions' or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

ATMPs and SoHO-based interventions -including cell and gene therapies, tissue-engineered products, and treatments derived from blood, plasma, or other human sources- face unique scientific, regulatory and operational challenges. These include heightened ethical considerations, complex trial logistics, and fragmented regulatory interpretations. As the EU continues to lag significantly in ATMP clinical trial activity, this event aims to build momentum for targeted action, drawing on recent EU initiatives such as the “Biotechnology and Biomanufacturing Communication”⁴⁶⁷, the “Strategy for European Life Sciences”⁴⁶⁸ and the upcoming Biotech Act⁴⁶⁹.

This conference will bring together policymakers, regulators, academic and clinical researchers, patient representatives, and industry experts in the field of regenerative medicine to align efforts on advancing innovation, ensuring robust regulation, accelerating patient access, and fostering collaboration across the European health ecosystem.

The conference should aim to address the following goals:

- Identify key obstacles to effective implementation of ATMP and SoHO clinical research in the EU and Associated Countries, including regulatory fragmentation, variability in ethics review, manufacturing, site activation delays, funding and lack of trial-readiness.
- Highlight best practices from countries supporting innovation-friendly environments, including national strategies and reimbursement policies.
- Explore enabling infrastructures for ATMP and SoHO product development, such as academic clinical trial networks and decentralised, hospital-based manufacturing of ATMPs.
- Advance policy recommendations aimed at supporting pan-European ATMP/SoHO networks, fast-track regulatory pathways, and strategic investment in trial and manufacturing capacity and capabilities.

The outcomes of the conference will directly inform EU-level policy discussions on the future of ATMP and SoHO clinical research ecosystems, the next Research and Innovation (R&I) framework programme, the European Health Union⁴⁷⁰, and the European Medicines Regulatory Network⁴⁷¹, contributing to a resilient and innovation-driven health system across the EU.

⁴⁶⁷ Commission Communication on Building the future with nature: Boosting Biotechnology and Biomanufacturing in the EU; COM(2024) 137 final: https://research-and-innovation.ec.europa.eu/document/download/47554adc-dffc-411b-8cd6-b52417514cb3_en

⁴⁶⁸ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

⁴⁶⁹ https://ec.europa.eu/info/law/better-regulation/have-your-say/initiatives/14627-Biotech-Act_en

⁴⁷⁰ https://commission.europa.eu/document/download/98c6e4dc-0fc3-4ec6-8ec2-bfcdcb2f018a_en?filename=policy_com-2024-206_en.pdf

⁴⁷¹ <https://www.ema.europa.eu/en/about-us/how-we-work/european-medicines-regulatory-network>

By focusing on ATMPs and SoHO-based interventions, this initiative underscores the EU's commitment to the EU's competitiveness as well as scientific excellence, health sovereignty, and equitable patient access to frontier therapies.

This grant will be awarded without a call for proposals according to Article 198(e) of the Financial Regulation and Article 24(3)(b) of the Horizon Europe Regulation to the legal entity identified below.

Award criteria:

The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

Procedure:

The evaluation committee will be composed fully by representatives of EU institutions.

Legal and financial set-up of the Grant Agreements:

Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025)⁴⁷².

The funding rate will be 100%.

Subcontracting is not restricted to a limited part of the action.

Legal entities:

Ministry of Health of the Republic of Lithuania, Vilniaus g. 33, Vilnius, 01506 Vilniaus m. sav., Lithuania

Form of Funding: Grants not subject to calls for proposals

Type of Action: Grant to identified beneficiary according to Financial Regulation Article 198(e) - Coordination and support action

The general conditions, including admissibility conditions, eligibility conditions, award criteria, evaluation and award procedure, legal and financial set-up for grants, financial and operational capacity and exclusion, and procedure are provided in parts A to G of the General Annexes

Indicative timetable: Third Quarter of 2026

⁴⁷² This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under 'Simplified costs decisions' or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

Indicative budget: EUR 0.30 million from the 2026 budget

4. Contribution to the activities of the Coalition for Epidemics Preparedness Initiative (CEPI) in 2027

The Coalition for Epidemic Preparedness Innovations (CEPI) is a global initiative focused on vaccine development for pathogens causing epidemic threats. It has played a crucial role in the Union's response to COVID-19. This funding will enable CEPI to issue competitive calls to develop medical countermeasures for diseases with epidemic potential. The grants will support research on new vaccines to prevent future epidemics.

Expected Outcome: Proposals should set out a credible pathway to contributing to one or several expected impacts of destination: “Tackling diseases and reducing disease burden”. Project results under this action are expected to contribute to all the following expected outcomes:

- Healthcare providers have access to newly developed medical countermeasures against prioritised pathogens with epidemic potential.
- Citizens benefit from improvements in prevention and containment of epidemics.
- Research funders, policymakers and the research community will have better tools for achieving Sustainable Development Goal (SDG) 3.3⁴⁷³ “to combat communicable diseases” and to implement SDG Target 3.b⁴⁷⁴ “to support the research and development of vaccines and medicines for the communicable and non-communicable diseases”.

Scope: CEPI is an international non-profit association established under Norwegian Law. Its objective is to finance and coordinate the development of new vaccines to prevent and contain infectious diseases that have epidemic potential. The Horizon Europe funding will be used to enhance and expand CEPI’s activities. This action will also contribute to the implementation of the Union’s strategy for international cooperation in research and innovation and the EU’s development policy.

Accordingly, the proposals should cover all the following activities:

- Vaccine research and development for emerging pathogens.
- Development of adaptable vaccine technologies.
- Collaboration with stakeholders in epidemic preparedness.

This action is expected to engage with other relevant initiatives, such as the co-funded European Partnership for Pandemic Preparedness⁴⁷⁵ and the European Vaccine Hub⁴⁷⁶. Where

⁴⁷³ https://www.who.int/data/gho/data/themes/topics/sdg-target-3_3-communicable-diseases

⁴⁷⁴ <https://www.who.int/data/gho/data/themes/topics/indicator-groups/indicator-group-details/GHO/sdg-target-3.b-development-assistance-and-vaccine-coverage>

⁴⁷⁵ <https://cordis.europa.eu/project/id/101226682>, <https://beready4pandemics.eu>

⁴⁷⁶ https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1306

appropriate, this action can also consider One Health approaches and engage with relevant stakeholders in the field, considering the increased risks for animal-to-human contacts and for human-to-human transmission of disease on a global scale in a globalised world.

With the grant from the European Union, CEPI will be able to award one or several grants to third parties through competitive calls for proposals. The call(s) will be issued to fund advanced pre-clinical as well as clinical research on new vaccines for the prevention of emerging and re-emerging infectious diseases. For this purpose this action is also expected to engage with HERA, and support the EU policy goals of scientific leadership in pharmaceutical research and development and strategic autonomy. Therefore, in its financing of third parties, this action should favour activities conducted in Member States or Associated Countries. Such activities may include early-phase clinical trials, and/or enabling activities, for instance the manufacturing of GMP-grade⁴⁷⁷ batches of investigational products. The action is expected to incentivise the development of local expertise and strengthen regional scientific capacities in Member States and Associated Countries, in balance with the overarching need to prioritise scientific excellence as well as logistical and strategic considerations.

The expected recipients of the grant(s) issued by CEPI include research institutes, universities, small and medium-sized enterprises (SMEs)⁴⁷⁸ as well as large companies, all active in research and innovation on new and improved vaccines.

This grant will be awarded without a call for proposals according to Article 198(e) of the Financial Regulation and Article 24(3)(b) of the Horizon Europe Regulation to CEPI, as CEPI has been a key partner for implementing the common Union response to the COVID-19 epidemic.

Award criteria:

The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

Legal and financial set-up of the Grant Agreements:

The funding rate will be 70%.

The granting authority may, up to 4 years after the end of the action, object to a transfer of ownership or to the exclusive licensing of results, as set out in the specific provision of Annex 5.

Financial support provided by CEPI to third parties is one of the primary activities of this action in order to be able to achieve its objectives as CEPI does not have the capacity to develop new medical countermeasures itself. The maximum amount to be granted to a third

⁴⁷⁷ Good Manufacturing Practices: https://health.ec.europa.eu/medicinal-products/eudralex/eudralex-volume-4_en

⁴⁷⁸ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

party is EUR 15 million. This is justified by the high cost of development for new vaccines, that reaches tens of millions of Euros⁴⁷⁹. However, if the objectives of the action would otherwise be impossible or overly difficult (and duly justified in the proposal) the maximum amount may be higher.

The financial support to third parties (FSTP) provided by CEPI shall comply with the conditions set in above and in Annex B of the General Annexes. Notwithstanding, by way of derogation, in the context of emergence or development of a serious cross-border threat to health within the meaning of Regulation (EU) 2022/2371⁴⁸⁰, and with the prior agreement of the granting authority, CEPI may, when necessary to achieve underlying emergency response objectives in a quick and efficient manner given the exceptional circumstances, apply shortened submission windows, publish calls through equivalent means to the Funding and Tenders Portal provided wide dissemination is ensured, and/or provide FSTP to targeted entities. The conditions governing any such derogation will be set out in the proposal.

Legal entities:

Coalition for Epidemic Preparedness Innovations, Marcus Thranes gate 2, 0473 Oslo, Norway

Form of Funding: Grants not subject to calls for proposals

Type of Action: Grant to identified beneficiary according to Financial Regulation Article 198(e) - Programme co-fund action

The general conditions, including admissibility conditions, eligibility conditions, award criteria, evaluation and award procedure, legal and financial set-up for grants, financial and operational capacity and exclusion, and procedure are provided in parts A to G of the General Annexes

Indicative timetable: Fourth Quarter of 2027

Indicative budget: EUR 34.40 million from the 2027 budget

5. Presidency event - Greece. Climate change and health - Aligning Research and Innovation for a resilient future (European and global perspectives)

Objective: This action will cover the organisation of a high-level conference by the Greek Presidency, focusing on Climate Change and Health. The primary objective of the proposed conference is to facilitate high-level dialogue among policymakers, researchers, industry leaders and civil society organisations on the interlinkages between climate change and health. The focus will be on understanding and addressing the impacts of climate change on human health, increasing climate adaptation and resilience and reducing the health sector's

⁴⁷⁹ Gouglas, D. et al.: Estimating the cost of vaccine development against epidemic infectious diseases: a cost minimisation study. *Lancet Global Health* Vol. 6 (12) E1386-E1396. DOI: [https://doi.org/10.1016/S2214-109X\(18\)30346-2](https://doi.org/10.1016/S2214-109X(18)30346-2),
[https://www.thelancet.com/journals/langlo/article/PIIS2214-109X\(18\)30346-2/fulltext](https://www.thelancet.com/journals/langlo/article/PIIS2214-109X(18)30346-2/fulltext)

⁴⁸⁰ Regulation (EU) 2022/2371 of the European Parliament and of the Council of 23 November 2022 on serious cross-border threats to health and repealing Decision No 1082/2013/EU.

contribution to climate change. The conference aims to identify and propose integrated strategies for aligning research and innovation as well as funding to address the dual challenges posed by climate change and public health.

Scope: The interplay between climate change and human health represents one of the defining challenges of our time. The climate crisis is a health crisis with impacts at global level. Across Europe, the fastest-warming continent, heat and floods have caused devastating human and economic impact in recent years. The European Union has already demonstrated global leadership through the European Green Deal⁴⁸¹, the EU Mission on Climate Adaptation, the “Strategy for European Life Sciences”⁴⁸² flagship on health and climate change and the EU Strategic Research and Innovation Agenda on Health and Climate Change⁴⁸³ published in 2025. The Greek Presidency intends to reinforce the EU’s role as a driver of knowledge-based policy and international cooperation and advance this leadership by fostering dialogue and joint action on aligning research and innovation efforts to protect citizens’ health and wellbeing in a rapidly changing climate.

This forward-looking conference will take stock of the progress achieved by ongoing initiatives and serve as a pivotal platform for discussing pressing climate-related health issues, assessing the impacts of climate change on public health systems and evaluating innovative research and technological solutions, while addressing also the socio-economic dimensions of resilience. Emphasising collaboration across Europe and globally, it will encourage sharing of best practices, evidence, innovative technologies, and resources among stakeholders in public health, environmental science, and technology sectors. Discussions will focus on:

- Advancing climate-health research excellence.
- Integrating health considerations into climate adaptation and mitigation strategies.
- Boosting climate neutrality and circularity in the healthcare sector.
- Mobilising innovative technologies, nature-based solutions and social innovations.
- Enhancing preparedness and responsiveness of public health systems.

Target audience:

- EU policymakers and officials (EU institutions and agencies, ministries of research, health, and environment from EU Member States and Associated Countries).
- Researchers and academic experts in environmental science, public health and innovation.
- Innovators and industry stakeholders in climate and health sectors, industry representatives from pharmaceutical and technology sectors.

⁴⁸¹ https://commission.europa.eu/strategy-and-policy/priorities-2019-2024/european-green-deal_en

⁴⁸² https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en

⁴⁸³ <https://op.europa.eu/en/publication-detail/-/publication/616cce9c-39e5-11f0-8a44-01aa75ed71a1>

- International organisations and health agencies (WHO, UNEP, IPCC, OECD).
- Non-Governmental Organisations (NGOs) and civil society organisations focused on climate action and health, including youth and vulnerable community representatives.
- Media representatives covering environmental and health issues.

Outline of key elements to be addressed by the conference:

- Scientific evidence & risk assessment: Latest knowledge on vulnerabilities and health risk projections.
- Policy coherence: Aligning EU, national, and global frameworks to address climate-health challenges.
- Innovation pathways: Technological and social innovations for prevention, adaptation and mitigation.
- Financing synergies: Coordinating funding instruments (e.g. Horizon Europe, EU4Health Programme (2021-2027)⁴⁸⁴, national budgets, global funds).
- Equity & inclusion: Ensuring solutions are inclusive and socially equitable.
- Global partnerships: Strengthening multilateral climate-health cooperation and coordination.

Expected Outcomes and Deliverables: The conference will provide a timely opportunity to consolidate knowledge, foster dialogue and chart a forward-looking action plan by:

- Identifying actionable practices for integrating climate and health considerations into research and policy frameworks at national, European and global level.
- Informing future research and regulatory frameworks that address knowledge gaps through coordinated approaches.
- Fostering multi-stakeholder dialogue and creating synergies among academia, policymakers, industry, and civil society.
- Facilitating EU-level policy discussions, supporting the European Green Deal, Horizon Europe programme, the “Strategy for European Life Sciences” and the European Health Union agenda⁴⁸⁵.
- Promoting the uptake of research results and innovative solutions, by connecting developers with policymakers and end-users.

⁴⁸⁴ https://commission.europa.eu/funding-tenders/find-funding/eu-funding-programmes/eu4health_en

⁴⁸⁵ https://commission.europa.eu/strategy-and-policy/priorities-2019-2024/promoting-our-european-way-life/european-health-union_en

The outcomes will directly support EU strategies and ongoing initiatives on climate and health research while ensuring coherence with European and global agendas, maximising the impact of research and innovation for societal benefit. The conference will deliver an action plan delineating key research and innovation activities to address the climate change and health crisis.

This grant will be awarded without a call for proposals according to Article 198(e) of the Financial Regulation and Article 24(3)(b) of the Horizon Europe Regulation to the legal entity identified below.

Award criteria:

The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

Procedure:

The evaluation committee will be composed fully by representatives of EU institutions.

Legal and financial set-up of the Grant Agreements:

Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the Research and Training Programme of the European Atomic Energy Community (2021-2025)⁴⁸⁶.

The funding rate will be 100%.

Subcontracting is not restricted to a limited part of the action.

Legal entities:

General Secretariat for Research and Innovation, Ministry of Development, 14-18 Mesogeion Avenue, 11527, Athens, Greece

Form of Funding: Grants not subject to calls for proposals

Type of Action: Grant to identified beneficiary according to Financial Regulation Article 198(e) - Coordination and support action

The general conditions, including admissibility conditions, eligibility conditions, award criteria, evaluation and award procedure, legal and financial set-up for grants, financial and

⁴⁸⁶ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

operational capacity and exclusion, and procedure are provided in parts A to G of the General Annexes

Indicative timetable: First Quarter of 2027

Indicative budget: EUR 0.30 million from the 2027 budget

6. Maintaining the European registry for human pluripotent stem cell lines

Expected Outcome: This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, project results are expected to contribute to the following expected outcomes:

- The public has access to a database of pluripotent cell lines used in Europe and beyond.
- Researchers can maximise the use of existing stem cell lines and avoid duplicated efforts.

Scope: The aim is to maintain, update and make available a registry containing information on the different human Pluripotent Stem Cell (hPSC) lines derived in Europe and their use in EU funded projects under Horizon Europe with the overall aim of avoiding needless creation of new cell lines. This registry should be open access and at a minimum consist of a robust website that will continue to provide high quality data about the lines (e.g. cell characteristics), details regarding their source and contact information regarding their location. Further work should be done to ensure robustness of the infrastructure and maintenance of the resource on the long term.

This grant will be awarded without a call for proposals according to Article 198(f) of the Financial Regulation to the legal entity that manages the European registry for hPSC lines.

Specific Type of Action:

Coordination and support action

Award criteria:

The criteria are described in General Annex D. The following exceptions apply: The thresholds for each criterion will be 4 (Excellence), 4 (Impact) and 4 (Implementation). The cumulative threshold will be 12.

Legal and financial set-up of the Grant Agreements:

Eligible costs will take the form of a lump sum as defined in the Decision of 7 July 2021 authorising the use of lump sum contributions under the Horizon Europe Programme – the Framework Programme for Research and Innovation (2021-2027) – and in actions under the

Research and Training Programme of the European Atomic Energy Community (2021-2025)⁴⁸⁷.

The funding rate will be 100%.

Subcontracting is not restricted to a limited part of the action.

Legal entities:

A Higher or Secondary Education Establishment or Research Organisation responsible for the European registry for hPSC lines

Form of Funding: Grants not subject to calls for proposals

Type of Action: Grant awarded without call for proposals according to Financial Regulation Article 198 (f)

The general conditions, including admissibility conditions, eligibility conditions, award criteria, evaluation and award procedure, legal and financial set-up for grants, financial and operational capacity and exclusion, and procedure are provided in parts A to G of the General Annexes

Indicative timetable: First Quarter of 2027

Indicative budget: EUR 1.47 million from the 2027 budget

Other Instruments

1. External expertise

This action will support the use of appointed independent experts for the monitoring of running actions (grant agreement, grant decision, public procurement actions, financial instruments) funded under Horizon Europe and previous Framework Programmes for Research and Innovation, for ethics checks, for the evaluation of large actions annual work plans, as well as for compliance checks regarding the Gender Equality Plan eligibility criterion. A special allowance of EUR 450/day will be paid to the experts appointed in their personal capacity who act independently and in the public interest.

Form of Funding: Other budget implementation instruments

Type of Action: Expert contract action

Indicative budget: EUR 1.95 million from the 2026 budget and EUR 1.95 million from the 2027 budget

⁴⁸⁷ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under 'Simplified costs decisions' or through this link: https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

2. External expertise in relation to EU research and innovation policy issues

This action will support the provision of independent expertise in support of the assessment, design, implementation, evaluation and valorisation of EU research and innovation policies in the areas currently in scope of the Health Cluster.

Individual experts will work on tasks such as, but not limited to: portfolio analysis of projects funded under Horizon Europe or previous European research and innovation programmes; analysis of the contribution of research results (at national, EU and/or international level) to EU policy objectives and emerging issues, including policy recommendations; analysis of the state-of-the-art at European and international level; participation in studies, conferences, events, symposia, etc, including the drafting of papers and reports on their conclusions; assistance for setting-up a research and innovation strategy for selected domains; policy recommendations and options assisting Commission services in elaborating evidence-based and scientifically sound policy proposals; assistance in the evaluation of calls for expression of interest; advice on the valorisation, communication, dissemination and exploitation of research results; identification of innovative solutions as well as potential gaps and synergies to be addressed by EU research and innovation policy; advice on promising technologies covered by European and nationally funded projects and on ways to stimulate synergies, etc.

In addition to individual experts, this action could provide for Commission expert groups.

A special allowance of maximum EUR 450/day will be paid to the experts appointed in their personal capacity who act independently and in the public interest.

Form of Funding: Other budget implementation instruments

Type of Action: Expert contract action

Indicative budget: EUR 0.09 million from the 2026 budget and EUR 0.09 million from the 2027 budget

3. Mobilisation of research funds in case of Public Health Emergencies

Expected Outcome:

Proposals should set out a credible pathway to contributing to one or several expected impacts of destination: “Tackling diseases and reducing disease burden”.

Project results are expected to contribute to the following expected outcome: Allow the Union to respond to Public Health Emergencies.

Scope:

In case of a public health emergency⁴⁸⁸ (such as a Public Health Emergency of International Concern (PHEIC) according to the World Health Organization, a public health emergency

⁴⁸⁸ Should there be no Public Health Emergency in 2026 or 2027, the indicative budget may be reallocated.

under Regulation (EU) 2022/2371⁴⁸⁹ or under applicable national frameworks and regulations), funding will be mobilised for:

- The award of grants without a call for proposals according to Article 198 (b) of the EU Financial Regulation⁴⁹⁰ in exceptional and duly substantiated emergencies. At that time, the Funding & Tenders Portal will open a dedicated section where proposals can be submitted. This will be communicated to the National Contact Points. The invitation to apply for funding will be open to all eligible entities or be limited to targeted entities, taking into account the need to achieve the underlying objectives in a quick and efficient manner considering the exceptional circumstances; and/or
- The award of additional funding for ongoing grant agreements funded through EU Framework Programmes for Research and Innovation to cover additional activities specifically linked to the public health emergency, in exceptional and duly substantiated emergencies. Providing such additional funding to ongoing EU Framework Programmes for Research and Innovation grants that can support pertinent short- and mid-term research efforts to confront the public health emergency will save valuable time and allow addressing the situation with the appropriate urgency. Restricted calls for expression of interest or proposals will develop such additional activities or add additional partners to existing EU Framework Programmes for Research and Innovation actions.

It is expected that quality-controlled data are shared in accordance with the FAIR⁴⁹¹ principles. The use of harmonised protocols in collaboration with other actors is recommended for this purpose.

The standard eligibility and admissibility criteria, evaluation criteria, thresholds, weighting for award criteria, maximum funding rate and conditions for providing financial support to third parties, are provided in the General Annexes.

The beneficiaries must comply with the public emergency related provisions listed in the General Annexes concerning the project implementation under Intellectual Property Rights (IPR), background and results, access rights and rights of use (article 16 and Annex 5) for the duration of the Public Health Emergency; and under Communication, dissemination, open science and visibility (article 17 and Annex 5) during the entire duration of the action and for four years after the end of the action.

The following derogations to the evaluation procedure described in General Annexes D and F apply to open invitations to submit applications:

In order to ensure a balanced portfolio covering responses to different aspects of the public health emergency, grants will be awarded to applications not only in order of ranking, but also

⁴⁸⁹ <https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX%3A32022R2371&qid=1673372768554>

⁴⁹⁰ Article 198 (b) of the Financial Regulation 2018/1046 "Grants may be awarded without a call for proposals only in the following cases: [...] (b) in other exceptional and duly substantiated emergencies;"

⁴⁹¹ See definition of FAIR data in the introduction to this Work Programme part.

to those projects that enhance the quality of the project portfolio through synergies between projects and avoidance of overlaps, provided that the applications attain all thresholds.

The action may also include justified derogations from the standard limits to financial support to third parties. Where applicable, the relevant grant agreement options will be applied.

The award will be without a call for proposals according to Article 198(b) of the Financial Regulation.

Form of Funding: Grants not subject to calls for proposals

Type of Action: Grant awarded without call for proposals according to Financial Regulation Article 198 (b)

The general conditions, including admissibility conditions, eligibility conditions, award criteria, evaluation and award procedure, legal and financial set-up for grants, financial and operational capacity and exclusion, and procedure are provided in parts A to G of the General Annexes

Indicative timetable: Will depend on the Public Health Emergency

Indicative budget: EUR 0.90 million from the 2026 budget and EUR 0.90 million from the 2027 budget

4. Studies, conferences, events and outreach activities

A number of specific contracts will be signed in order to: (i) support the dissemination and exploitation of project results; (ii) contribute to the definition of future challenge priorities; (iii) undertake citizen surveys such as Eurobarometers; (iv) carry out specific evaluations of programme parts; (v) support future European Research Area (ERA) policy actions; and (vi) organise conferences, events and outreach activities.

Subject matter of the contracts envisaged: studies, technical assistance, conferences, events and outreach activities.

Form of Funding: Procurement

Type of Action: Public procurement

Indicative timetable: 2026 and 2027

Indicative budget: EUR 0.41 million from the 2026 budget and EUR 0.65 million from the 2027 budget

5. Subscription to the Human Frontier Science Program Organization

An annual subscription to the international Human Frontier Science Program Organization (HFSP)⁴⁹² will allow researchers from EU non-G7 Member States to fully benefit from the Human Frontier Science Program (HFSP), enable initiatives to help the affected scientific community in and from areas recently severely ravaged by conflict and/or war on European ground and contribute to the implementation of the Global Approach to Research and Innovation, Europe's strategy for international cooperation in a changing world⁴⁹³.

Type of Action: Subscription action

Indicative timetable: 2026 and 2027

Indicative budget: EUR 7.04 million from the 2026 budget and EUR 7.18 million from the 2027 budget

6. Valorising and boosting the science for policy link of climate-health research and innovation outcomes

Climate change is a global existential challenge that has introduced unprecedented pressures on the planet. It is now widely recognised that the climate crisis is a health crisis and that urgent action is needed to protect citizens and healthcare systems from the increasingly emerging risks and health threats posed by climate stressors.

Research and Innovation (R&I) funding schemes and programs play a central role in supporting and accelerating the health response required by ongoing climate threats, providing evidence, tools, technologies and solutions that equip societies to become more resilient and prepared.

The effective translation of health and climate research into policy and implementation has been identified as a key challenge in the recently published Strategic Research and Innovation Agenda on Health and Climate Change⁴⁹⁴. There is a need for robust and consistent frameworks that bridge actionable outcomes with evidence-based climate and health action and yield research finding better suited to application in the real world.

To fill this gap the following action is proposed: Valorising and boosting the science for policy link of climate-health research and innovation outcomes.

The objective of this action is to develop a pilot study to test pathways and approaches for building an effective climate and health evidence-for-implementation ecosystem.

⁴⁹² The Commission is a member of the HFSP Organization (HFSPO) and has funded HFSP under previous Framework Programmes

⁴⁹³ Communication from the Commission on the Global Approach to Research and Innovation. Europe's strategy for international cooperation in a changing world, COM(2021) 252, 18.5.2021 (<https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=COM%3A2021%3A252%3AFIN>).

⁴⁹⁴ <https://op.europa.eu/en/publication-detail/-/publication/616cce9c-39e5-11f0-8a44-01aa75ed71a1>

The European Climate and Health Observatory⁴⁹⁵ is a multi-disciplinary partnership, jointly managed by the European Commission and the European Environment Agency (EEA), comprised of international and European organisations working in the fields of climate, health and environmental policy and research, that supports with accessible and reliable information Europe in preparing for and adapting to the impacts of climate change on human health.

By leveraging on the privileged position of the Observatory as a central hub for access to evidence on climate and health at the European level, this action aims at developing a case study on the use of thematic platforms as providers of:

- Structured interaction with EU funded climate and health research to support the development of solutions that are fit for purpose, incorporate needs of end-users and are grounded on the wide EU landscape.
- A sounding board for the identification of existing needs of stakeholders and targeted feedback collection.
- A specialised platform for dissemination of relevant outputs, including adaptation solutions, to climate and health stakeholders and communities.

The pilot study will provide a lessons learned analysis with actionable insights for the implementation of research valorisation frameworks using the case of climate change and health research as a model.

The action should be implemented through a Service Level Agreement with the European Environment Agency (EEA), as foreseen under Article 59.2 of the Financial Regulation.

Type of Action: Service Level Agreement

Indicative timetable: Fourth Quarter of 2026

Indicative budget: EUR 0.50 million from the 2026 budget

7. Measuring the impact of the Life Sciences Strategy through ecosystem monitoring

The development of a monitoring tool is needed in order to assess progress towards the 2030 ambition of the “Strategy for European Life Sciences”⁴⁹⁶ to position the Union as a global leader in life sciences. Building on the existing web-based interactive infrastructure of the European Commission's Joint Research Centre (JRC), the action will support the development, operation and annual updating of a dashboard providing a single factual reference point for the Strategy's implementation. The dashboard will present baseline macroeconomic indicators together with a set of key performance indicators tracking the impact of the “Strategy for European Life Sciences” ecosystem. Activities conducted under the action may include the identification and refinement of indicators, the automation of data

⁴⁹⁵ <https://climate-adapt.eea.europa.eu/en/observatory>

⁴⁹⁶ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en;
https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

ingestion, processing and visualisation, the provision of EU-level trends and, where possible, Member State and subsector breakdowns.

The action will be implemented, following Article 59 and Article 21.2.(g).(iii) of the Financial Regulation, in the form of an Administrative Agreement/Service Level Agreement.

Type of Action: Provision of technical/scientific services by the Joint Research Centre - SLA in the form of administrative agreement.

Indicative timetable: Fourth Quarter of 2026

Indicative budget: EUR 0.10 million from the 2026 budget and EUR 0.20 million from the 2027 budget

8. Connecting key actors in the climate change space to identify barriers and foster uptake and implementation of actionable climate-health research and innovation outcomes

Climate change is a global existential challenge that has introduced unprecedented pressures on the planet. It is now widely recognised that the climate crisis is a health crisis and that urgent action is needed to protect citizens and healthcare systems from the increasingly emerging risks and health threats posed by climate stressors.

Research and Innovation (R&I) funding schemes and programs play a central role in supporting and accelerating the health response required by ongoing climate threats, providing evidence, tools, technologies and solutions that equip societies to become more resilient and prepared.

The effective translation of health and climate research into policy and implementation has been identified as a key challenge in the recently published Strategic Research and Innovation Agenda on Health and Climate Change⁴⁹⁷. There is a need for robust and consistent frameworks that bridge actionable outcomes with evidence-based climate and health action and yield research finding better suited to application in the real world.

To fill this gap the following action is proposed: Connecting key actors in the climate change space to identify barriers and foster uptake and implementation of actionable climate-health research and innovation outcomes.

The objective of this action is to build on the extensive World Health Organization (WHO) multi-stakeholder networks and policy forums to facilitate dedicated exchanges between health authorities and evidence providers and investigate pathways for increased impact and adequacy of research and innovation outcomes. Based on enhanced conversations between needs and impacts practitioners and implementers, existing barriers will be evaluated and

⁴⁹⁷ <https://op.europa.eu/en/publication-detail/-/publication/616cce9c-39e5-11f0-8a44-01aa75ed71a1>

recommendations provided on potential requirements and effective approaches and frameworks for increased social impact and uptake of scalable and transferable R&I solutions.

Legal entities:

World Health Organization (WHO), Avenue Appia 20, 1211, Geneva, Switzerland

Form of Funding: Indirectly managed actions

Type of Action: Indirectly managed action

Indicative timetable: First Semester of 2027

Indicative budget: EUR 0.10 million from the 2026 budget and EUR 0.15 million from the 2027 budget

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Horizon Europe - Work Programme 2026-2027
Health

Budget^{498 499}

	Budget line(s)	2026 Budget (EUR million)	2027 Budget (EUR million)
Calls			
HORIZON-HLTH-2026-01		471.60	
	<i>from 01.020210</i>	<i>471.60</i>	<i>0.00</i>
HORIZON-HLTH-2026-02		48.70	42.60
	<i>from 01.020210</i>	<i>48.70</i>	<i>42.60</i>
HORIZON-HLTH-2026-03		30.00	33.00
	<i>from 01.020210</i>	<i>30.00</i>	<i>33.00</i>
HORIZON-HLTH-2026-04		9.80	
	<i>from 01.020210</i>	<i>9.80</i>	<i>0.00</i>
HORIZON-HLTH-2027-01			331.60 ⁵⁰⁰
	<i>from 01.020210</i>	<i>0.00</i>	<i>331.60</i>
HORIZON-HLTH-2027-02-two-stage			186.60
	<i>from 01.020210</i>	<i>0.00</i>	<i>186.60</i>
HORIZON-HLTH-2027-03			81.50

⁴⁹⁸ The budget figures given in this table are rounded to two decimal places.

The budget amounts are subject to the availability of the appropriations provided for in the general budget of the Union for 2026 and 2027.

⁴⁹⁹ The contribution from Cluster 1 for year 2026 is EUR 124.70 million for the Missions work programme part and EUR 23.31 million for the New European Bauhaus Facility work programme part.

The contribution from Cluster 1 for year 2027 is EUR 122.60 million for the Missions work programme part and EUR 22.23 million for the New European Bauhaus Facility work programme part.

⁵⁰⁰ To which EUR 10.00 million from the 'Climate, Energy and Mobility' budget will be added making a total of EUR 341.60 million for this call.

Horizon Europe - Work Programme 2026-2027
Health

	<i>from 01.020210</i>	<i>0.00</i>	<i>81.50</i>
Other actions			
Grant awarded without a call for proposals according to Financial Regulation Article 198(e)		39.90	34.70
	<i>from 01.020210</i>	<i>39.90</i>	<i>34.70</i>
Grant awarded without a call for proposals according to Financial Regulation Article 198		0.90	2.37
	<i>from 01.020210</i>	<i>0.90</i>	<i>2.37</i>
Expert contract action		2.04	2.04
	<i>from 01.020210</i>	<i>2.04</i>	<i>2.04</i>
Public procurement		0.41	0.65
	<i>from 01.020210</i>	<i>0.41</i>	<i>0.65</i>
Subscription action		7.04	7.18
	<i>from 01.020210</i>	<i>7.04</i>	<i>7.18</i>
Service Level Agreement		0.50	
	<i>from 01.020210</i>	<i>0.50</i>	<i>0.00</i>
Provision of technical/scientific services by the Joint Research Centre - SLA in the form of administrative agreement.		0.10	0.20
	<i>from 01.020210</i>	<i>0.10</i>	<i>0.20</i>
Indirectly managed action		0.10	0.15
	<i>from 01.020210</i>	<i>0.10</i>	<i>0.15</i>
Estimated total budget		611.09	722.59